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Certificate of Need Application
to Establish
a
Medicare/Medicaid Certified Hospice Agency
Yakima County

December 2025

Certificate of Need Application
Hospice Agency

Certificate of Need applications must be submitted with a fee in accordance with Washington Administrative Code (WAC) 246-310-990.

Application is made for a Certificate of Need in accordance with provisions in Revised Code of Washington (RCW) 70.38 and WAC 246-310, rules and regulations adopted by the Washington State Department of Health. I attest that the statements made in this application are correct to the best of my knowledge and belief.

Signature and Title of Responsible Officer Nathan Yemane, MSW, LCSW, LICSW Managing Director Email Address nyemane@heartnsoulhospice.com	Date December 19, 2025 Telephone Number (206) 659 9998
Legal Name of Applicant Heart'n Soul Hospice of Yakima County Address of Applicant 15 S. Grady Way, Suite #527 Renton, WA 98057	Provide a brief project description ☒ New Agency ☐ Expansion of Existing Agency ☐ Other: _____ Estimated capital expenditure: $ 22.000
Identify the county proposed to be served for this project. Note: Each hospice application must be submitted for one county only. If an applicant intends to obtain a Certificate of Need to serve more than one county, then an application must submitted for each county separately. Yakima	

Section 1
APPLICATION DESCRIPTION

1. **Provide the legal name(s) and address(es)of the applicant(s).**

The legal name of the applicant is Heart'n Soul Hospice of Yakima, LLC (Heart'n Soul). Throughout the application, the applicant will be referred to as "Heart'n Soul." Heart'n Soul is registered with the Washington Secretary of State. The members/governors are Nathan Yemane, David Turner, and Tracy Wood.

The legal address is:

15 South Grady Way
Renton, WA 98057

2. **Identify the legal structure of the applicant (LLC, PLLC, etc.) and provide the Unified Business Identifier (UBI)**

Heart'n Soul is a Washington State limited liability company organized under chapter 25.15 RCW. Heart'n Soul's UBI # is 606 020 831.

3. **Provide the name, title, address, telephone number, and email address of the contact person for this application.**

Questions regarding this application should be directed at both:

Nathan Yemane MSW, LCSW, LICSW
15 South Grady Way
Renton, WA 98057
nyemane@heartnsoulhospice.com
206-659-9998

Tracy Wood
51 Century Blvd Suite 110
Nashville, TN 37214
twood@heartnsoulhospice.com
(972) 517-6300

4. **Provide the name, title, address, telephone number, and email address of the consultant authorized to speak on your behalf related to the screening of this application (if any)**

The consultant for this application is:

Peggy Bertals, Contractor
Heart'n Soul Hospice
51 Century Blvd Suite 110
Nashville, TN 37214
(215) 740-6898
pbertals@heartnsoulhospice.com

5. **Provide an organizational chart that clearly identifies the business structure of the applicant(s)**

An organizational chart is included in **Exhibit 1**.

6. **Identify all healthcare facilities and agencies owned, operated by, or managed by the applicant or its affiliates with overlapping decision-makers. This should include all facilities in Washington State as well as out-of-state facilities.**
   The following identifying information should be included:
   - **Facility and Agency Name(s)**
   - **Facility and Agency Location(s)**
   - **Facility and Agency License Number(s)**
   - **Facility and Agency CMS Certification Number(s)**
   - **Facility and Agency Accreditation Status**
   - **If acquired in the last three full calendar years, list the corresponding month and year the sale became final**
   - **Type of facility or agency (home health, hospice, other)**

There are no other facilities or agencies owned by the Applicant. However, Heart'n Soul members, David Turner and Nathan Yemane, own Y.B.G. Healthcare LLC dba Heart and Soul, operating in King County. The details of that agency are:

- **Y*.B.G Healthcare LLC, dba Heart and Soul Hospice, Renton, WA:***

Members David Turner and Nathan Yemane each own 50% of this agency. It is licensed in Washington, accredited by the Community Health Accreditation Program (CHAP), and certified by both Medicare and Medicaid. The agency secured CN approval in 2022 and became accredited and operational on October 17, 2023. Its address and license number is below:

Heart’n Soul Hospice (King County)
15 S Grady Way Ste 527, Renton, WA, 98057-3217
License #: IHS.FS.61379202
Medicare Provider Number: 501552
Medicaid Provider Number: 2269278

In addition, Members, David Turner, Nathan Yemane, and Tracy Wood are the owners of Heart’n Soul Hospice of Pierce County LLC. The details of that agency are:

- ***Heart'n Soul Hospice of Pierce County LLC dba Heart’n Soul Hospice.***

Heart’n Soul Members, David Turner, Nathan Yemane, and Tracy Wood each own one-third of Heart’n Soul Hospice of Pierce County dba Heart’n Soul Hospice. This entity was granted Certificate of Need (CN) approval in the 2025 concurrent review cycle and is currently seeking licensure, Medicare and Medicaid certification and CHAP accreditation. Once operational, the Pierce County agency will share space with the King County agency but will remain a separate entity.

A listing of other agencies for which Governing members have ownership interests, and/or operational or management control is provided in **Exhibit 2**.

Section 2
PROJECT DESCRIPTION

1. **Provide the name and address of the existing agency, if applicable.**

Heart'n Soul Hospice of Yakima County LLC is not an existing agency.

2. **If an existing Medicare and Medicaid certified hospice agency, explain if/how this proposed project will be operated in conjunction with the existing agency.**

Heart'n Soul is not an existing agency.

3. **Provide the name and address of the proposed agency. If an address is not yet assigned, provide the county parcel number and the approximate timeline for assignment of the address.**

The name and address of the proposed agency is:

Heart'n Soul Hospice of Yakima County LLC
6 South 2nd Street
Suite #706
Yakima, WA 98901

4. **Provide a detailed description of the proposed project.**

Heart'n Soul Hospice is seeking approval to establish a hospice agency in Yakima County, where the CN Program's updated hospice agency need projection shows need for an additional agency. Yakima County's 2024 overall hospice utilization rate (41.5%) is more than 20% lower than the state rate of 51.7% and is 46% lower than the nation (60.6%). Through Heart'n Soul's documented and successful community engagement (described in more detail later in the application) we will meet the defined community need and improve access, thereby improving hospice utilization rates throughout the County.

While we do fully intend to serve all individuals that qualify for hospice and choose our care, we will also have a strong emphasis on reducing disparities and increasing acceptance and use of hospice in traditionally underserved communities. We will accomplish this by addressing cultural relevance, and by increasing trust in the Black, Indigenous, People of Color (BIPOC) and Hispanic communities, as well as in other traditionally underserved communities in Yakima County.

According to the U.S. Census Bureau's 2023 American Community Survey, more than half of Yakima County's population identify as Hispanic. The largest group within the Hispanic population is of Mexican origin, accounting for nearly 90% of the total Hispanic population in Yakima County. Nineteen percent of the Yakima County population is also foreign-born (47,572). Of this foreign-born population in Yakima County, almost 90% are from Mexico. Thirteen percent of Yakima County residents are non-citizens, which is higher than the state (8%) and national (7%) average. Over 40% of Yakima County residents also speak a language other than English at home.

Yakima County is located on the ancestral homelands and traditional territories of the people of the Yakama Nation. Today, the Yakama Nation Reservation includes more than 1.3 million acres. Four percent of the Yakima County residents identify as American Indian or Alaska Native. 23% as some other race alone, and another 26% identify with two or more races.[1]

Yakima County also has higher rates of poverty than the state, with nearly 34% of families classified as poor (below 100% of the Federal Poverty Level) or struggling (below 200% of the FPL), compared to only 17% statewide.

Statistics by race and ethnicity also show significant income disparities; with 12.3% of the white population in Yakima classified as poor compared to 16% of the Black alone population, 23% of the American Indian and Alaska Native alone population, 20% of the Some Other Race alone population, and 18% of Two or More Races alone population; and 19% of the Hispanic population.

Yakima County residents also experience disproportionately higher rates of homelessness. In 2023, 10,763 homeless residents resided in Yakima County, a 55% increase since 2016. At a rate of 41.54 per 1,000 residents, Yakima County's homelessness rate is significantly higher than the state rate of 21.41 per 1,000.

Heart'n Soul will be Medicare and Medicaid certified and will serve all persons with terminal illness in Yakima County desiring our services designed to manage pain and maintain dignity through the end of life. We will also have a specific focus on serving BIPOC, Hispanic, and other underserved communities including:

- The dual eligible Medicare/Medicaid
- Homeless/Housing Insecure/Migrant Farmworker Housing
- LGTBQIA+
- Veterans

[1] *According to U.S. Census Bureau Guidance on the Presentation and Comparison of Race and Hispanic Origin Data, Race and Hispanic origin are two separate concepts in the federal statistical system. People who are Hispanic may be of any race. This results in overlap between race and Hispanic origin data. For example, black Hispanics are included in both the number of Blacks and the number of Hispanics.*

5. **Confirm that this agency will be available and accessible to the entire geography of the county proposed to be served.**

Yes, Heart'n Soul will be available and accessible to individuals regardless of where they reside in the County.

6. **With the understanding that the review of a Certificate of Need application typically takes at least six to nine months, provide an estimated timeline for project implementation, below.**

The estimated timeline for project implementation is:

Event	Anticipated Month/Year
CN Approval	September 2026
Design Complete (if applicable)	NA
Construction Commenced (if applicable)	NA
Construction Completed (if applicable)	NA
Agency Prepared for Survey	April 2027
Agency Providing Medicare and Medicaid hospice services in the proposed county.	July 2027

7. **Identify the hospice services to be provided by this agency by checking all applicable boxes below. For hospice agencies, at least two of the services identified below must be provided.**

The following services will be provided in a linguistically and culturally competent manner:

X Skilled Nursing	X Durable Medical Equipment (contracted)
X Home Health Aide	X IV Services
X Physical Therapy (contracted)	X Nutritional Counseling
X Occupational Therapy (contracted)	X Bereavement Counseling
X Speech Therapy (contracted)	X Symptom and Pain Management
X Respiratory Therapy (contracted)	X Pharmacy Services (contracted)-
X Medical Social Services	X Respite Care
X Palliative Care	X Spiritual Counseling
X Other – services and programming designed to respect the needs of each individual patient and community, including but not limited to: the Senior Journey Program that promotes health equity and addresses the social determinants of health (SDOH), ensuring that all individuals receive the care and support they deserve, regardless of their background or circumstances; the Hero Journey Program through which we provide knowledgeable compassionate care, emotional, and spiritual support to our Veterans and First Responders; and the End-of-Life Doula program that provides non-medical, trained support individuals who provide companionship, comfort, and guidance to those facing a terminal illness or death.	

8. **If this application proposes expanding an existing hospice agency, provide the county(ies) already served by the applicant and identify whether Medicare and Medicaid services are provided in the existing county(ies).**

Heart'n Soul is not proposing to expand an existing agency. Rather, we will establish a separate legal entity and a separately licensed agency. Our affiliate, Y.B.G. Healthcare LLC dba Heart'n Soul Hospice is providing Medicare and Medicaid services in King County. Another affiliate, Heart'n Soul Hospice of Pierce County, recently received CN approval to provide hospice services in Pierce County and is currently seeking licensure, certification and accreditation for that agency.

9. **If this application proposes expanding the service area of an existing hospice agency, clarify if the proposed services identified above are consistent with the existing services provided by the agency in other planning areas.**

This application does not propose expanding an existing agency but will operate with the same fundamental values and philosophy of care as our affiliates in King and Pierce Counties.

10. **Provide a general description of the types of patients to be served by the agency at project completion (age range, diagnoses, special populations, etc.).**

Heart'n Soul will provide a full range of hospice services designed to meet the physiological, psychological, social, cultural, linguistic and spiritual needs of people and their families facing the end of life and bereavement in Yakima County. As noted earlier, while serving all individuals that qualify for hospice and choose our care, we will have a strong emphasis on reducing disparities and increasing cultural relevance and trust so as to increase the rate of hospice use in the BIPOC, Hispanic and other underserved communities in Yakima County.

The BIPOC and Hispanic communities in the United States often experience trust issues with healthcare due to injustices that have occurred historically, and unfortunately, continue to occur today. This mistrust, among other impacts, has had consequences on end-of-life care for BIPOC and Hispanic persons and their families and loved ones. Heart'n Soul, as a BIPOC owned healthcare facility and a member of the BIPOC community, will help build trust within the underserved communities in Yakima County so that care at the end of life is improved. This will include a special focus on developing specific strategies for each underserved group that respect their unique culture, history and barriers to access.

As data in this application shows, the BIPOC community is disproportionately lower income and experiences disproportionately higher rates of homelessness and/or other housing challenges (including residing in migrant farmworker congregate housing). For example, the Yakima Housing Authority provides 172 low-income units throughout Yakima County, specifically set aside for farm workers and their often multi-generational families. Based on these realities, this project is expected to have a higher percentage of Medicaid, dual eligible, uninsured, and homeless than is typical.

We intend to initially and primarily serve adults, aged 18 and over. However, we are well aware that BIPOC and Hispanic children suffer from chronic diseases at a higher rate than non-BIPOC and Hispanic children. A National Institutes of Health study demonstrated that racial/ethnic minority youth have higher rates of chronic disease in the United States. The most common major chronic diseases of youth in the United States are asthma, diabetes mellitus, obesity, hypertension, dental disease, attention-deficit/hyperactivity disorder, mental illness, cancers, sickle-cell anemia, cystic fibrosis, and a variety of genetic and other birth defects[2]. Because the incidence of these conditions, some of which are fatal, are higher in these underserved communities, if requested by families and the community, we would develop and provide services for pediatric patients and their families.

11. Provide a copy of the letter of intent that was already submitted according to WAC 246-310-080 and WAC 246-310-290(3)

A copy of the letter of intent is included in **Exhibit 3**.

12. Confirm that the agency will be licensed and certified by Medicare and Medicaid. If this application proposes the expansion of an existing agency, provide the existing agency's license number and Medicare and Medicaid numbers.

Heart'n Soul will secure State of Washington licensure, Medicare and Medicaid certification, and accreditation by the Community Health Accreditation Program, Inc. (CHAP) upon CN approval.

[2] National Institutes of Health, Racial/Ethnic Disparities in Chronic Diseases of Youths and Access to Health Care in the United States, 2013

Section 3
CERTIFICATE OF NEED REVIEW CRITERIA
NEED
(WAC 246-310-210)

1. **For existing agencies, using the table below, provide the hospice agency's historical utilization broken down by county for the last three full calendar years. Add additional tables as needed.**

Heart'n Soul is not an existing agency. Our affiliate, Y.B.G. Healthcare LLC dba Heart'n Soul Hospice, is currently providing Medicare and Medicaid services in King County; and our affiliate Heart'n Soul Hospice of Pierce County dba Heart'n Soul Hospice recently received Certificate of Need approval and is currently proceeding with state licensure and will provide Medicare and Medicaid services in Pierce County.

2. **Provide the projected utilization for the proposed agency for the first three full years of operation. For existing agencies, also provide the intervening years between historical and projected. Include all assumptions used to make these projections.**

Table 1 details the projected utilization.

Table 1
Heart'n Soul Yakima County
Projected Patient Days and Admissions, 2027-2030

	Partial Year 2027	2028	2029	2030
Total number of admissions	40	148	194	228
Total number of patient days	2,562	9,481	12,428	14,606
Average daily census	13.9	25.9	34.0	40.0

Source: Applicant

Specific assumptions include:

Admissions: Because numeric need exist and given the magnitude and severity of the low hospice penetration rate in the County (more than 20% below the State), we conservatively chose to develop admissions for two cohorts: 1) the cohort of residents that have historically accessed hospice (and serve as the basis for the hospice penetration rate), referred to as the general hospice population, and 2) the underserved BIPOC and Hispanic communities.

Our overall assumption is that, consistent with the WAC methodology's underlying assumption, Yakima County will be able to turn around its low use of hospice and achieve at least the same level of hospice use that the State experienced, on-average in 2024. However, because the WAC-based projections use historical data for its projections, to the extent that the BIOPC, Hispanic, and other underserved communities underutilize hospice, projections will be understated, so there is still opportunity/work that needs to be done to increase hospice utilization for these underserved communities.

- **General Population Admission Assumptions:**

Per the WAC-based methodology, the unmet Average Daily Census (ADC) in Yakima County is estimated at 43 in 2027, or 246 admissions. Assuming the historical rate of growth continues at the same rate through 2030, the unmet ADC is expected to be 55 by 2030 or 321 admissions.

As can be identified in **Table 2**, while Yakima County is growing slowly, it is aging at a much higher rate. Given the aging, even with relatively low population growth, and because 85% of hospice admissions in the State are generated by the 65+ population, the aging of the County will result in increased demand for hospice,

Table 2
Yakima County Demographics

	2010	2020	Pct Chg 2010-2020	2026 est	Pct Chg 2020-2026	2031 proj	Pct Chg 2026-2031
Tot. Pop.	**244,099**	**257,597**	**5.5%**	**260,610**	**1.2%**	**263,929**	**1.3%**
Pop. By Age							
Tot. 0-64	215,884	219,911	1.9%	220,189	0.1%	220,220	0.0%
Tot. 65 +	28,215	37,686	33.6%	40,421	7.3%	43,709	8.1%

Source: Nielsen Claritas.

As identified in **Table 3**, in our 2027 partial year, we have estimated that we will have a conservative 13% market share of the general community unmet need as calculated by the CN program. By 2030, that market share will increase to 56%. These estimated market shares are reasonable and necessary based on our analysis of the market, which has seen existing providers experiencing declining admissions over the last several years, despite the aging of the population and the significant underservice in the County. Data collected and reported by the CN Program demonstrates that over the period of 2019-2024, there has been a ***29% decline*** in hospice admissions for current CN approved hospice agencies in the County; while statewide admissions over the same timeframe ***increased*** by ***over 20%***. While we are well aware of the instability in the Yakima County health care market over the past decade (closure of a hospital, bankruptcy of a health care system, change of ownership of the County's major provider, significant loss of providers, etc.), the trajectory must change if the community is to have the access it needs and deserves to end of life care.

We also fully acknowledge that this instability was compounded in the COVID years by the fact that the County experienced some of the highest infection rates and hospitalizations of any county on the West Coast. A June 2022 Washington State Department of Health report entitled *COVID-19 Outbreaks in Washington State Agriculture and Food Manufacturing Settings,* reports that Counties like Yakima that are heavily reliant on agriculture, produce packing and food manufacturing experienced very high outbreaks and attributed to the fact that agriculture and food manufacturing employees often live, socialize, commute, and work together. State analysis showed that these cases were epidemiologically linked in the congregate setting (e.g., case-patients share a work shift or building, or benefit from employee sponsored transportation or housing).

We are now four years+ post-COVID, but the reduced use of hospice continues and must pivot if residents are to receive the care that they and their families need at end of life.

Table 3
Heart'n Soul Admission Assumptions for General Community

	2027 (Partial Year)	2028	2029	2030
Estimated unmet admissions in Yakima County per WAC methodology	246	271	296	321
Heart'n Soul's estimated admissions from the general population	**31**	**120**	**157**	**181**
Heart'n Soul's Market Share	13%	44%	53%	56%

Source: Department of Health's 2025 Hospice Methodology; Internal Assumptions

- **Underserved Admission Assumptions:**

In addition to addressing the unmet need for the general population and increasing the overall utilization of hospice in Yakima County, Heart'n Soul is committed to addressing inequities in hospice care, and reaching communities traditionally underserved by Hospice.

The Department of Health's WAC-based projections average three years of historical data; as such, the extent that the BIPOC, Hispanic, and other underserved communities underutilize hospice, even employing the State hospice penetration, the projections will continue to understate future demand and perpetuate the underutilization.

As identified in **Table 4**, CMS data as well as data from the Centers for Disease Control confirm the underserved nature of the BIPOC community for hospice care. In 2024 the hospice use rate for the BIPOC and Hispanic communities in Yakima County was significantly less than that of the white population.

Table 4
Yakima County Hospice Utilization Data by Race, 2024

Race	Utilization Rate
White	43.5%
Black	NA
Asian	NA
Hispanic or Latino	34.3%
North American Native	26.9%
Other	34.2%
Unknown	23.5%
Total	41.5%

Source: Developed from Medicare Files; Bergdata.com.

Using this 2024 data as the baseline, had the BIPOC and Hispanic communities accessed hospice at the same rate as the white population, an estimated 181 additional admissions would have occurred. Heart'n Soul believes that this estimate is an extremely conservative estimate as we know that race and ethnicity data is often underreported as it is dependent on patient disclosure and provider accuracy, which are both subject to question due to bilingual communication, fear of immigration status, etc. These underserved estimates also do not include other underserved populations described in this application.

As identified in **Table 5**, the estimated admissions were conservatively estimated to increase 1% per year (less than the general population growth and significantly less than the growth of several BIPOC/Hispanic communities). Heart'n Soul market share of underserved admissions was estimated at 5% in the first partial year, increasing to 25% by the third full year of operation. This is consistent with Heart'n Soul's affiliates' experiences in other underserved communities and provides the needed time to develop trust within each unique underserved community.

Table 5
Heart'n Soul Admission Assumptions for Underserved Communities

	2027 (Partial Year)	**2028**	**2029**	**2030**
Yakima County Hospice estimated additional admissions from underserved BIPOC/Hispanic communities	181	183	185	187
Heart'n Soul's Estimated Admissions from Underserved	**9**	**28**	**37**	**47**
Heart'n Soul's Market Share	5%	15%	20%	25%

Source: Developed from Medicare Files; Bergdata.com; Washington State Department of Health, Center for Health Statistics, Death Certificate Data, Community Health Assessment Tool (CHAT), October 2025; Internal Assumptions.

Average Length of Stay: We have assumed the Washington State average, per the CN Program's methodology (64.06).

Patient Days: Admissions x the ALOS calculate to the projected patient days.

Average Length of Stay: Patient days/365 = ALOS

3. **Identify any factors in the planning area that could restrict patient access to hospice services.**

The Department of Health's 2024 hospice need methodology (included as **Exhibit 4**) projects need for an additional hospice agency in Yakima County.

As stated above, Yakima County's use rate is significantly lower than the statewide use rate, demonstrating a lack of access today. The demographics and significant health disparities that exist within Yakima County also restrict access, including:

- More than half of Yakima County's population identify as Hispanic. Over 40% of Yakima County residents speak a language other than English at home.
- The Migrant Policy Institute estimates that the County is home to approximately 33,000 non-citizens, migrant and seasonal farmworkers, and their dependents.
- Yakima County is also located on the ancestral homelands and traditional territories of the people of the Yakama Nation. Four percent of the Yakima County population identify as American Indian or Alaska Native. 23% as some other race alone, and another 26% identify with two or more races.[3]
- In 2023, American Indian/Alaska Native adults (16%) and Hispanic adults (17%) in Yakima County were more than three times more likely to be uninsured compared to white adults (5%).
- In 2023, the percent of the AI/AN (40%) and Hispanic (40%) populations 0-64 covered by Medicaid was significantly higher than the White population (19%).
- As referenced earlier in the application, the BIPOC and Hispanic communities are also much more likely to be low-income and homeless or living in temporary or congregate housing.

These factors must be given full consideration in the review and CN award of any new hospice agency. Because Heart'n Soul is both BIPOC owned and governed, we are better positioned to gain trust and overcome the insensitivity to cultural variations in attitudes towards death and dying than other providers. We have personally experienced the lack of cultural sensitivity in our specific communities and are fully committed to learning about and developing unique strategies specific to each culture or subgroup we serve. In Yakima County, this will include developing unique strategies for engaging with and addressing the unique needs of the Hispanic and American Indian/Alaska native populations in the County.

A January 2015 publication on *Equity of Care and Eliminating Health Care Disparities* released by a partnership including the American Hospital Association, American College of Healthcare Executives, America's Essential Hospitals, Association of American Medical Colleges and Catholic Health Association of the United States, identified a three-fold process for reducing inequities. These include:

- Increase the collection and use of race, ethnicity, and language preference data;
- Increase cultural competency training; and
- Increase diversity in governance and leadership.

Heart'n Soul has addressed all three in this application.

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[3] *Race and Hispanic origin are two separate concepts in the federal statistical system. People who are Hispanic may be of any race. This results in overlap between race and Hispanic origin data. For example, black Hispanics are included in both the number of Blacks and the number of Hispanics.*

Heart'n Soul Hospice has developed the Senior Journey program for the elderly who may live alone, have neurological disorders, lack many of the social deterrents to have a true quality of life environment. The program promotes health equity and addresses the social determinants of health {SDOH) to ensure that all seniors receive the care and support they deserve, regardless of their background or circumstances and no matter where they live.

The Lower Use of Hospice by the Yakima County BIPOC and Hispanic Community Demonstrates the Magnitude of Inequities.

Table 5 above includes 2024 Yakima County data and demonstrates that the BIPOC/Hispanic communities experienced significantly lower utilization in Yakima County in 2024 than the white population.

Nationally, Heart n' Soul has a number of affiliates that serve predominantly Hispanic communities and has had measurable success in growing the use of hospice through engaging with Hispanic community organizations, including local churches, hiring bi-lingual and tri-lingual local community members to provide culturally appropriate education focusing on end of life care as well working with the local Hispanic community members and organizations to identify and implement specific cultural end of life needs. The goal is to engage and build trust within each unique underserved community. Examples of flyers used in these efforts are included in **Exhibit 5**.

Through Heart'n Soul's Senior Journey Program, a cultural liaison will engage with each underserved community to ensure the same level of understanding and care occurs in each underserved community. For example, the American Indian/Alaska Native (AI/AN) community, the liaison will strive to provide culturally appropriate educational opportunities, raising awareness about hospice services that empower the AI/AN community to make informed decisions about end of life. They will also specifically engage the AI/AN community in the development and implementation of hospice programs that foster trust and acceptance and incorporate traditional health practices and spiritual support.

Within all communities served, Heart'n Soul will additionally offer culturally appropriate support services for family caregivers that can support them through the stress and challenges of elder care.

Dual eligible Medicare/Medicaid Enrollees are Disproportionately BIPOC and Hispanic, and Care Needs are Greater.

A 2023 JAMA report found that in the Medicaid only and dual-eligible populations, Hispanic and non-Hispanic Black individuals had the lowest rates of receiving hospice, and Hispanic individuals had the highest rates of short hospice stays, which result in less positive end-of-life experiences for patients and their families, as well as increased financial and operational strain for health care systems. [4]

[4] 2023 JAMA Health Forum

More than twelve million individuals nationally are simultaneously enrolled in Medicare and Medicaid. A March 2020 CMS study noted that dual eligible individuals have high rates of chronic illness (60% have multiple chronic illnesses) and 18% reported 'poor' health status (compared to 6% of other Medicare beneficiaries)[5]. This report also demonstrates that while the dual eligible represent only 20% of the Medicare program enrollment, they account for 34% of the costs. There were similar findings for the Medicaid Program (15% of the enrollment but 30% of the cost).

A 2024 report found that 44% of the Medicare-Medicaid enrollee populations are from communities of color, compared to 17% of the Medicare-only population and identified the added barriers experienced by dual eligibles from communities of color when navigating both Medicare and Medicaid to obtain their care.

Heart'n Soul's goal will be to increase the hospice utilization rate of dual eligible residents. That said, we recognize that the BIPOC dual-eligible are a sub-set of our larger BIPOC community. In an effort to be conservative in our projections, we did not make this group an incremental add-on to our volume estimates.

The Homeless in Yakima County is also disproportionately BIPOC, and Yakima County also has a significant number of migrant farmworkers living in congregate housing/sanctioned encampments.

Yakima County's 2024 Homeless Point-In-Time Count demonstrates that the homeless community in Yakima County is disproportionaley BIPOC/Hispanic. In the survey, they combined race and ethnicity and determined that 71% of individuals in sanctioned encampments were from the BIPOC/Hispanic community; as were 54% of unsheltered individuals, 65% of those in emergency shelters, and 56% of those in transitional housing.

Heart'n Soul intends to be an active participant in a regional and community approach to address chronic homelessness on a countywide scale. Our goal is to assure the homeless and those that are housing insecure have an opportunity to have end of life care provided in a safe and secure setting. We will also work directly with the Hispanic community to respect the unique needs of the migrant population and their families living in employer provided housing, sanctioned encampments, and congregate housing. We recognize that the BIPOC homeless are a sub-set of our larger BIPOC community, so again, in an effort to be conservative in our projections, we did not make this group an incremental add-on to our volume estimates.

The LGBTQIA+ Community in general, and the BIPOC LBGTQIA+ Community in particular have unmet needs for end of life care.

While specific data is harder to secure, Heart'n Soul knows from experience that the LGBTQIA+ community is underserved. The issue can be magnified for the nonwhite members of the community who often do not feel included—or necessarily even safe—within the larger LGBTQIA+ community. As in other spaces, racism is all too prevalent. People who are part of communities across the BIPOC spectrum also face increased oppression and unique challenges because of the intersection of their cultural and LGBTQIA+ identities.

Heart'n Soul will have specific programs for the LBGTQ IA+ community and will participate in the Northwest LBGTQ+ Senior Care Providers Network, which is an informal group of providers working together to provide advocacy and quality of care for the LGBTQ+ seniors of Washington State. Its vision statement is to:

> *provide education and support for the LGBT senior community when navigating through the health care continuum, while ensuring that their dignity and individual choices are respected and honored.*

We recognize that the BIPOC LBGTQIA+ are a sub-set of our larger community as well and so to be conservative in our projections, we did not make this group an incremental add-on to our volume estimates.

The Veteran Community Would Also Benefit:

Yakima County is home to approximately 11,275 Veterans, representing about 5% of the total population. These numbers highlight a strong veteran presence in the region, serving as a notable demographic within Yakima County.

Improving end-of-life care has been a goal of the Department of Veterans Affairs (VA) for a number of years, either through the Veterans Health Administration (VHA), or through care provided in the community when needed care is not available within its networks. The VA began developing specific initiatives more than 10 years ago and has greatly improved the use of hospice by the VA population. It has done this by honoring Veterans' preferences for care at the end of life and by increasing enrollment in hospice.

Specifically, Heart'n Soul has implemented the Hero Journey Program through which we provide knowledgeable compassionate care, emotional, and spiritual support to our Veterans and First Responders. We recognize their sacrifice and service to our country and where we live. The program was specifically developed to:

- Provide emotional support through our hospice team.
- Honor the Veteran with pinning ceremonies.
- Provide Certificates of Recognition for their outstanding service.
- Provide Anticipatory grief and bereavement counseling for Veteran and first responders' family members/next of kin.
- Provide assistance with completing funeral arrangements.
- Provide volunteers matched to their service.

Again, we recognize that the Veteran's population is a sub-set of our larger community, so did not make this group an incremental add-on to our volume estimates.

4. **Explain why this application is not considered an unnecessary duplication of services for the proposed planning area. Provide any documentation to support the response.**

Numeric need for an additional agency has been identified by the CN Program's methodology, and our program will capture a portion of that defined need and the rest of our admissions will come from growing the underserved. This is not an unnecessary duplication because our intent, via our programming and expertise in community engagement, culturally appropriate education and outreach, and listening to the unique needs of all residents of a community, is to grow the overall hospice use rate and at the same time, grow use in traditionally underserved groups. We will do this by working collaboratively with existing agencies in Yakima County, to ensure all patients desiring hospice services receive those services in a timely, equitable and culturally competent manner, guided by the U.S. Department of Health and Human Services' Culturally and Linguistically Appropriate Services (CLAS) Standards.

Heart'n Soul Hospice's ownership is of the BIPOC community and is committed to increasing the acceptance and utilization of hospice in underserved communities. Our goal is better end of life care, and while we will serve all, our greatest contribution will be in increasing acceptance, trust and ultimately, the use of hospice in the BIPOC, Hispanic, and other underserved communities. Included in **Exhibit 5** are several examples from our affiliate agency in King County and in other programs nationally that demonstrate the comprehensive and culturally appropriate outreach and education we will provide in these communities to build the trust needed to increase acceptance and use of hospice. These outreach programs will be specifically targeted to meet the unique needs of the Yakima County community.

As referenced above, Heart'n Soul Hospice has also developed the Senior Journey Program to promote health equity and address the social determinants of health (SDOH), ensuring that all individuals receive the care and support they deserve, regardless of their background or circumstances. SDOH are non-medical factors that influence health outcomes. SDOH have a major impact on our health, well-being, and quality of life - and SDOH significantly affect our chances of staying healthy as we age. Examples of SDOH include:

- Income and social protection
- Education
- Food insecurity
- Housing, basic amenities, and the environment
- Social inclusion and non-discrimination
- Access to affordable, quality health services

The Program works to reduce health disparities in underserved and vulnerable communities. The goals of the Program are:

- Improving access to healthcare services.

- Reestablishing connections to churches, senior centers for additional support.
- Better understanding between the community and the health and social service system.
- Creating an equitable and inclusive environment for the patient.
- Promoting increased use of healthcare services.
- Promoting improved adherence to health recommendations.
- Reducing the need for emergency and specialty services.

Through a Cultural Liaison Team, the Senior Journey Program can work with first responders, hospitals, churches, senior centers and other community agencies and resources to offer:

- Bereavement support to the community.
- Leverage community resources to support patients' non-medical needs.
- Provide culturally appropriate health education and information.
- Help people get the care they need.
- Give informed counseling and guidance on health behaviors.
- Advocate for individual and community health needs.

To further support all patients and meet them where they are, Heart 'n Soul also provides End of Life Doulas, non-medical, trained support individuals, to provide companionship, comfort, and guidance to those facing a terminal illness or death. Doulas assist patients and families in a variety of ways, from guiding necessary conversations to physical comfort to caregiver relief. They can:

- Offer the family time to talk openly about dying without judgment, including advance care directives, end-of-life wishes (pain management support, funeral wishes, etc.), and the space to process grief and emotions.
- Help with legacy projects to honor the patient.
- Create a comfortable physical space for the patient.
- Acknowledge and celebrate desired traditions, rituals, or spiritual practices.
- Provide comfort care via massage or guided visualization.
- Educate in-home caregivers on physical contact with the patient.
- Sit bedside and ease caregiver burdens via practical help or presence.
- Provide respite for caregivers.
- Educate the patient and caregivers on signs and symptoms of dying.
- Coordinate care with the hospice provider or other medical team members.
- Make referrals to other professionals or community resources.

5. Confirm the proposed agency will be available and accessible to the entire planning area.

Heart'n Soul intends to be available and accessible to residents residing throughout Yakima County.

6. **Identify how this project will be available and accessible to under-served groups.**

The need for an additional provider is demonstrated via WAC and the data on Yakima County's disparities is both compelling and documented. While serving all, Heart'n Soul will focus on the reduction of disparities in access to and use of hospice among certain historically underserved ethnicities, races, and other underserved populations.

Historically, to evaluate this requirement, the Department has evaluated an applicant's admission policies, willingness to serve Medicare and Medicaid patients, and to serve patients that cannot afford to pay for services. Heart'n Soul will secure both Medicare and Medicaid certification and has included a charity care allowance in our pro forma that acknowledges the demographics of the communities we intend to target.

7. **Provide a copy of the following policies:**
   • Admissions policy
   • Charity care or financial assistance policy
   • Patient Rights and Responsibilities policy
   • Non-discrimination policy
   Suggested additional policies include any others believed to be directly related to patient access (death with dignity, end of life, advanced care planning)

The requested policies are included in **Exhibit 6**.

8. **If there is not sufficient numeric need to support approval of this project, provide documentation supporting the project's applicability under WAC 246-310-290(12). This section allows the department to approve a hospice agency in a planning area absent numeric need if it meets the following review criteria: • All applicable review criteria and standards with the exception of numeric need have been met;**
   • The applicant commits to serving Medicare and Medicaid patients; and • A specific population is underserved; or
   • The population of the county is low enough that the methodology has not projected need in five years, and the population of the county is not sufficient to meet an ADC of thirty-five.

There is more than sufficient need to support the approval of this project.

Section 3
CERTIFICATE OF NEED REVIEW CRITERIA
FINANCIAL FEASIBILITY
(WAC 246-310-220)

1. **Provide documentation that demonstrates the immediate and long-range capital and operating costs of the project can be met. This should include but is not limited to:**
   • Utilization projections. These should be consistent with the projections provided under the Need section. Include all assumptions.
   • Pro Forma revenue and expense projections for at least the first three full calendar years of operation using at a minimum the following Revenue and Expense categories identified at the end of this question. Include all assumptions.
   • Pro Forma balance sheet for the current year and at least the first three full calendar years of operation. Include all assumptions. Page 8 of 16 DOH 260-035 September 2021
   • For existing agencies proposing addition of another county, provide historical revenue and expense statements, including the current year. Ensure these are in the same format as the projections. For incomplete years, identify whether the data is annualized.

The applicable requested information is included in **Exhibit 7**.

2. **Provide the following agreements/contracts:**
   - **Management agreement.**
   - **Operating agreement**
   - **Medical director agreement**
   - **Joint Venture agreement**

   Note, all agreements above must be valid through at least the first three full years following completion or have a clause with automatic renewals. Any agreements in draft form must include a document signed by both entities committing to execute the agreement as submitted following CN approval.

There is no management agreement, operating agreement, or joint venture agreement. Heart'n Soul has a draft a medical director agreement with Dr. Josh Kretchmar, license #OP60765453, which is in process of being finalized and signed. A copy of the draft agreement is included in **Exhibit 8**.

3. **Provide documentation of site control. This could include either a deed to the site or a lease agreement for the site.**

If this is an existing hospice agency and the proposed services would be provided from an existing main or branch office, provide a copy of the deed or lease agreement for the site. If a lease agreement is provided, the agreement must extend through at least the projection year. Provide any amendments, addendums, or substitute agreements to be created as a result of this project to demonstrate site control.

If this is a new hospice agency at a new site, documentation of site control includes one of the following:

a. **An executed purchase agreement or deed for the site.**
b. **A draft purchase agreement for the site. The draft agreement must include a document signed by both entities committing to execute the agreement as submitted following CN approval.**
c. **An executed lease agreement for at least three years with options to renew for not less than a total of two years.**
d. **A draft lease agreement. For Certificate of Need purposes, draft agreements are acceptable if the draft identifies all entities entering into the agreement, outlines all roles and responsibilities of the entities, identifies all costs associated with the agreement, includes all exhibits referenced in the agreement. The draft agreement must include a document signed by both entities committing to execute the agreement as submitted following CN approval.**

A draft lease agreement is included as **Exhibit 9**. We respectfully request that the Program review the draft and detail any additional information it may need in screening.

4. **Complete the table on the following page with the estimated capital expenditure associated with this project. Capital expenditure is defined under WAC 246-310-010(10). If you have other line items not listed in the table, include the definition of the line item. Include all assumptions used to create the capital expenditure estimate.**

The capital expenditure is provided in **Table 6**.

Table 6
Proposed Capital Expenditure

Line Item	Cost
Phone/Equipment/IT	$12,820
Furniture	$9,950
Total	**$22,770**

Source: Applicant, includes sales tax

5. **Identify the entity responsible for the estimated capital costs identified above. If more than one entity is responsible, provide breakdown of percentages and amounts for each.**

Th estimated capital costs are based on actual recent experience in our affiliate agencies in King and Pierce counties. Included in **Exhibit 10** is a letter from member Nathan Yemane confirming the intent to fund these costs. This exhibit also includes a bank letter confirming the availability of funds.

6. **Identify the amount of start-up costs expected to be needed for this project. Include any assumptions that went into determining the start-up costs. Start-up costs should include any non-capital expenditure expenses incurred prior to the facility opening or initiating the proposed service. If no start-up costs are expected, explain why.**

Start up costs are included in **Table 7** below.

Table 7
Heart'n Soul Yakima County Hospice
Start-Up Costs

Item	Cost
Application Fee	$21,968
Pre-Opening Rent Expense	$8,400
Pre-Certification Staff Costs	$45,000
Office Space Security Deposit	$1,500
Legal Retainer	$5,000
Total	$81,868

7. **Identify the entity responsible for the estimated start-up costs identified above. If more than one entity is responsible, provide breakdown of percentages and amounts for each.**

Heart'n Soul estimated the startup costs identified above based on the experience of its affiliate agency in King County, and then re-reviewed based on our current start-up work in Pierce County. Included in **Exhibit 10** is a letter from member Nathan Yemane confirming the intent to fund these costs. This exhibit also includes a bank letter confirming the availability of funds.

8. **Explain how the project would or would not impact costs and charges for healthcare services in the planning area.**

We fully expect this project will reduce costs and charges for health care. Heart'n Soul will provide care to all that seek our services and qualify for hospice, regardless of their race, ethnicity, sexual orientation, payer status, ability to pay or location within Yakima County. To the extent that these populations currently use higher-cost health care services (specifically, hospitalizations and ED visits) that are reduced when they are enrolled in a hospice program, overall costs for health care services will decrease. This is especially relevant in Yakima, with its high BIPOC and Hispanic communities.

At the end of life, the BIPOC and Hispanic communities are much more likely to experience ED visits and to be hospitalized. Studies focusing on Medicare decedents and hospital utilization in the U.S have shown:

- Black and Hispanic patients were more likely to visit the ED or be hospitalized in the last 30 days of life compared to White patients.
- They are far less likely to access hospice or palliative care services early, which increases the likelihood of high intensity care near death.
- For example, one cohort study found that among Medicare beneficiaries, Black and Hispanic decedents had higher rates of multiple hospitalizations, ICU admissions, and short-term readmissions in the last month of life.

Additional data from the Kaiser Family Foundation (2023) found:

- Higher uninsured rates among the under 65 BIPOC and Hispanic communities is also associated with ER use as a default source of care.
- AI/AN adults also have shorter life expectancy, higher rates of chronic illness, and limited access to reliable outpatient care, contributing to ER and hospitalization overutilization at the end of life.

Research published in 2023 by the University of Chicago demonstrates that patients' use of hospice care contributed to $3.5 billion in savings for Medicare in 2019, while providing multiple benefits to patients, families, and caregivers.

Key findings from the study include the following:

- In the last year of life, the total costs of care to Medicare for beneficiaries who used hospice were 3.1 percent lower than for beneficiaries who did not use hospice.
- Hospice is associated with lower Medicare end-of-life expenditures when hospice lengths of stay are longer than 10 days. In other words, earlier enrollment in hospice reduces Medicare spending even further.
- Hospice stays of six months or more result in savings for Medicare. For those who spent at least six months in hospice in the last year of their lives, spending was on average 11 percent lower than the adjusted spending of beneficiaries who did not use hospice.
- At any length of stay, hospice care benefits patients, family members, and caregivers, including increased satisfaction and quality of life, improved pain control, reduced physical and emotional distress, and reduced prolonged grief and other emotional distress.

9. Explain how the costs of the project, including any construction costs, will not result in an unreasonable impact on the costs and charges for health services in the planning area.

The capital costs for the project are limited to minor equipment. These costs are not used for any rate setting purposes.

10. Provide the projected payer mix by revenue and by patients by county as well as for the entire agency using the example table below. Medicare and Medicaid managed care plans should be included within the Medicare and Medicaid lines, respectively. If "other" is a category, define what is included in "other."

Table 8 provides the requested information:

Table 8
Estimated Sources of Revenue by Payer

Payer	Percentage of Gross Revenue	Percentage by Patient
Medicare/Medicare Advantage	87.00%	86.73%
Medicaid	8.00%	8.15%
Commercial/VA/TriCare	1.50%	2.05%
Self-Pay/Private	2.00%	1.54%
Other	1.50%	1.54%
Total	**100.00%**	**100.00%**

Source: Applicant-

11. If this project proposes the addition of a county for an existing agency, provide the historical payer mix by revenue and patients for the existing agency. The table format should be consistent with the table shown above.

This question is not applicable.

12. Provide a listing of equipment proposed for this project. The list should include estimated costs for the equipment. If no equipment is required, explain.

The equipment list is provided in **Table 9.** The costs are provided in **Table 6** above.

Table 9
Equipment List

Item	Quantity
Conference Table	1
Conference Chairs	8
Employee Desk Chair	5
Employee Work Desks	4
Filing Cabinets	3
Reception Area Chair	3
Reception Area End Table	1
Phone Licenses	4
Zultys 43G Gigabit Telephone	3
Vtech Conf. Rm. Phone	1
Dell Latitude 3520 Laptop	4
Dell P2219H Monitor	4
Dell WD19 Dock	4
Kyrocera TASKalfa 2552ci Printer	1

Source: Applicant

13. Identify the source(s) of financing (loan, grant, gifts, etc.) and provide supporting documentation from the source. Examples of supporting documentation include: a letter from the applicant's CFO committing to pay for the project or draft terms from a financial institution.

Heart'n Soul will use member contributions to fund the capital expenditure, startup costs and initial operating deficits. Included in **Exhibit 10** is a letter from Nathan Yemane confirming the commitment to cover these costs as well as a bank letter confirming the availability of funds.

14. If this project will be debt financed through a financial institution, provide a repayment schedule showing interest and principal amount for each year over which the debt will be amortized.

The capital expenditure will not be debt financed; this question is not applicable.

15. Provide the most recent audited financial statements for:

- **The applicant, and**
- **Any parent entity responsible for financing the project.**

Heart'n Soul is a new entity, and no audited financial statement exists. There is no parent entity. As noted in response to Question #13, Heart'n Soul has the financial resources necessary to establish and sustain the new agency until revenues exceed expenses.

Section 3
CERTIFICATE OF NEED REVIEW CRITERIA
Structure and Process (Quality) of Care
(WAC 246-310-230)

1. **Provide a table that shows FTEs [full time equivalents] by category for the county proposed in this application. All staff categories should be defined.**

Table 10 details the projected FTEs for the initial year (6 months) and for the first three years of operation thereafter.

Table 10
Projected FTEs

Patient Care Employees	**2027**	**2028**	**2029**	**2030**
Skilled Nurse (RN)	1.00	1.85	2.43	2.86
Hospice Aide	1.55	2.88	3.78	4.45
Social Worker	1.00	1.04	1.36	1.60
Volunteer Coordinator	1.00	1.00	1.25	1.25
Chaplain	1.00	1.00	1.50	1.50
Director of Clinical Services	1.00	1.00	1.00	1.00
On Call Nurse	1.00	1.00	1.00	1.14
Admission/Triage RN	1.00	1.00	1.00	1.00
Total Patient Care FTE's	**8.55**	**10.76**	**13.33**	**14.80**
Administrative Employees	**2027**	**2028**	**2029**	**2030**
Executive Director	0.33	0.33	0.33	0.33
Office Manager	0.33	0.33	0.33	0.33
Community Education Representative/Cultural Liaison	1.00	1.00	1.00	1.00
PCS/Medical Records	1.00	1.00	1.00	1.00
Intake Coordinator	0.20	0.20	0.20	0.20
Total Administrative FTE's	**2.86**	**2.86**	**2.86**	**2.86**
Total FTE's	**11.41**	**13.62**	**16.19**	**17.66**

2. **If this application proposes the expansion of an existing agency into another county, provide an FTE table for the entire agency, including at least the most recent three full years of operation, the current year, and the first three full years of operation following project completion. There should be no gaps in years. All staff categories should be defined.**

This project does not propose the expansion of an existing agency.

3. **Provide the assumptions used to project the number and types of FTEs identified for this project.**

Heart'n Soul staffing ratios are included in **Table 11**. The number and types of projected FTEs was based on the ratio of staff to patients in Heart'n Soul's members' other hospice agency operations.

Table 11
Proposed Staff to Patient ADC Ratio

Type of Staff	FTE/Patient
Skilled Nursing (RN)	1:14
Medical Social Worker	1:25
Hospice Aide	1:9
On Call Nurse	1:35

Source: Applicant

4. **Provide a detailed explanation of why the staffing for the agency is adequate for the number of patients and visits projected.**

Heart'n Soul's proposed staffing is based on its member's historical operating experience. A review of National Hospice and Palliative Care Organization (NHPCO) data also demonstrates that staffing is consistent with the ratios and standards for community hospice agencies.

5. **Provide the name and professional license number of the current or proposed medical director. If not already disclosed under 210(1) identify if the medical director is an employee or under contract.**

Heart'n Soul's is in the process of finalizing an agreement with Dr. Josh Kretchmar, license #OP60765453. The draft agreement is included as Exhibit 8.

6. **If the medical director is/will be an employee rather than under contract, provide the medical director's job description.**

Medical Director Services will be contracted. A copy of the draft agreement is included as **Exhibit 8**.

7. **Identify key staff by name and professional license number, if known. (nurse manager, clinical director, etc.)**

This application proposes a new hospice agency. Key staff have not yet been recruited. Heart'n Soul anticipates that it will begin recruiting staff following CN approval and will have key staff in place by May of 2027. We would be glad to have our CN conditioned such that we provide this information prior to opening.

8. **For existing agencies, provide names and professional license numbers for current credentialed staff.**

Heart'n Soul is not an existing agency. This question is not applicable.

9. **Describe your methods for staff recruitment and retention. If any barriers to staff recruitment exist in the planning area, provide a detailed description of your plan to staff this project.**

Heart'n Soul's general recruitment and retention policies are included in **Exhibit 6**.

Additionally, Heart'n Soul will be guided by the U.S. Department of Health and Human Services' Culturally and Linguistically Appropriate Services (CLAS) Standards in our effort to deliver equitable and culturally competent care. The National CLAS Standards are a set of 15 action steps intended to advance health equity, improve quality, and help eliminate health care disparities by providing a blueprint for individuals and health care organizations to implement culturally and linguistically appropriate services.

We will adopt the suggestions stated in the National Standards for CLAS in Health and Health Care: A Blueprint for Advancing and Sustaining CLAS Policy and Practice (dated April 2013), which include:

- Advertise job opportunities in targeted foreign language and minority health professional associations' job boards, publications, and other media (e.g., social media networks, professional organizations' email Listservs, etc.), and post information in multiple languages.
- Develop relationships with local schools, training programs, and faith-based organizations to expand recruitment base.
- Recruit at minority health fairs.
- Collaborate with businesses, public school systems, and other stakeholders to build potential workforce capacities and recruit diverse staff. In particular, linkages between academic and service settings can help identify potential recruits already in the educational "pipeline" and provide them with additional academic support and resources necessary to meet job requirements.

- Assess the language and communication proficiency of staff to determine fluency and appropriateness for serving as interpreters.

Further we will continue to engage/support BIPOC/Hispanic organizations and post to their job boards.

10. Identify your intended hours of operation and explain how patients will have access to services outside the intended hours of operation.

Heart'n Soul's business hours will be Monday through Friday from 8:00 a.m. to 5:00 p.m., though a Hospice RN will be available 24 hours a day/7 days per week. This person will be accessible through a central phone line, will have access to the patient record, and will assist family with any concerns and help manage the patient's symptoms and facilitate any additional care needed. Response time is scheduled to be 30 minutes or less.

11. For existing agencies, clarify whether the applicant currently has a method for assessing customer satisfaction and quality improvement for the hospice agency.

Heart'n Soul is not an existing agency. However, its affiliate in King County is currently using Healthcare First, a company that provides various data support to home health and hospice agencies, to assess and report patient/family satisfaction via surveys. Heart'n Soul in Yakima County will also use Healthcare First.

12. For existing agencies, provide a listing of ancillary and support service vendors already in place.

Heart'n Soul is not an existing agency. Therefore, this question is not applicable.

13. Identify whether any of the existing ancillary or support agreements are expected to change as a result of this project.

Heart'n Soul is not an existing agency, so there are no existing ancillary or support agreements.

14. For new agencies, provide a listing of ancillary and support services that will be established.

Heart'n Soul will provide many of the services directly. We anticipate providing the following via a vendor or informal relationships:

- Inpatient Care
- PT/OT/ST/RT/IV therapy
- X-Ray
- Pharmacy
- Durable Medical Equipment
- Medical Supplies
- Laboratory
- Dietary/Nutritionist
- Ambulance
- Biowaste removal
- Specialty therapies
- Short-term respite
- Nurse Practitioner
- Freelance Interpreters

We will finalize vendor selection/informal relationships after CN approval.

15. For existing agencies, provide a listing of healthcare facilities with which the hospice agency has working relationships.

Heart'n Soul is not an existing agency. This question is not applicable.

16. Clarify whether any of the existing working relationships would change as a result of this project.

Heart'n Soul is not an existing agency. This question is not applicable.

17. For a new agency, provide a listing of healthcare facilities with which the hospice agency would establish working relationships.

Heart'n Soul proposes to work closely with local physicians, the FQHCs, hospitals, long-term care (assisted living, adult family homes and nursing homes), other providers, and most importantly directly with the BIPOC and Hispanic communities and the churches and organizations it trusts.

Other providers that Heart'n Soul intends to develop working relationships with include:

- Yakima County Area Agency on Aging.
- Home Care Association of Washington and the National Association for Home Care
- DSHS, Aging and Disability Services
- Home Health and Home Care Agencies
- Nursing Homes, Assisted Living and Adult Family Homes
- HMOs and other payers
- Washington State and Yakima County Veteran's Programs.
- Yakima County Health District
- Senior Centers
- Yakima County Housing Authority
- Yakima County Community Health Centers

In addition, because we will have a specific focus on building trust with and providing care to the underserved populations in the County, we will seek to partner with existing community resources serving these populations including but not limited to a variety of social, community organizations and places of worship, such as:

- Yakima County Hispanic Community Organizations including:
  - Together in Community
  - Unidos Nueva Alianza
  - ELLA – Leading the Way
- The Washington State Commission on Hispanic Affairs
- Latino Community fund of Washington
- Confederated Tribes and Bands of the Yakama Nation
- BIPOC and Hispanic Churches and Community Centers
- Northwest LBGTQ+ Senior Care Providers Network
- The Health Through Housing Initiative
- The Yakima Housing Authority
- Washington State Commission on African American Affairs
- Washington State Office of Equity

18. Identify whether any facility or practitioner associated with this application has a history of the actions listed below. If so, provide evidence that the proposed or existing facility can and will be operated in a manner that ensures safe and adequate care to the public and conforms to applicable federal and state requirements. WAC 246-310-230(3) and (5)

a. A criminal conviction which is related to the applicant's competency to exercise responsibility for the ownership or operation of a hospice care agency; or
b. A revocation of a license to operate a health care facility; or
c. A revocation of a license to practice a health profession; or
d. Decertification as a provider of services in the Medicare or Medicaid program because of failure to comply with applicable federal conditions of participation.

Neither Heart'n Soul nor its members or affiliates have any history with respect to the items noted.

19. Provide a discussion explaining how the proposed project will promote continuity in the provision of health care services in the planning area, and not result in an unwarranted fragmentation of services. WAC 246-310-230

According to WAC, there is a documented need for an additional provider, even assuming the current low utilization continues in Yakima County. Hospice is the "gold standard" for quality and continuity at the end of life, and our goal is to increase hospice use throughout Yakima County with emphasis on the BIPOC, Hispanic and other underserved communities.

Data on Yakima County disparities is documented in the *Project Description* and *Need Section*, above. While serving all, Heart'n Soul will focus on the reduction of disparities in access to, and use of, hospice among certain historically underserved ethnicities and races. We will do so by outreach, building trust, developing culturally appropriate services and by assuring our staff is trained and respectful of culture, values, and beliefs.

Published data including a recent 2020 article in the Journal of the American Medical Association (JAMA) found that, despite the increase in the use of hospice care in recent decades, racial disparities in the use of hospice remain.

20. Provide a discussion explaining how the proposed project will have an appropriate relationship to the service area's existing health care system as required in WAC 246-310-230.

Heart'n Soul will engage the existing health and social services systems in Yakima County to ensure patients' comprehensive medical, social, and spiritual needs are met. We intend to adopt the National Standards for Culturally and Linguistically Appropriate Services (CLAS) in Health and Health Care[6]. These standards are intended to advance health equity, improve quality, and help eliminate health care disparities by establishing a blueprint for health and health care organizations by the following:

Principal Standard:

1. Provide effective, equitable, understandable, and respectful quality care and services that are responsive to diverse cultural health beliefs and practices, preferred languages, health literacy, and other communication needs.

Governance, Leadership, and Workforce:

2. Advance and sustain organizational governance and leadership that promotes CLAS and health equity through policy, practices, and allocated resources.
3. Recruit, promote, and support culturally and linguistically diverse governance, leadership, and workforce that are responsive to the population in the service area.
4. Educate and train governance, leadership, and workforce in culturally and linguistically appropriate policies and practices on an ongoing basis.

[6] [6]U.S. Department of Health and Human Services, Office of Minority Health, Think Culture Health, https://www.thinkculturalhealth.hhs.gov/

Communication and Language Assistance:

5. Offer language assistance to individuals who have limited English proficiency and/or other communication needs, at no cost to them, to facilitate timely access to all health care and services.
6. Inform all individuals of the availability of language assistance services clearly and in their preferred language, verbally and in writing.
7. Ensure the competence of individuals providing language assistance, recognizing that the use of untrained individuals and/or minors as interpreters should be avoided.
8. Provide easy-to-understand print and multimedia materials and signage in the languages commonly used by the populations in the service area.

Engagement, Continuous Improvement, and Accountability:

9. Establish culturally and linguistically appropriate goals, policies, and management accountability, and infuse them throughout the organization's planning and operations.
10. Conduct ongoing assessments of the organization's CLAS-related activities and integrate CLAS-related measures into measurement and continuous quality improvement activities.
11. Collect and maintain accurate and reliable demographic data to monitor and evaluate the impact of CLAS on health equity and outcomes and to inform service delivery.
12. Conduct regular assessments of community health assets and needs and use the results to plan and implement services that respond to the cultural and linguistic diversity of populations in the service area.
13. Partner with the community to design, implement, and evaluate policies, practices, and services to ensure cultural and linguistic appropriateness.
14. Create conflict and grievance resolution processes that are culturally and linguistically appropriate to identify, prevent, and resolve conflicts or complaints.
15. Communicate the organization's progress in implementing and sustaining CLAS to all stakeholders, constituents, and the general public.

21. The department will complete a quality-of-care analysis using publicly available information from CMS. If any facilities or agencies owned or operated by the applicant reflect a pattern of condition-level findings, provide applicable plans of correction identifying the facility's current compliance status.

None of the agencies operated by Heart'n Soul's members have any pattern of condition level negative findings.

22. If information provided in response to the question above shows a history of condition-level findings, provide clear, cogent, and convincing evidence that the applicant can and will operate the proposed project in a manner that ensures safe and adequate care, and conforms to applicable federal and state requirements.

This question is not applicable.

Section 3

CERTIFICATE OF NEED REVIEW CRITERIA

Cost Containment (WAC 246-310-240)

Projects are evaluated based on the criteria in WAC 246-310-240 in order to identify the best available project for the planning area.

1. **Identify all alternatives considered prior to submitting this project. At a minimum include a brief discussion of this project versus no project.**

Heart'n Soul considered the following options:

- Do nothing now, and wait until there is no numeric need and apply under WAC 246-310-590 (12);
- Establish a licensed only agency; and
- Undertake the project described in this application.

2. **Provide a comparison of the project with alternatives rejected by the applicant. Include the rationale for considering this project to be superior to the rejected alternatives. Factors to consider can include but are not limited to patient access to healthcare services, capital cost, legal restrictions, staffing impacts, quality of care, and cost or operation efficiency.**

Do nothing now and wait until there is no numeric need in King County so as to allow consideration under WAC 246-310-590 (12): Taking no action now was quickly disregarded based on a number of factors, including:

1) There is numeric need for an additional agency identified in the need methodology.
2) National, state and Yakima County specific data all demonstrate the underutilization of hospice services in the County overall (as described throughout this document).
3) Our expertise in increasing outreach/community engagement/trust-building through persons known and trusted will have a real impact on overall hospice utilization as well as on underserved communities – addressing the current low Yakima County hospice utilization overall (unmet need per WAC) and the inequities within the County.

The Licensed Only Agency option was rejected because Licensed only agencies are not able to bill for Medicare and Medicaid services. The majority of hospice patients utilize Medicare and Medicaid, and this is particularly true for the most underserved (the focus of this application). Without the revenue from these programs, we would be unable to meet the needs of the most underserved and unable to sustain services.

3. **If the project involves construction, provide information that supports conformance with WAC 246-310-240(2):**
    - **The costs, scope, and methods of construction and energy conservation are reasonable; and**
    - **The project will not have an unreasonable impact on the costs and charges to the public of providing health services by other persons.**

This project does not involve construction, so this question is not applicable.

4. **Identify any aspects of the project that will involve appropriate improvements or innovations in the financing and delivery of health services which foster cost containment, and which promote quality assurance and cost effectiveness.**

Hospice care has been demonstrated to be a cost-effective service. Patients that choose to enroll in hospice forego curative treatment and opt for comfort care and symptom management, which are significantly lower cost options that produce better care for patients.

As stated earlier in this application, research published in 2023 by the University of Chicago demonstrates that patients' use of hospice care contributed to $3.5 billion in savings for Medicare in 2019, while providing multiple benefits to patients, families, and caregivers.

In terms of staffing, hospice fosters efficiency by allocating scarce RN and other resources to those most in need. For example, instead of a patient requiring a 1:1 ratio in the ICU, the patient is at home with nursing resources to provide comfort care.

Importantly, Yakima County's overall hospice utilization is significantly lower than the state or the nation and as described above, there is also significant health disparities in the County as well, resulting in additional access issues.

Specific to the need for Heart'n Soul's focus on underserved populations, the data demonstrates (described in more detail above) that the BIPOC and Hispanic populations are much more likely to experience ED visits and to be hospitalized in the last 30 days of life. They are also less likely to access hospice care early, increasing the likelihood of high intensity care near death. They are also more likely to be uninsured and to have higher rates of chronic illness and limited access to reliable outpatient care, contributing to ED and hospitalization overutilization.

The lead author of a 2020 JAMA article study, Katherine Onstein, Ph.D., M.P.H., emphasizes these disparities: *"**Despite tremendous growth in palliative care and hospice use in the United States, our work highlights a pressing need to address racial disparities in end-of-life care.**"*

The study team recommends that more sustained efforts be made to reduce disparities in end-of-life-care through efforts to better educate and train health care providers and to promote the discussion of personal values and treatment preferences for the end of life.

Heart'n Soul's community engagement and outreach programs contribute to health equity and social responsibility and improve quality care delivery.

Hospice Agency Superiority

In the event that two or more applications meet all applicable review criteria and there is not enough need projected for more than one approval, the department uses the criteria in WAC 246-310-290(11) to determine the superior proposal.

Heart'n Soul does not know if another application will be submitted. When that information is available, we will do a formal superiority analysis. Regardless, we are confident that we will compare favorably against any other applicant.

Multiple Applications in One Year

In the event you are preparing more than one application for different planning areas under the same parent company – regardless of how the proposed agencies will be operated – the department will require additional financial information to assess conformance with WAC 246-310-220. The type of financial information required from the department will depend on how you propose to operate the proposed projects. Related to this, answer the following questions:

1. **Is the applicant (defined under WAC 246-310-010(6)) submitting any other hospice applications under either of this year's concurrent review cycles? This could include the same parent corporation or group of individuals submitting under separate LLCs under their common ownership.**

   If the answer to this question is no, there is no need to complete further questions under this section.

Heart'n Soul's affiliate agency, Heart'n Soul Hospice of Kitsap County, LLC has submitted an LOI for Kitsap County, with an application due at the end of January 2026. This agency will operate as a unique legal entity and will operate under a separate license and certification.

2. **If the answer to the previous question is yes, clarify:**
   - **Are these applications being submitted under separate companies owned by the same applicant(s); or**
   - **Are these applications being submitted under a single company/applicant?**
   - **Will they be operated under some other structure? Describe in detail.**

Yes, the applications are being submitted by separate legal entities that share the same members/governors. Each agency will be operated as a unique legal entity under separate licensure and certification.

3. **Under the financial feasibility section, you should have provided a pro forma balance sheet showing the financial position of this project in the first three full calendar years of operation. Provide pro forma balance sheets for the applicant, assuming approval of this project showing the first three full calendar years of operation. In addition, provide a pro forma balance sheet for the applicant assuming approval of all proposed projects in this year's review cycles showing the first three full calendar years of operation.**

Since the Kitsap County application is not due until the end of January, the pro forma financials have not yet been finalized. With that said, we have provided in **Exhibit 7,** a draft balance sheet that assumes approval of both agencies using the current assumptions for the Kitsap agency. We can provide an updated balance sheet if the financials in are adjusted prior to submittal of the Kitsap County CN or if any changes are required to either pro forma during either of the screening periods.

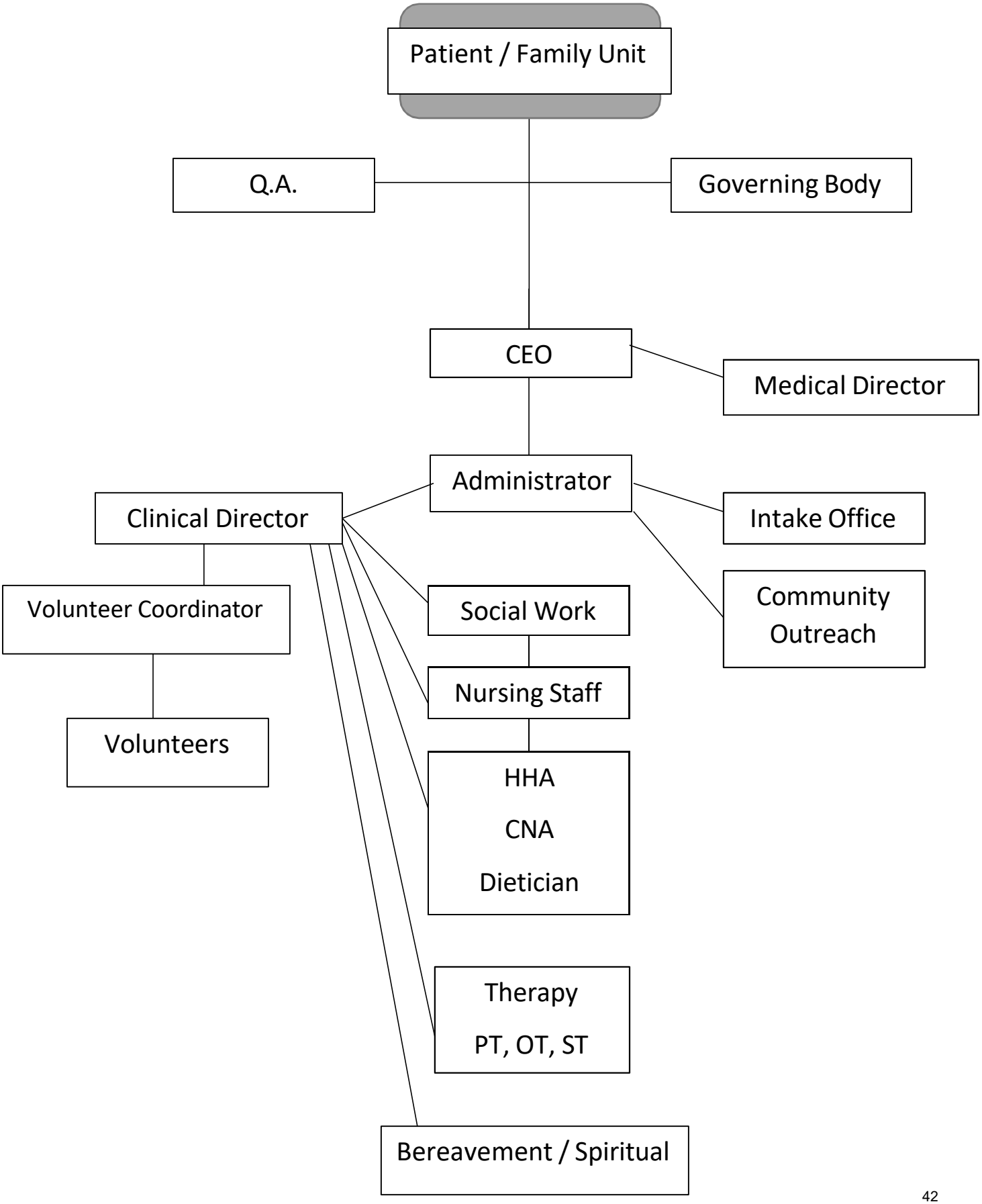
4. **In the event that the department can approve more than one county for the same applicant, further pro forma revenue and expense statements may be required.**
   • If your applications propose operating multiple counties under the same license, provide combined pro forma revenue and expense statements showing the first three full calendar years of operation assuming approval of all proposed counties.
   • If your applications propose operating multiple counties under separate licenses, there is no need to provide further pro forma revenue and expense statements.

This question is not applicable. Each agency will be operated under a separate license.

Exhibit 1 – Organizational Chart

Heart'n Soul Hospice of Yakima County

Hospice Organization Chart



Heart'n Soul Hospice

Organizational Chart

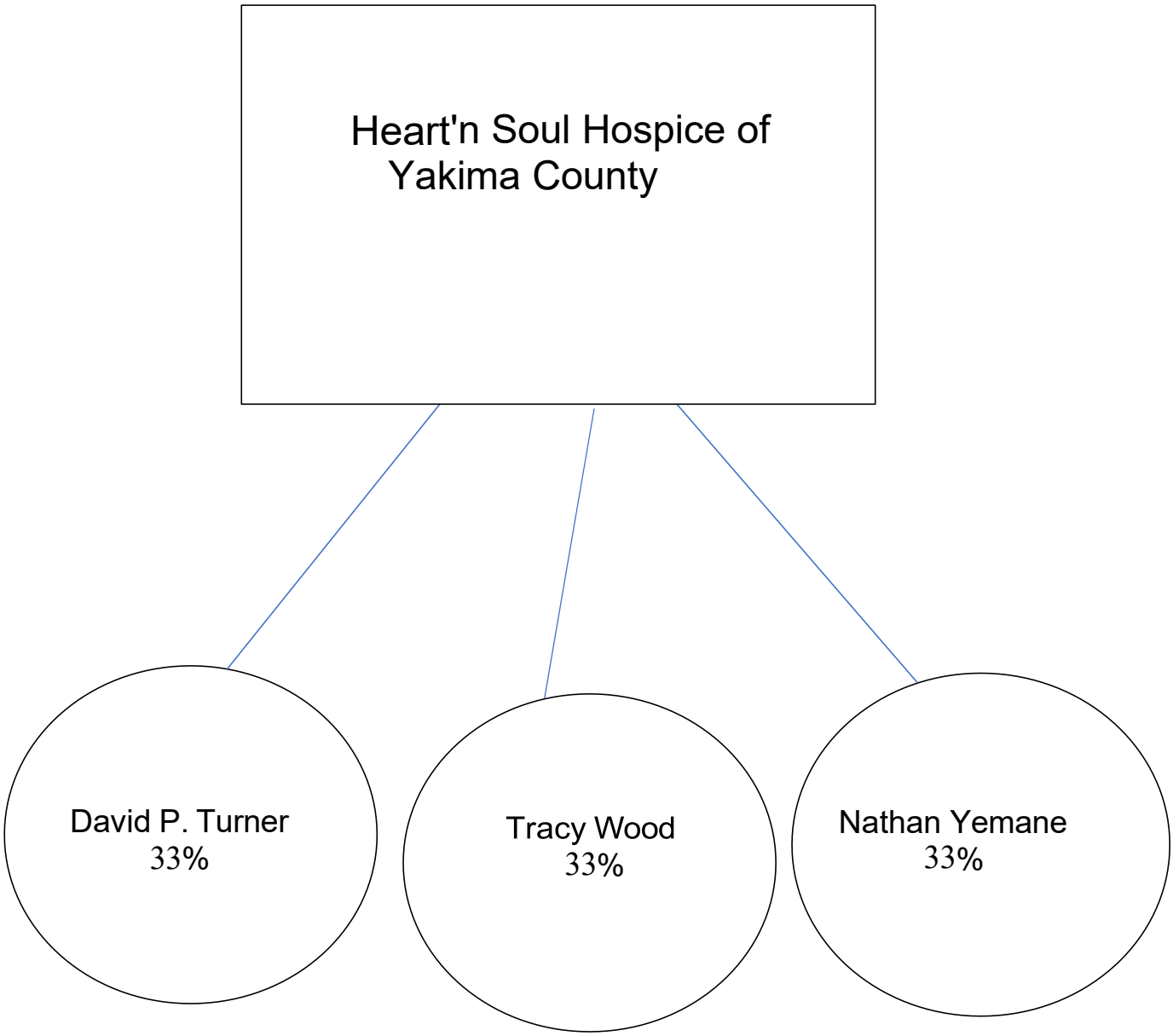


Exhibit 2 – List of Affiliate Agencies

Heart'n Soul Hospice of Yakima LLC List of Affiliate Agencies	
Heart' N Soul Hospice of Arkansas	
Address	TBD
Phone	TBD
Fax	TBD
EIN	99-487-7360
NPI	1952114001
License No	Pending AR6124
State	Arkansas
Counties	Poinsett, Mississippi
Members	David Turner and Tracy Wood
Heart' N Soul Hospice of Memphis	
Address	1255 Lynnfield St, Suite 258, Memphis, TN 38119
Phone	901-256-8921
Fax	901-881-1260
EIN	93-4767092
NPI	1699507434
License No	631
CON#	CN2403-007 A
PTAN	CHAP Accreditation process
State	Tennessee
Counties	Shleby, Tipton, Fayette
Members	David Turner and Tracy Wood
Heart and Hospice of Nashville	
Address	51 Century Blvd, Suite 100, Nashville, TN 37214
Phone	615-835-3822
Fax	615-835-3923
EIN	84-5179194
NPI	1770184079
License Number	624
PTAN	441605
State	Tennessee
Counties	Davidson, Robertson, Rutherfoord, Sumner, Williamson, Wilson
Members	David Turner and Tracy Wood

Y.B.G. Healthcare LLC dba Heart and Soul Hospice	
Address	15 S Grady Way, Renton, WA 98057
Phone	206-659-9998
Fax	425-572-6639
EIN	87-3769256
NPI	1295450922
License #	IHS.FS.61379202
PTAN	501552
Medicaid	2269278
State	Washington
Counties	King
Members	David Turner and Nathan Yemane
Heart ' N Soul Hospice of Florida	
Address	2639 N. Monroe Suite B-112, Tallahassee, FL 32303
Phone	850-201-2238
Fax	850-900-9540
EIN	99-1595782
NPI	1841022688
License Number	50370997
CON #	2014
PTAN	Pending
State	Florida
Counties	Franklin, Gadsden, Jefferson, Leon, Liberty, Madison, Taylor, Walkulla
Members	David Turner and Tracy Wood

Heart's Soul Hospice of Miami, LLC
9100 S. Dadeland Blvd, Suite 1500
Miami, FL 33156
TEL: 786-983-9918
FAX: 786-363-8300
License #50371003
EIN 99-4737387
NPI 1386431948
CON 10822
Pending CHAP survey

Heart'n Soul Hospice of Fort Myers, LLC
11934 Fairway Lakes Drive, Unit 3 Fort
Myers, FL 33913
Tel: (239) 507-9030
EIN: 99-4737387
NPI: 1700752490 CON10856

Heart'n Soul Hospice of South Carolina, LLC
4000 S. Faber Place Drive Suite 300
North Charleston SC 29405
Mobile: 843-901-0797
EIN:
License pending
No CON

Exhibit 3 – Letter of Intent

November 24, 2025

Eric Hernandez, Program Manager
Certificate of Need Program
Department of Health
111 Israel Road Southeast
Tumwater, WA 98501
Via email: eric.hernandez@doh.wa.gov; DOH HSQA CHS CON <CN@doh.wa.gov>

Dear Mr. Hernandez:

This letter serves as Heart'n Soul Hospice of Yakima County's (Heart'n Soul) letter of intent to establish a Medicare certified/Medicaid eligible hospice agency. In conformance with the requirements of WAC 246-310-080, the following information is provided:

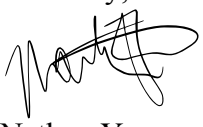
1. A Description of the Extent of Services Proposed:
Heart'n Soul proposes to establish a Medicare certified/Medicaid eligible hospice agency in Yakima County

2**. Estimated Cost of the Proposed Project:**
The capital required to establish the agency is estimated at $23,500.

3. Description of the Service Area:
The primary service area for the hospice agency will be Yakima County.

Please contact me at 206-659-9998 with any questions.

Sincerely,



Nathan Yemane, MSW, LCSW, LICSW

Exhibit 4 – DOH Hospice Methodology

Department of Health
2025-2026 Hospice Numeric Need Methodology
Distributed November 14, 2025

WAC246-310-290(8)(h) Step 8:
Determine the number of hospice agencies in the planning area that could support the unmet need with an ADC of thirty-five.

Application Year

Step 7 (Patient Days / 365) = Unmet ADC				Step 8 - Numeric Need	
County	**2025 Unmet Need ADC*†**	**2026 Unmet Need ADC***	**2027 Unmet Need ADC***	**Numeric Need?**	**Number of New Agencies Needed?****
Adams	3	3	4	FALSE	FALSE
Asotin	8	9	9	FALSE	FALSE
Benton	(14)	(9)	(4)	FALSE	FALSE
Chelan	(31)	(29)	(27)	FALSE	FALSE
Clallam	19	20	21	FALSE	FALSE
Clark	(91)	(77)	(62)	FALSE	FALSE
Columbia	1	1	1	FALSE	FALSE
Cowlitz	(16)	(13)	(10)	FALSE	FALSE
Douglas	(52)	(51)	(51)	FALSE	FALSE
Ferry	4	4	4	FALSE	FALSE
Franklin	6	8	9	FALSE	FALSE
Garfield	1	1	1	FALSE	FALSE
Grant	16	19	21	FALSE	FALSE
Grays Harbor	26	27	28	FALSE	FALSE
Island	(6)	(5)	(4)	FALSE	FALSE
Jefferson	23	24	25	FALSE	FALSE
King	(21)	20	61	***TRUE***	***1***
Kitsap	33	39	46	***TRUE***	***1***
Kittitas	6	7	8	FALSE	FALSE
Klickitat	3	4	4	FALSE	FALSE
Lewis	3	4	5	FALSE	FALSE
Lincoln	5	6	6	FALSE	FALSE
Mason	(9)	(7)	(6)	FALSE	FALSE
Okanogan	24	25	26	FALSE	FALSE
Pacific	12	13	13	FALSE	FALSE
Pend Oreille	4	4	5	FALSE	FALSE
Pierce	61	85	108	***TRUE***	***3***
San Juan	(5)	(5)	(5)	FALSE	FALSE
Skagit	(21)	(18)	(14)	FALSE	FALSE
Skamania	4	4	4	FALSE	FALSE
Snohomish	(49)	(26)	(3)	FALSE	FALSE
Spokane	(93)	(79)	(65)	FALSE	FALSE
Stevens	22	22	23	FALSE	FALSE
Thurston	1	8	14	FALSE	FALSE
Wahkiakum	2	2	2	FALSE	FALSE
Walla Walla	14	15	16	FALSE	FALSE
Whatcom	(124)	(119)	(115)	FALSE	FALSE
Whitman	26	27	27	FALSE	FALSE
Yakima	35	39	43	***TRUE***	***1***

*a negative number indicates existing hospice service capacity exceeds the projected utilization based on the statewide use rate.

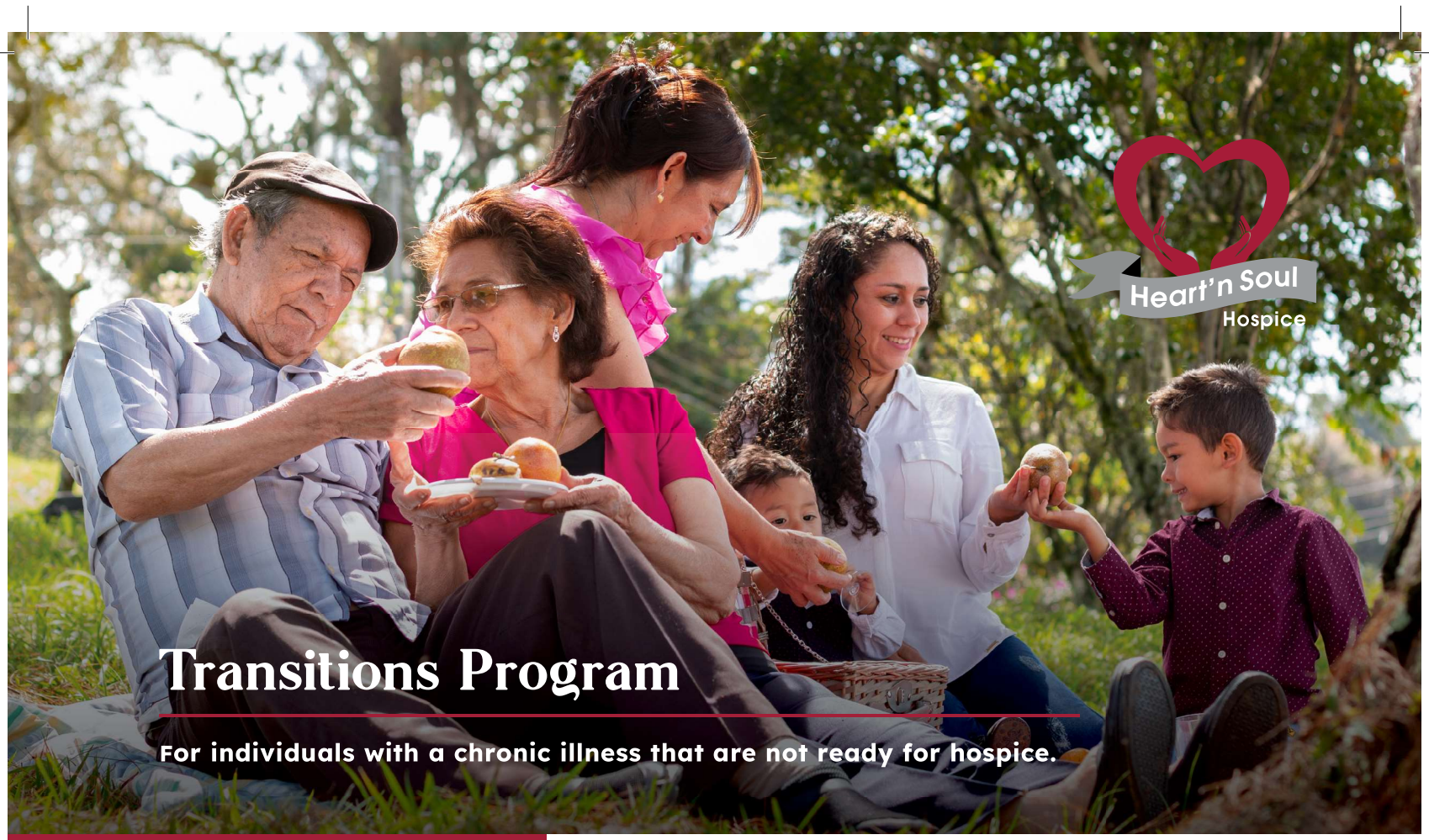
**The numeric need methodology projects need for whole hospice agencies only - not partial hospice agencies. Therefore, the results are rounded down to the nearest whole number.

Note: At the time of distribution, **Pierce County** has two hospice applications in review. An updated methodology will be distributed once these evaluations are released in November 2025.

Sources:
Self-Report Provider Utilization Surveys for Years 2022-2024
Vital Statistics Death Data for Years 2022-2024
Prepared by DOH Program Staff

Exhibit 5- Examples of Community Outreach and Engagement

- **Examples of English and Spanish language community information fliers**
- **Examples of on the ground outreach in King County**



Transitions Program

For individuals with a chronic illness that are not ready for hospice.

THE TRANSITIONS PROGRAM:

- Focuses on the caregiver to offer support groups
- No charge for the service
- Patients can continue with curative treatments
- Can be provided in the home setting, skilled nursing facility or assisted living facility

Call Us Anytime

To make a referral, please call **(615) 835-3822**. Our goal is to admit all eligible patients within 2 hours of assessment.

AT HEART N SOUL HOSPICE, THE TRANSITIONS PROGRAM OFFERS:

- Initial Case management services to identify patient and family needs.
- Identifies Community resources to enhance the quality of life
- Trained volunteers to help provide companionship, run errands, and
- Respite care for the primary caregivers.
- Support groups on many topics to help the caregiver
- Monthly check-in calls by a nurse or social worker

TRANSITIONS IS NOT A MEDICAL PROGRAM BUT A VOLUNTEER PROGRAM THAT PROVIDES EDUCATION TO THE PATIENT AND CAREGIVER.

Caminos: Transición a cuidados paliativos©

Para personas con una enfermedad crónica que no están preparadas para recibir cuidados paliativos.

EL PROGRAMA DE TRANSICIONES:

- Se centra en el cuidador para ofrecer grupos de apoyo.
- Sin costo por el servicio
- Los pacientes pueden continuar con tratamientos curativos
- Se pueden proporcionar en el domicilio, en un centro de enfermería especializada o en un centro de vida asistida.

Llámenos en cualquier momento

Para hacer una derivación, llame al:

Nashville: **615-835-3822**

Memphis: **901-256-8921**

Seattle: **206-659-9998**

Tallahassee: **850-900-9540**

EN EL HOSPICIO HEART N SOUL, EL PROGRAMA TRANSICIONES OFRECE:

- Servicios de gestión de casos iniciales para identificar las necesidades del paciente y su familia.
- Identifica recursos comunitarios para mejorar la calidad de vida.
- Voluntarios capacitados para ayudar a brindar compañía, hacer mandados y brindar atención de relevo a los cuidadores principales.
- Grupos de apoyo sobre muchos temas para ayudar al cuidador.
- Llamadas de control mensuales por parte de una enfermera o un trabajador social.

TRANSICIONES NO ES UN PROGRAMA MÉDICO SINO UN PROGRAMA DE VOLUNTARIOS QUE BRINDA EDUCACIÓN AL PACIENTE Y AL CUIDADOR.

Senior Journey©

Heart'n Soul Hospice

Heart'n Soul Hospice developed the Senior Journey© program to promote health equity and address the social determinants of health (SDOH), ensuring that all individuals receive the care and support they deserve, regardless of their background or circumstances.

The SDOH are non-medical factors that influence health outcomes. SDOH have a major impact on our health, well-being, and quality of life — and SDOH significantly affect our chances of staying healthy as we age. Examples of SDOH include:

- Income and social protection
- Education
- Food insecurity
- Housing, basic amenities, and the environment
- Social inclusion and non-discrimination
- Access to affordable, quality health services

Senior Journey© is for the elderly who may live alone, have neurological disorders and lack many of the social deterrents to have a true quality of life environment. The Senior Journey© program works to reduce health disparities in underserved and vulnerable communities, helping to ensure that every individual has the opportunity to live a healthy life.

THE GOAL OF SENIOR JOURNEY© IS:

- Improved access to health care services.
- Help reestablish connections to churches, senior centers for additional senior support.
- Better understanding between the community and the health and social service system.
- Creating an equitable and inclusive environment for the patient
- Promote an increased use of health care services.
- Promote an improved adherence to health recommendations.
- Reduced need for emergency and specialty services.

Through the Cultural Liaison Team, Senior Journey© can work with 911 dispatch and responders, police, fire, hospitals, churches and senior centers, to name a few, to offer:

1. **Bereavement support to the community.**
2. **Leverage community resources to support patients' non-medical needs.**
3. **Provide culturally appropriate health education and information.**
4. **Help people get the care they need.**
5. **Give informal counseling and guidance on health behaviors.**
6. **Advocate for individual and community health needs.**
7. **Provide some services such as blood pressure screenings throughout the year.**

For referrals, call (901) 256-8921 or fax to (901) 881-1260

Our goal is to admit all eligible patients within 2 hours of assessment.

Travecto Para Personas Mavores©

Heart and Soul desarrolló el programa Travecto Para Personas Mavores© para promover la equidad en la salud y abordar los determinantes sociales de la salud (SDOH) para garantizar que todas las personas reciban la atención y el apoyo que merecen, independientemente de sus antecedentes o circunstancias.

Travecto Para Personas Mavores© está dirigido a personas mayores que viven solas, tienen trastornos neurológicos y carecen de muchos de los determinantes sociales necesarios para tener un entorno de verdadera calidad de vida. Travecto Para Personas Mavores© ayuda a reducir las disparidades de salud en comunidades marginadas y vulnerables.

EL OBJETIVO DE TRAVECTO PARA PERSONAS MAVORES© ES:

- Mejor acceso a los servicios de atención sanitaria.
- Ayudar a restablecer conexiones con iglesias y centros para personas mayores para brindar apoyo adicional a estas personas.
- Mejor entendimiento entre la comunidad y el sistema de salud y servicios sociales.
- Crear un ambiente equitativo e inclusivo para el paciente
- Mayor uso de los servicios de atención sanitaria.
- Mejor cumplimiento de las recomendaciones sanitarias.
- Reducción de la necesidad de servicios de urgencia y especializados.

A través del Equipo de Enlace Cultural, Travecto Para Personas Mavores© puede trabajar con el personal de despacho y respuesta del 911, la policía, los bomberos, los hospitales, las iglesias y los centros para personas mayores, por nombrar algunos, para:

1. **Ofrecer servicios de interpretación y traducción.**
2. **Aprovechar los recursos de la comunidad para apoyar las necesidades no médicas de los pacientes.**
3. **Proporcionar educación e información sobre salud culturalmente apropiada.**
4. **Ayudar a las personas a obtener la atención que necesitan.**
5. **Brindar asesoramiento informal y orientación sobre conductas de salud.**
6. **Abogar por las necesidades de salud individuales y comunitarias.**
7. **Proporcionar algunos servicios directos como primeros auxilios y control de presión arterial.**

Llámenos en cualquier momento

Para hacer una derivación, llame al:

Nashville: **615-835-3822** Seattle: **206-659-9998**

Memphis: **901-256-8921** Tallahassee: **850-900-9540**

Nuestro objetivo es admitir a todos los pacientes elegibles dentro de las 2 horas posteriores a la evaluación.

WHAT IS THE HERO JOURNEY© PROGRAM?

At Heart'n Soul Hospice, we provide knowledgeable, compassionate care, emotional, and spiritual support to our Veterans and First Responders. We recognize their sacrifice and service to our country and where we live.

ACKNOWLEDGING OUR VETERANS

You served for our country and now we are here to serve you. Heart'n Soul Hospice is here for you with dedicated professionals that understand your needs and be your advocate.

ACKNOWLEDGING OUR FIRST RESPONDERS

You keep us safe! You serve the public without waiver. Heart'n Soul Hospice now serves you with end of life care that is specific for your needs.

Heart'n Soul Hospice recognizes your commitment to keeping us safe, now we are here for you!
Call us to learn more about the Hero Journey© at:
(850) 900-9540

THE PROGRAM WAS DEVELOPED TO:

- Provide emotional support through our hospice team
- Honor the veteran with pinning ceremonies
- Provide Certificate of Recognition for their outstanding service
- Provide Anticipatory grief and bereavement counseling for veteran and first responders family members/next of kin
- Provide assistance with completing funeral arrangements
- Provide volunteers matched to their service

WHV
We Honor Veterans

Heart'n Soul Hospice is a Proud Participant of the WHV program.

Doula para el programa de cuidados paliativos©

Aprende a ser un guía para las familias cuando más lo necesitan

¿QUÉ ES UNA DOULA DE FIN DE VIDA (EOL)?

Las doulas de fin de vida (EOL) son profesionales de la salud no clínicos totalmente capacitados para apoyar y guiar a las personas que enfrentan una enfermedad terminal o la muerte. Las doulas de fin de vida logran esto brindando compañía y comodidad que satisface no solo al paciente, sino también a su familia.

Desde iniciar conversaciones difíciles, pero necesarias, hasta facilitar el confort físico y la muerte digna, las doulas de fin de vida defienden a los pacientes y sus familias de varias maneras impactantes durante los cuidados paliativos. Estos esfuerzos pueden incluir:

- Educar a los pacientes y a sus familias sobre los signos y síntomas de la muerte.
- Discutir planes para el fin de la vida (es decir, directivas de atención anticipada, cuidado del cuerpo después de la muerte, celebraciones de la vida, servicios conmemorativos, arreglos funerarios)
- Proporcionar cuidados de relevo (relevo del cuidador a corto o largo plazo)
- Identificar y ayudar con necesidades de cuidado adicionales (es decir, limpiar la casa, alimentar a las mascotas, regar las plantas, mantener el césped y el jardín)
- Proporcionar despedidas tangibles (es decir, proyectos heredados, cartas, álbumes de recortes)
- Involucrar a los pacientes en sus actividades favoritas durante sus últimas horas de vida
- Atender a los pacientes y sus familias con su presencia (“Simplemente estando ahí”)

Nos complace anunciar el lanzamiento de nuestra propia capacitación de Doula de fin de vida para hospicio diseñada para defender nuestra misión de brindar atención compasiva al final de la vida a comunidades diversas y marginadas. La formación está prevista para 2025. El espacio será limitado.

¿Está interesada en convertirse en una Doula de fin de vida para cuidados paliativos y hacer que el final de la vida sea tan significativo como el comienzo? Llame a uno de nuestros coordinadores de voluntarios al:

Nashville: **615-835-3822**
Seattle: **206-659-9998**
Memphis: **901-256-8921**
Tallahassee: **850-900-9540**

Heart'n Soul Hospice

Doula para el programa de cuidados paliativos©

Aprende a ser un guía para las familias cuando más lo necesitan

¿QUÉ ES UNA DOULA DE FIN DE VIDA (EOL)?

Las doulas de fin de vida (EOL) son profesionales de la salud no clínicos totalmente capacitados para apoyar y guiar a las personas que enfrentan una enfermedad terminal o la muerte. Las doulas de fin de vida logran esto brindando compañía y comodidad que satisface no solo al paciente, sino también a su familia.

Desde iniciar conversaciones difíciles, pero necesarias, hasta facilitar el confort físico y la muerte digna, las doulas de fin de vida defienden a los pacientes y sus familias de varias maneras impactantes durante los cuidados paliativos. Estos esfuerzos pueden incluir:

- Educar a los pacientes y a sus familias sobre los signos y síntomas de la muerte.
- Discutir planes para el fin de la vida (es decir, directivas de atención anticipada, cuidado del cuerpo después de la muerte, celebraciones de la vida, servicios conmemorativos, arreglos funerarios)
- Proporcionar cuidados de relevo (relevo del cuidador a corto o largo plazo)
- Identificar y ayudar con necesidades de cuidado adicionales (es decir, limpiar la casa, alimentar a las mascotas, regar las plantas, mantener el césped y el jardín)
- Proporcionar despedidas tangibles (es decir, proyectos heredados, cartas, álbumes de recortes)
- Involucrar a los pacientes en sus actividades favoritas durante sus últimas horas de vida
- Atender a los pacientes y sus familias con su presencia (“Simplemente estando ahí”)

Nos complace anunciar el lanzamiento de nuestra propia capacitación de Doula de fin de vida para hospicio diseñada para defender nuestra misión de brindar atención compasiva al final de la vida a comunidades diversas y marginadas. La formación está prevista para 2025. El espacio será limitado.

¿Está interesada en convertirse en una Doula de fin de vida para cuidados paliativos y hacer que el final de la vida sea tan significativo como el comienzo? Llame a uno de nuestros coordinadores de voluntarios al:

Nashville: **615-835-3822**
Seattle: **206-659-9998**
Memphis: **901-256-8921**
Tallahassee: **850-900-9540**

Apoyo en el duelo©

Recursos para cuidadores y personas en duelo

APOYO ATENTO PARA QUIENES LO NECESITAN

Heart'n Soul ofrece programas de apoyo para ayudar y consolar a quienes sufren un duelo o cuidan a un ser querido gravemente enfermo. Nuestros grupos se reúnen en línea mediante videoconferencia y/o en persona. Nunca se cobra nada a nadie por participar, todos son bienvenidos. No es necesario que los participantes hayan tenido un ser querido en nuestros programas de cuidados paliativos.

REUNIONES DE GRUPO

Nuestros grupos están abiertos a cualquier persona de la comunidad que haya experimentado la muerte de un ser querido, especialmente para aquellos que han sufrido una pérdida durante el último año.

Hemos descubierto que a algunos participantes les gusta escuchar mientras que otros se sienten cómodos compartiendo pensamientos y sentimientos. Durante las sesiones, nuestros consejeros de duelo ayudan a los participantes a comprender cómo la mente y el cuerpo procesan el duelo y cómo es el duelo "normal". También pueden ayudar a los miembros del grupo a encontrar otros recursos comunitarios si es necesario.

REUNIONES DE GRUPO PARA CUIDADORES

Estos grupos se centran en identificar y comprender los sentimientos relacionados con ser el cuidador de alguien que está gravemente enfermo. Son bienvenidos a asistir padres, cónyuges, hijos o amigos de personas con enfermedades crónicas. Estos grupos se centran en identificar y comprender los sentimientos relacionados con ser el cuidador de alguien que está gravemente enfermo. Son bienvenidos a asistir padres, cónyuges, hijos o amigos de personas con enfermedades crónicas.

Llámenos en cualquier momento

No es necesario esperar a una reunión de un grupo de apoyo si tiene una necesidad inmediata de ayuda. Llame a una de nuestras sucursales de Heart'n Soul Hospice para hablar con un consejero de duelo.

Nashville: **615-835-3822**

Seattle: **206-659-9998**

Memphis: **901-256-8921**

Tallahassee: **850-900-9540**

Ofertas adicionales

DUELO POR MASCOTA

Los consejeros de duelo de Heart'n Soul Hospice entienden que la pérdida de una mascota es importante y profundamente significativa. Están aquí para hablar con miembros de la comunidad por teléfono, virtualmente o en persona.

APOYO PARA NIÑOS EN DUELO

Apoyamos a niños y familias que atraviesan experiencias de duelo y pérdida, que incluyen (entre otras) muerte, divorcio, temores pandémicos, despliegue, deportación, inseguridades del sistema familiar y más.

LLAME EN CUALQUIER MOMENTO

No es necesario esperar a una reunión de un grupo de apoyo si tiene una necesidad inmediata de ayuda. Llame a una de nuestras sucursales de Heart'n Soul Hospice para hablar con un consejero de duelo.

Para más información llame al:

Nashville: **615-835-3822**
Seattle: **206-659-9998**
Memphis: **901-256-8921**
Tallahassee: **850-900-9540**

Nuestros Grupos se reúnen en persona y/o de forma virtual, a través de herramientas como la videoconferencia Zoom®. Antes de unirse a un grupo, llámenos para que podamos ayudarlo a elegir el grupo que mejor se adapte a sus necesidades.

Un viaje lleno de significado© para pacientes con Alzheimer

Hoja de consejos para proveedores de servicios de salud

¿CUÁNDO ES EL MOMENTO DE CONSIDERAR UN CENTRO DE CUIDADOS PALIATIVOS?

Al igual que muchas otras enfermedades crónicas y debilitantes, los pacientes elegibles para cuidados paliativos con enfermedad de Alzheimer avanzada y que presentan lo siguiente son admitidos en el programa Meaningful Journey© de Heart and Soul Hospice.

Se considerará que los pacientes están en la etapa terminal de la enfermedad de Alzheimer (esperanza de vida de seis meses o menos) si cumplen los siguientes criterios:

- Estadio 7b o superior en la escala FAST
- Pérdida de peso superior a 11% en los últimos 6 meses
- Dependencia en la mayoría de las actividades de la vida diaria (AVD)
- Incapaz de hablar/comunicarse de manera inteligible (menos de 5 palabras)
- Incontinencia (intestino y vejiga)
- Incapaz de caminar sin ayuda
- Pérdida de expresión facial
- Múltiples comorbilidades que pueden perjudicar la salud del paciente, tales como, entre otras: ICC, EPOC, accidente cerebrovascular, diabetes, neoplasia maligna.
- Afecciones comórbidas: neumonía por aspiración, septicemia, úlceras por decúbito múltiples (estadio 3-4), fiebres recurrentes con terapia antibiótica posterior.
- Hospitalizaciones repetidas

Heart'n Soul Hospice

¿CÓMO PUEDE BENEFICIARSE DEL PROGRAMA MEANINGFUL JOURNEY© MI PACIENTE CON DEMENCIA EN ETAPA TERMINAL?

- **Equipo de cuidados paliativos dedicado y capacitado en los síntomas y tratamientos de la demencia para maximizar la calidad de vida de su paciente.**
- **Manejo del dolor y los síntomas para ayudar a reducir las rehospitalizaciones.**
- **Llamadas y/o visitas frecuentes del equipo de cuidados paliativos.**
- **Gestión de medicamentos y equipos**
- **Apoyo médico, emocional y espiritual 24 horas al día, 7 días a la semana.**
- **Profesionales exclusivos de guardia para los pacientes de Meaningful Journey©.**
- **Procedimientos de manejo de síntomas para guiar a los pacientes y cuidadores a controlar mejor los síntomas en el domicilio.**
- **Educación de pacientes y cuidadores**
- **Terapias complementarias que incluyen Gentle Touch, intervenciones musicales, arte y compañía de mascotas para aliviar el estrés y mejorar la calidad de vida.**

Llámenos en cualquier momento

Para hacer una derivación, llame al:

Nashville: **615-835-3822**
Memphis: **901-256-8921**
Seattle: **206-659-9998**
Tallahassee: **850-900-9540**

Nuestro objetivo es admitir a todos los pacientes elegibles dentro de las 2 horas posteriores a la evaluación.

¿QUÉ ES EL PROGRAMA Viaje al Confort©?

En Heart'n Soul Hospice, hemos desarrollado un enfoque para el cuidado del cáncer que se centra tanto en el paciente como en la familia. Consideramos a la persona en su totalidad y nuestra atención es individualizada y basada en evidencia.

Nos gustaría tener la oportunidad de asociarnos con usted y ayudarlo a brindar una atención óptima a su población de pacientes o ser querido durante su recorrido de cuidados paliativos.

Su paciente puede mantenerlo como su médico: recibirá un nivel adicional de atención administrado por nuestro equipo interdisciplinario que incluye:

- Médicos certificados por la junta en cuidados paliativos y de hospicio
- Enfermera habilitada
- Capellán
- Auxiliar de salud a domicilio
- Coordinador de voluntarios
- Asistente social
- Director médico de cuidados paliativos
- Coordinador de duelo

TODOS LOS PACIENTES CON DIAGNÓSTICO DE CÁNCER EN ETAPA TERMINAL SON ELEGIBLES. LOS BENEFICIOS PARA EL PACIENTE Y SU FAMILIA INCLUYEN:

- Reducción de visitas a urgencias
- El paciente puede permanecer en la comodidad de su hogar
- Acceso diario a nuestro equipo multidisciplinario las 24 horas, los 7 días de la semana
- Botiquín de emergencia
- Kit de confort personalizado para pacientes con cáncer

EL KIT DE CONFORT Viaje al Confort© INCLUYE:

- Cómoda manta Sherpa para el regazo
- Protector labial
- Caramelos de menta verde para aliviar la sequedad de boca y las aftas
- Loción humectante
- Bolsa de mano personalizada

Llámenos en cualquier momento

Para hacer una derivación, llame al:

Nashville: **615-835-3822**

Memphis: **901-256-8921**

Seattle: **206-659-9998**

Tallahassee: **850-900-9540**

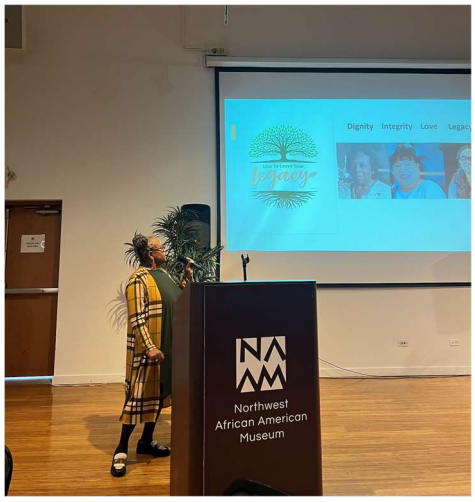
Nuestro objetivo es admitir a todos los pacientes elegibles dentro de las 2 horas posteriores a la evaluación.

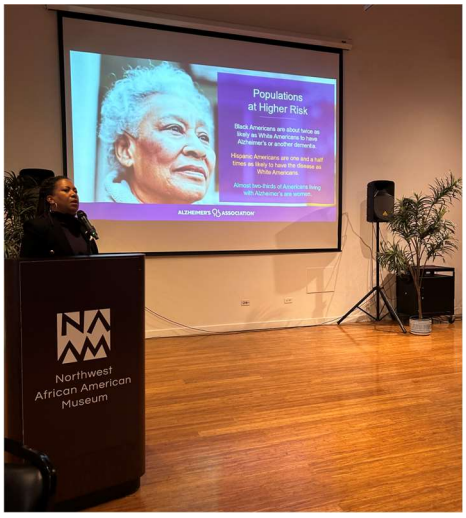
Y.B.G. Healthcare LLC dba Heart and Soul Hospice
Community Outreach King County 2023 & 2024

Where: Muslim Association of Puget Sound in Redmond, WA

When: November 2, 2024

Description: The 2024 African American Caregivers Forum was an annual event with a special focus on memory care for those impacted by memory impairment in community. Heart'n Soul Hospice was presented during National Family Caregivers Month. This community event was held today at the Northwest African American Museum. Our hospice was able to provide education and resources at the African American Caregivers Forum and for the MCAAE.





Y.B.G. Healthcare LLC dba Heart and Soul Hospice
Community Outreach King County 2023 & 2024

Where: Muslim Association of Puget Sound in Redmond, WA

When: April 17, 2024

Description: The MUSLIM ASSOCIATION OF PUGET SOUND (MAPS) is the largest Islamic center in King County. During our hospice's recent outreach, we toured the beautiful masjid and the community center with Hamzah and Abdul, staff members at MAPS. Abdul is the community center's director. We learned about all the varying services offered at MAPS during our talk with Abdul. Abdul expressed that MAPS is committed to provide community services. We were also able to leave educational information about our hospice services with Abdul and provided education about how Heart'n Soul Hospice can support the communities that MAPS serves. Hamzah was quick to point out to us that MAPS serves people of many diverse nationalities and ethnicities.

Y.B.G. Healthcare LLC dba Heart and Soul Hospice
Community Outreach King County 2023 & 2024

Where: Hosted by Ethio Edir at the Ethiopian Community Center in South Seattle

When: March 31, 2024

Description: Ethio Edir is a non-profit organization in the Seattle area, offering an insurance system for funeral costs to members of the Ethiopian community of Seattle. With over 700+ members, it is a robust organization with resources for individuals and families. Our hospice was present at their annual members meeting. Over 500+ people were present. Our hospice was provided a booth and we were able to educate individuals about where hospice care takes place, who pays for hospice, and how one qualifies for hospice care. We provided education and engagement to the attendees.

Y.B.G. Healthcare LLC dba Heart and Soul Hospice
Community Outreach King County 2023 & 2024

Where: Northwest African American Museum and Public Health Seattle King County

When: April 6, 2024

Description: Heart'n Soul Hospice appreciates the Northwest African American Museum and Public Health Community Health Services for holding a very important health fair for the community today. Our hospice was present to network, learn, and provide hospice education at this event. We spent time having meaningful visits with Dementia Friends Washington, Therapy Fund Foundation, Community Health Access Program for Public Health Seattle King County, and more. We were able to provide hospice education and how our hospices is able increase access to minority communities in King County by being present at health fairs which focus on minority population health.

Y.B.G. Healthcare LLC dba Heart and Soul Hospice
Community Outreach King County 2023 & 2024

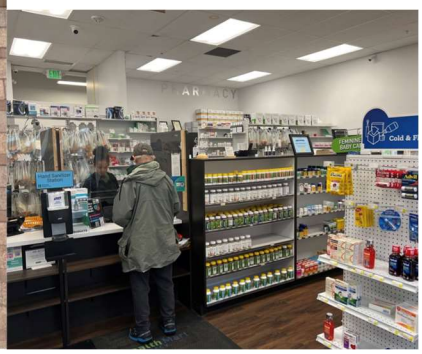
Where: Othello Station Pharmacy Seattle, WA

When: March 22, 2024

Description: Heart'n Soul Hospice had a visit with Dr. Ahmed Ali, PharmD co-owner of Othello Station Pharmacy, located in South Seattle. OSP is Heart'n Soul Hospice's primary pharmacy provider in King County. They've supported us and our patients and have provided exceptional services to us since April of 2023.

Dr. Ali is an irreplaceable staple in the Seattle community and surrounding areas. He and OSP have helped improve local access to medical services for the East African community, other BIPOC communities, and the general public. They have done this by providing quality and culturally appropriate pharmacy services, vaccines, and health education among other things.

During the visit, he remarked that hospice is very key for the community because it allows for individuals to receive end-of-life care in their home. He stated, "their options are not only limited to spending their last days in the hospital." Dr. Ali believes that individuals with a terminal illness truly have more options (not less) with hospice. Dr. Ali was gracious enough to make our hospice education materials available to the patrons of his pharmacy.



Y.B.G. Healthcare LLC dba Heart and Soul Hospice
Community Outreach King County 2023 & 2024

Where: Ethiopian Community of Seattle

When: Spring 2024

Description:
Sophia Benalfew took time to visit with Heart'n Soul Hospice and hold a rich conversation. Sophia is the Executive Director for Ethiopian Community in Seattle https://ecseattle.org, the largest Ethiopian community center in the state of Washington. She was able to have an informative dialogue about hospice care and health care for underserved communities with us. During our visit, she asked a great question about hospice. She inquired, "do they(the patients) come to you?"
We discussed that hospice care is home-based care. At Heart'n Soul Hospice, we go to our patient's residence to provide our services. Wherever our patients call home, we come to them thus removing service barriers like transportation among other things.

Y.B.G. Healthcare LLC dba Heart and Soul Hospice
Community Outreach King County 2023 & 2024

Where: Haller Lake UMC

When: Spring 2024

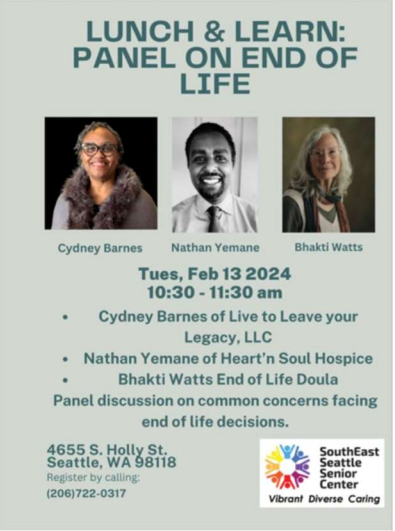
Description:
Heart'n Soul Hospice was invited to come to a faith community (https://hallerlakeumc.org) to provide hospice education. After our presentation, an individual approached us and remarked, "I never knew that hospice was covered under Medicare. I always thought hospice was charity care." We educated the community that hospice is a Medicare benefit. Many in communities, especially ethnic minority communities, do not know this. It takes the work of us in the hospice field to educate the communities that we live in about hospice care.

Y.B.G. Healthcare LLC dba Heart and Soul Hospice
Community Outreach King County 2023 & 2024

Where: SouthEast Seattle Senior Center

When: Winter 2024

Description:
Heart'n Soul Hospice appreciative of Jaime Clark and the SouthEast Seattle Senior Center https://sessc.org for hosting today's lunch and learn. SESSC consistently creates educational opportunities for the community to learn about end-of-life care, while also helping identify past and current barriers to care. Heart'n Soul Hospice was able to provide essential education about who pays for hospice and what types of care are provided through hospice at this event. Heart'n Soul Hospice was part of a panel to discuss access to hospice for minority populations and emphasized the importance of hospices being visible in the communities of minority populations.

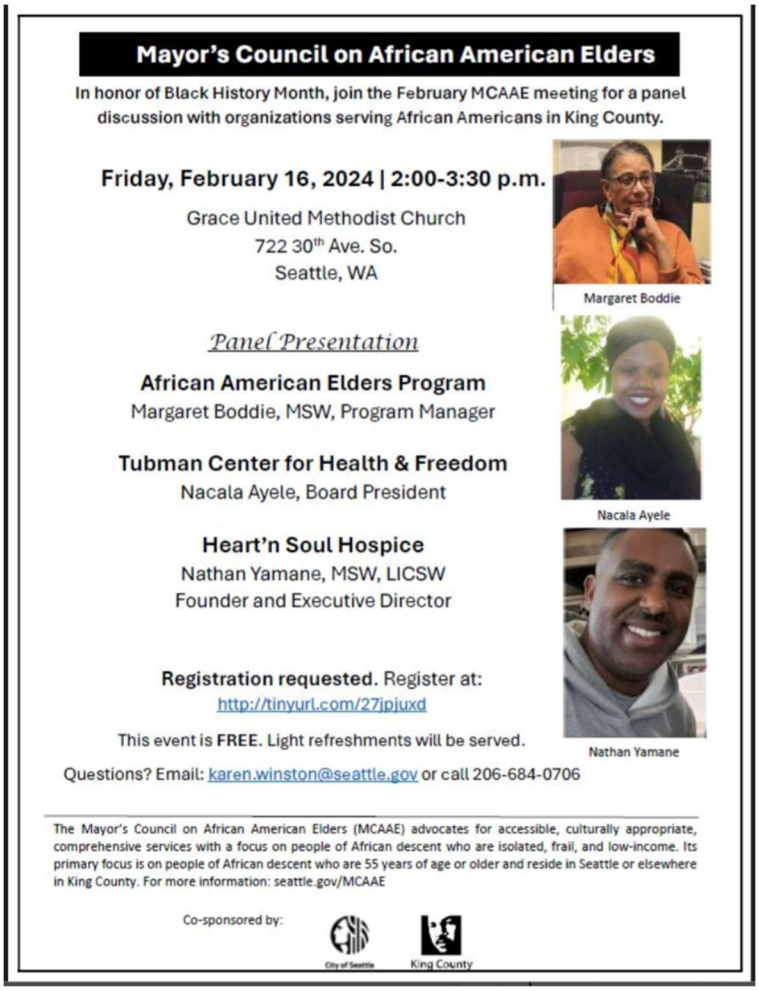


Y.B.G. Healthcare LLC dba Heart and Soul Hospice
Community Outreach King County 2023 & 2024

Where: Grace United Methodist Church/Mayor's Council on African American Elders

When: Winter (February) 2024

Description: Heart'n Soul Hospice was a part of a panel presentation, hosted by the Mayor's Council on African American Elders and a faith community. We were able to educate the attendants about how hospice care differs from some of the other health programs which target the African American community in the Seattle area. We were asked many questions during the panel, the questions came from church members and also those in the community. The most common question was, "how are you improving disparities in health?" Heart'n Soul Hospice believes that disparities are address by our hospice first being present in the communities that we serve and by providing education about the Medicare hospice benefit. It starts with being present and seen in the community regularly, not sparingly present.



Y.B.G. Healthcare LLC dba Heart and Soul Hospice
Community Outreach King County 2023 & 2024

Where: MB Daniel Mortuary and Funeral Home

When: December 8, 2023

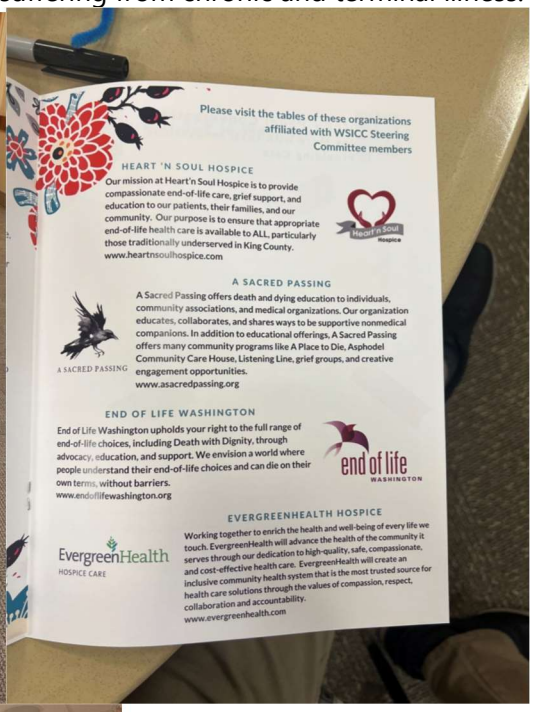
Description:
Heart'n Soul Hospice team visited and toured MB Daniel Mortuary Service's new facility in Kent (https://lnkd.in/g4k7_SDw). Rev. Micah who is the President and General Manager is a good friend and has helped our hospice by being there for our patients and their families when they've needed funeral care. MB Daniel Mortuary is the only Black-owned funeral service in the Seattle area and King County. It serves the underserved populations of the community and many in the Black/African American community contact Rev. Micah for funeral care. Our hospice has reached out to Rev. Micah multiple times because a patient's family member desperately needed guidance in making funeral plans. During our visit, we were able to provide hospice education and hospice information to Rev. Micah so that he can make it available at his funeral home for the community to have access.

Y.B.G. Healthcare LLC dba Heart and Soul Hospice
Community Outreach King County 2023 & 2024

Where: Washington State Medical Association

When: Fall 2023

Description:
The Washington State Medical Association (WSMA)'s WSICC Annual Meeting (11/17/23) was an incredible event which focused on "reckoning with serious illness in challenging times." Impactful dialogue and discussions occurred centered around issues like advance care planning, the Washington State POLST, VSED, and the Washington DWD Act. Our hospice is grateful to have participated in so many important conversations around patient self-determination, compassionate hospice care and access to care for those suffering from chronic and terminal illness.



Y.B.G. Healthcare LLC dba Heart and Soul Hospice
Community Outreach King County 2023 & 2024

Where: Northwest African American Museum

When: November 4, 2023

Description:
The African American Caregivers Forum is an annual event with a special focus on memory care for those impacted by memory impairment in community, it was presented during National Family Caregivers Month. This community event was held today at the Northwest African American Museum. Heart'n Soul Hospice was able to help sponsor the event which was organized by Winston Karen and the City of Seattle Aging and Disability Services. Our hospice was able to provide education and resources at the African American Caregivers Forum and for the MCAAE.

Y.B.G. Healthcare LLC dba Heart and Soul Hospice
Community Outreach King County 2023 & 2024

Where: East African Senior Center

When: October 4, 2023

Description:
Heart'n Soul Hospice participated in a community event for our elders from the East African diaspora community in the Seattle area. The event is hosted weekly by the East African Senior Center of Sound Generations and promotes activities like farming, gardening, health and wellness, socializing, etc. for our elders and all within a linguistically and culturally appropriate framework. Thankful to Michael Neguse for inviting Heart'n Soul Hospice and for allowing us to educate and build rapport with the East African Senior Center in Seattle.

Y.B.G. Healthcare LLC dba Heart and Soul Hospice
Community Outreach King County 2023 & 2024

Where: Martin Luther King Junior Memorial Baptist Church

When: September 16, 2023

Description:
Earlier today, our hospice was present at MARTIN LUTHER KING JR MEMORIAL BAPTIST CHURCH which hosted a health fair for our elder population in King County. Heart'n Soul Hospice collaborated with the African Americans Reach & Teach Health Ministry and Providence ElderPlace PACE , both were responsible for hosting the event. Our hospice was able to have multiple conversations with community members where I listened, learned, and provided hospice education. Thankful for MARTIN LUTHER KING JR MEMORIAL BAPTIST CHURCH for serving the community by hosting this

event!.

Y.B.G. Healthcare LLC dba Heart and Soul Hospice
Community Outreach King County 2023 & 2024

Where: KIXI Radio Station

When: Summer 2023

Description: Nathan Yemane of Heart'n Soul Hospice sat down with Shane Fowler who is the Executive Director at Holden Southcenter for a radio interview ab out the role hospice plays in the lives of individuals and how it can benefit all communities.

YOUR LIFE YOUR WAY

JULY 21, 2023

Heart'n Soul Hospice

SHARE:

Shane Fowler, host of Your Life Your Way, sits down with Nathan Yemane MSW, LICSW and the Executive Director of Heart'n Soul Hospice. Together they chat about hospice to provide compassionate end of life care, grief support, and education to our patients, their families, and our community. Website: https://heartnsoulhospice.com/ Phone: 206-659-9998

Heart'n Soul Hospice

Y.B.G. Healthcare LLC dba Heart and Soul Hospice
Community Outreach King County 2023 & 2024

Where: New Hope Missionary Baptist Church and Northwest African American Museum

When: August 19, 2023

Description:
Renewing Communities Revival was a two day community event hosted by Rev. Dr. Robert Jeffrey and Cassandra Oakes of New Hope Missionary Baptist Church in Seattle. Wonderful event that our hospice was able to participate in and support along with the Northwest African American Museum, Clean Greens Farm & Market, Black Coffee Northwest, and other community organizations. Our hospice was also an official sponsor at this event and we were able to provided hospice education and hospice resources to those in attendance.

Y.B.G. Healthcare LLC dba Heart and Soul Hospice
Community Outreach King County 2023 & 2024

Where: Starbucks Community Event in Seattle

When: July 29, 2023

Description:
Grateful that Heart'n Soul Hospice partnered yesterday with Frankie Manning, Joycelyn Thomas, and the Mary Mahoney Professional Nurses Organization along with the Center for MultiCultural Health to provide health screenings and health resources to the community at the Starbucks Partner Networks' ALL.IN. Community Event. MMPNO and the Center for MultiCultural Health do so much for and offer so much to communities in Seattle and King County.

Y.B.G. Healthcare LLC dba Heart and Soul Hospice
Community Outreach King County 2023 & 2024

Where: Northwest Kidney Center

When: Summer 2023

Description:
Thankful for Sauntia Griffin and Northwest Kidney Centers for allowing Heart'n Soul Hospice to present and speak to the NKC social work staff about equitable hospice care. Education was provided about the work that our hospice is doing and how we can support Northwest Kidney Centers patients and families.

Y.B.G. Healthcare LLC dba Heart and Soul Hospice
Community Outreach King County 2023 & 2024

Where: African American Male Wellness Agency

When: Summer 2023

Description:
Heart'n Soul Hospice took part today in the African American Male Wellness Agency's National African American Male Wellness Walk (NAAMWW) at the Renton Memorial Stadium. Our hospice had a booth where we were able to provide education about what hospice is to individuals taking part in the wellness walk. Our hospice also was able to provide educational material and network with many community organizations which focus on wellness for the African American community.

Exhibit 6 – Draft Policies & Procedures

DRAFT ADMISSION CRITERIA AND PROCESS

Policy No. 1-009

PURPOSE

To establish standards and a process by which a patient can be evaluated and accepted for admission.

POLICY

Heart and Soul Hospice will admit any patient with a life-limiting illness that meets the admission criteria.

Patients will be accepted for care without discrimination on the basis of race, color, religion, age, gender, sexual orientation, disability (mental or physical), communicable disease, place of national origin, or ability to pay.

Patients will be accepted for care based on need for hospice services. Consideration will be given to the adequacy and suitability of hospice personnel, resources to provide the required services, and a reasonable expectation that the patient's hospice care needs can be adequately met in the patient's place of residence.

While patients are accepted for services based on their hospice care needs, the patient's ability to pay for such services, whether through state or federal assistance programs, private insurance, or personal assets is a factor that will be considered.

The patient's life-limiting illness and prognosis of six (6) months or less will be determined by utilizing standard clinical prognosis criteria developed by the fiscal intermediary's Local Coverage Determinations (LCDs).

Heart and Soul Hospice reserves the right not to accept any patient who does not meet the admission criteria.

A patient will be referred to other resources if Heart and Soul Hospice cannot meet his/her needs.

Once a patient is admitted to service, the organization will be responsible for providing care and services within its financial and service capabilities, mission, and applicable law and regulations.

Admission Criteria

1. The patient must be under the care of a physician. The patient's physician (or other authorized independent practitioner) must order and approve the provision of hospice care, be willing to sign or have a representative who is willing to sign the death certificate, and be willing to discuss the patient's resuscitation status with the patient and family/caregiver.

2. The patient must identify a family member/caregiver or legal representative who agrees to be a primary support care person if and when needed. Persons without such an identified

individual and who are independent in their activities of daily living (ADLs) will require a specific plan to be developed at time of admission with the social worker.

3. The patient must have a life-limiting illness with a life expectancy of six (6) months or less, as determined by the attending physician and hospice Medical Director, utilizing standard clinical prognosis criteria developed by LCD.

4. The patient must desire hospice services, and be aware of the diagnosis and prognosis.

5. The focus of care desired must be palliative versus curative.

6. The patient and family/caregiver desire hospice care, agree to participate in the plan of care, and sign the consent form for hospice care.

7. The patient and family/caregiver agree that patient care will be provided primarily in the patient's residence, which could be his/her private home, a family member's home, a skilled nursing facility, or other living arrangements.

8. The physical facilities and equipment in the patient's home must be adequate for safe and effective care.

9. The patient must reside within the geographical area that the Heart and Soul Hospice services.

10. Eligibility for participation will not be based on the patient's race, color, religion, age, gender, sexual orientation, disability (mental or physical), communicable disease, place of national origin, or ability to pay.

11. If applicable, the patient must meet the eligibility criteria for Medicare, Medicaid, or private insurance hospice benefit reimbursement.

12. Eligibility criteria will be continually reviewed on an ongoing basis by the interdisciplinary team to assure appropriateness of hospice care.

PROCEDURE

1. The organization will utilize referral information provided by family/caregiver, health care clinicians from acute care facilities, skilled or intermediate nursing facilities, other agencies, and physician offices in the determination of eligibility for admission to the program. If the request for service is not made by the patient's physician, he/she will be consulted prior to the evaluation visit/initiation of services.

2. The Clinical Supervisor will assign hospice personnel to conduct initial assessments of eligibility for services within the time frame requested by the referral source, or based on the information regarding the patient's condition or as ordered by the physician (or other authorized independent practitioner).

3. Assignment of appropriate hospice personnel to conduct the initial assessments of patient's eligibility for admission will be based on:

   A. Patient's geographical location

B. Complexity of patient's hospice care needs/level of care required

C. Hospice personnel's education and experience

D. Hospice personnel's special training and/or competence to meet patient's needs

E. Urgency of identified need for assessment

4. In the event that the time frame for assessment cannot be met, the patient's physician and the referral source, as well as the patient, will be notified for approval of the delay.

   A. Such notification and approval will be documented.

   B. If approval is not obtained for the delay, the patient will be referred to another hospice for services.

5. A hospice registered nurse will make an initial contact prior to the patient's hospital discharge, if possible or appropriate. The initial home visit will be made within the time frame requested by the referral source and according to organization policy, or as ordered by the physician (or other authorized independent practitioner). The purpose of the initial visit will be to:

   A. Explain the hospice philosophy of palliative care with the patient and family/caregiver as unit of care.

   B. Explain the patient's rights and responsibilities and grievance procedure. (See "Bill of Rights" Policy No. 9-005.)

   C. Provide the patient with a copy of Heart and Soul Hospice notice of privacy practices.

   D. Assess the family/caregiver's ability to provide care.

   E. Evaluate physical facilities and equipment in the patient's home to determine if they are safe and effective for care in the home.

   F. Allow the patient and family/caregiver to ask questions and facilitate a decision for hospice services especially provided under the Medicare/Medicaid hospice benefit.

   G. Review appropriate forms and subsequently sign forms by patient and family/caregiver once agreement for the hospice program has been decided.

   H. Provide services as needed and ordered by physician (or other authorized independent practitioner), and incorporate additional needs into the hospice plan of care.

   I. Give patient information about durable power of attorney for health care, if the patient has not already done so.

6. During the initial assessment visit, the admitting clinician will assess the patient's eligibility for hospice services according to the admission criteria and standard prognosis criteria to determine/confirm further:

   A. Level of services required and frequency criteria

B. Eligibility (according to organization admission criteria)

C. Source of payment

7. If eligibility criteria is met the patient and family/caregiver will be provided with a hospice brochure and various educational materials providing sufficient information on:

   A. Nature and goals of care and/or service

   B. Hours during which care or service are available (physician, nursing, drugs and biological are available 24 hours/day. All other services are available to meet individual patient care needs)

   C. Access to care after hours

   D. Costs to be borne by the patient, if any, for care

   E. Hospice mission, objectives, and scope of care provided directly and those provided through contractual agreement

   F. Safety information

   G. Infection control information

   H. Emergency preparedness plans

   I. Available community resources

   J. Complaint/grievance process

   K. Advance Directives

   L. Availability of spiritual counseling in accordance with religious preference

   M. Hospice personnel to be involved in care

   N. Mechanism for notifying the patient and family/caregiver of changes in care and any related liability for payment as a result of those changes

8. The hospice registered nurse will document that the above information has been furnished to the patient and family/caregiver and any information not understood by the patient and family/caregiver.

9. The patient and family/caregiver, after review, will be given the opportunity to either accept or refuse services.

10. The patient or his/her representative will sign the required forms indicating election of hospice care and receipt of patient rights and privacy information.

11. Refusal of services will be documented in the clinical record. Notification of the Clinical Supervisor, attending physician, and referral source will be completed and documented in the clinical record.

12. The hospice registered nurse will assist the family in understanding changes in the patient's status related to the progression of an end-stage disease.

13. The hospice registered nurse will educate the family in techniques for providing care.

14. The hospice registered nurse will contact the physician for clinical information in writing to certify patient for hospice care.

15. The hospice registered nurse will complete an initial assessment during this visit within 48 hours after the election of the hospice care (unless the physician, patient or representative requests that the initial assessment be completed in less than 48 hours.) (See "Initial Assessment" Policy No. 1-013)

16. The hospice registered nurse will contact at least one (1) other member of the interdisciplinary group for input into the plan of care, prior to the delivery of care. The two (2) remaining core services must be contacted and provide input into the plan of care within two (2) days of start of care; this may be in person or by phone.

17. If the patient is accepted for hospice care, a comprehensive assessment of the patient will be performed no later than 5 calendar days after the election of hospice care. A plan of care will be developed by the attending hospice physician, the Medical Director or physician designee, and the hospice team. It will then be submitted to the attending physician for signature. The patient's wishes/desires will be considered and respected in the development of the plan of care. (See "Comprehensive Assessment" Policy No.1-014)

18. The time frames will apply for weekends and holidays, as well as weekday admissions.

19. A clinical record will be initiated for each patient admitted for hospice services.

20. If a patient does not meet the admission criteria or cannot be cared for by Heart and Soul Hospice, the Clinical Supervisor should be notified and appropriate referrals to other sources of care made on behalf of the patient.

21. The following individuals should be notified of non-admits:

    A. Patient

    B. Physician

    C. Referral source (if not physician)

22. A record of non-admits will be kept for statistical purposes, with date of referral, date of assessment, patient name, services required, physician, reason for non-admit, referral to other hospice care facilities, etc.

23. In instances where patient does not meet the stated criteria for admission to the program, exceptions will be decided upon by the Executive Director/Administrator in consultation with the Medical Director, upon request of the referring party and/or the patient.

24. In instances where continued care to a patient contradicts the recommendations of an external or internal entity performing a utilization review, the Executive Director/Administrator will be notified. All care, service, and discharge decisions must be made in response to the care required by the patient, regardless of the external or internal

organization's recommendation. The patient and family/caregiver, as appropriate, and physician will be involved in deliberations about the denial of care or conflict about care decisions.

25. A record of conflict of care issues and outcomes will be kept for statistical purposes, referencing the date of the conflict of care issue, the patient name, the external or internal organization recommendations and reasons, and complete documentation of organization decision and patient care needs.

DRAFT CHARITY CARE POLICY

Policy No. 5-017

PURPOSE

To identify the criteria to be applied when accepting patients for charity care.

POLICY

It is the policy of Heart and Soul that to provide hospice services to eligible community members without delay, regardless of their ability to pay.

Patients without third-party payer coverage and who are unable to pay for medically necessary care will be accepted for charity care admission, per established criteria.

Heart and Soul Hospice will establish objective criteria and financial screening procedures for determining eligibility for charity care.

The organization will consistently apply the charity care policy.

PROCEDURE

1. While patients are accepted for services solely based on their hospice care needs, the patient's ability to pay for services will be evaluated for state or federal assistance programs, charity care, private insurance

2. When it is identified that the patient has no source for payment of services and requires medically necessary care/service, a social worker, as available, will meet with the patient to determine potential eligibility for financial assistance from other community resources.

3. The Executive Director/Administrator, with the appropriate program director, will review all applicable patient information, including financial declarations, physician (or other authorized licensed independent practitioner) orders, initial assessment information, and social work notes to determine acceptance for charity care.

4. The full amount of hospice services will be determined to be charity care for any guarantor whose gross household income is at or below the state regulatory federal poverty guideline level provided that such persons are not eligible for other private or public health coverage sponsorship.

5. All documentation utilized in the determination for acceptance for charity care will be maintained in the patient's billing record.

6. When financial declarations reveal the patient is able to make partial payment for services, the Executive Director/Administrator, with the appropriate program director, will determine the sliding-fee schedule to be implemented.

7. The sliding-fee schedule will be presented to the patient for agreement and signature.

8. After acceptance for charity care, the patient's ability to pay will be reassessed every 60–90 days.

DRAFT NONDISCRIMINATION POLICY AND GRIEVANCE PROCESS

Policy No. 8-013

PURPOSE

To prevent organization personnel from discriminating against other personnel, patients, or other organizations on the basis of race, color, religion, age, sex (an individual's sex, gender identity, sex stereotyping, pregnancy, childbirth and related conditions), sexual orientation, disability (mental or physical), communicable disease, national origin, or ability to pay.

POLICY

In accordance with Title VI of the Civil Rights Act of 1964, Section 1157 of the Affordable Care Act (ACA) of 2010 and its implementing regulation, Heart and Soul Hospice will, directly or through contractual or other arrangement, admit and treat all persons without regard to race, color, religion, age, sex (an individual's sex, gender identity, sex stereotyping, pregnancy, childbirth and related conditions), sexual orientation, disability (mental or physical), communicable disease, national origin, or ability to pay in its provision of services and benefits, including assignments or transfers within facilities.

In accordance with Section 504 of the Rehabilitation Act of 1973, Section 1557 of the Affordable Care Act (ACA) of 2010 and its implementing regulations, Heart and Soul Hospice will not, directly or through contractual or other arrangements, discriminate on the basis of disability (mental or physical) in admissions, access, treatment or employment.

In accordance with the Age Discrimination Act of 1975, Section 1557 of the Affordable Care Act (ACA) of 2010 and its implementing regulation, Heart and Soul Hospice will not, directly or through contractual or other arrangements, discriminate on the basis of age in the provision of services unless age is a factor necessary to the normal operation or the achievement of any statutory objective.

In accordance with Title II of the Americans with Disabilities Act of 1990, Heart and Soul Hospice will not, on the basis of disability, exclude or deny a qualified individual with a disability from participation in, or benefits of, the services, programs or activities of the organization.

In accordance with other regulations, the organization will not discriminate in admissions, access, treatment, or employment on the basis of gender, sexual orientation, religion, or communicable disease.

PROCEDURE

1. The Section 504/ADA Compliance Coordinator designated to coordinate the efforts of Heart and Soul Hospice to comply with the regulations will be the Executive Director/Administrator. Contact the Executive Director/Administrator at _____ (insert telephone number.)

2. Heart and Soul Hospice will identify an organization or person in their service area who can interpret or translate for persons with limited English proficiency and who can disseminate information to and communicate with sensory impaired persons. These contacts will be listed and kept in the policy manual. (See "Facilitating Communication" Policy No. 9-006.)

3. A copy of this policy will be posted in the reception area of Heart and Soul Hospice, given to each organization staff member, and sent to each referral source.

4. The following statement will be posted in the reception of the organization in English and at least the top 15 non-English languages spoken in the state: "Patient services are provided without regard to race, color, religion, age, sex (an individual's sex, gender identity, sex stereotyping, pregnancy, childbirth and related conditions), sexual orientation, disability (mental or physical), communicable disease, national origin, or ability to pay.

5. The following statement will be printed in English and other non-English languages spoken in the state on brochures, other printed public materials and in a conspicuous location on the organization's web site accessible from the home page: "Patient services are provided without regard to race, color, religion, age, sex (an individual's sex, gender identity, sex stereotyping, pregnancy, childbirth and related conditions) , sexual orientation, disability (mental or physical), communicable disease, national origin, or ability to pay."

6. Any person who believes she or he has been subjected to discrimination or who believes he or she has witnessed discrimination, in contradiction of the policy stated above, may file a grievance under this procedure. It is against the law for Heart and Soul Hospice to retaliate against anyone who files a grievance or cooperates in the investigation of a grievance.

7. Grievances must be submitted to the Section 504 Coordinator within 30 days of the date the person filing the grievance becomes aware of the alleged discriminatory action.

8. A complaint may be filed in writing, or verbally, containing the name and address of the person filing it ("the grievant"). The complaint must state the problem or action alleged to be discriminatory and the remedy or relief sought by the grievant.

9. The Section 504 Coordinator (or her/his representative) will conduct an investigation of the complaint to determine its validity. This investigation may be informal, but it must be thorough, affording all interested persons an opportunity to submit evidence relevant to the complaint.

10. The Section 504 Coordinator will issue a written decision on the grievance no later than 30 days after its filing.

11. The grievant may appeal the decision of the Section 504 Coordinator by filing an appeal in writing to Heart and Soul Hospice within 15 days of receiving the Section 504 Coordinator's decision.

12. Heart and Soul Hospice will issue a written decision in response to the appeal no later than 30 days after its filing.

13. The Section 504 Coordinator will maintain the files and records of Heart and Soul Hospice relating to such grievances.

14. The availability and use of this grievance procedure does not preclude a person from filing a complaint of discrimination on the basis of handicap with the regional office for Civil Rights of the U.S. Department of Health and Human Services.

15. All organization personnel will be informed of this process during their orientation process.

16. Heart and Soul Hospice will make appropriate arrangements to assure that persons with disabilities can participate in or make use of this grievance process on the same basis as the nondisabled. Such arrangements may include, but will not be limited to, the providing interpreters for the deaf, providing taped cassettes of material for the blind, or assuring a barrier-free location for the proceedings. The Section 504 Coordinator will be responsible for providing such arrangements.

DRAFT BILL OF RIGHTS
Policy No. 9-005

PURPOSE

To encourage awareness of patient rights and provide guidelines to assist patients in making decisions regarding care and for active participation in care planning.

POLICY

Each patient will be an active, informed participant in his/her plan of care. To ensure this process, the patient will be empowered with certain rights and responsibilities as described. A patient, who has not been judged to lack legal capacity, may designate someone (surrogate decision maker), to act as his/her representative. This representative, on behalf of the patient, may exercise any of the rights provided by the policies and procedures established by the organization.

If the patient has been judged to lack legal capacity to make health care decisions as established by state law by a court of proper jurisdiction:

1. The rights of the patient may be exercised by the person appointed by the state court to act on the patient's behalf, OR

2. The patient may exercise his or her rights or designate a legal representative to exercise his or her rights to the extent allowed by court order.

To assist with fully understanding patient rights, all policies will be available to the organization personnel, patients, and his/her representatives as well as other organizations and the interested public.

PROCEDURE

1. The Bill of Rights statement defines the right of the patient to:

   A. Have his or her property and person treated with respect.

   B. Voice grievances regarding treatment or care that is (or fails to be) furnished, or regarding the lack of respect for property by anyone who is furnishing services on behalf of the organization and must not be subjected to discrimination or reprisal for doing so.

   C. Receive effective pain management and symptom control from the hospice for conditions related to the terminal illness.

   D. Be advised in advance of the right to participate in planning the care or service and in planning changes in the care and service; hospice patients have the right to refuse care or treatment.

E. Be involved in developing his or her hospice plan of care.

F. Refuse care or treatment.

G. Choose his or her attending physician.

H. Have a confidential clinical record maintained by the organization. Access to or release of patient information and clinical records is permitted in accordance with 45 CFR parts 160 and 164.

I. Be free from mistreatment, neglect, or verbal, mental, sexual, and physical abuse, including injuries from unknown source, and misappropriation of patient property.

J. Access to care/service is based upon nondiscrimination.

K. Have communication needs met.

L. Receive information about the services covered under the hospice benefit.

M. Receive information about the scope of services that the hospice will provide and specific limitations on those services.

N. Be advised that the Hospice Organization complies with Subpart 1 of 42 CFR 489 and receive a copy of the organization's written policies and procedures regarding advance directives, including a description of an individual's right under applicable state law and how such rights are implemented by the organization.

O. Use the hotlines to lodge complaints concerning the implementation of Advance Directive requirements.

P. Receive written information describing the organization's grievance procedure which includes the contact information, contact phone number, hours of operation, and mechanism(s) for communication problems. The program shall describe in writing patient and family responsibilities and the mechanism to file a grievance and obtain a receipt that the information has been received by the patient or family.

Q. Receive an investigation by the organization of complaints made by the patient or the patient's family or guardian regarding treatment or care that is (or fails to be) furnished, regarding the lack of respect for the patient's property by anyone furnishing services on behalf of the organization; and that the organization will document the existence of the complaint and the resolution of the complaint.

R. Receive information addressing any beneficial relationship between the organization and referring entities.

S. Be informed verbally and in writing of any changes in payment information as soon as possible, but no later than 30 days from the date that the organization becomes aware of the change.

T. Be informed, verbally and in writing, of billing and reimbursement methodologies prior to the start of care/service and as changes occur, including fees for services/products provided, direct pay responsibilities, and notification of insurance coverage.

U. Receive in writing, prior to the start of care, the telephone numbers for the State Hotline and the CHAP Hotline, including hours of operation, and the purpose of the hotlines to receive complaints or questions about the organization.

V. Be assured that the personnel who provide care are qualified through education and experience to carry out the services for which they are responsible.

2. The patient and family/caregiver responsibilities will be explained upon admission and as needed. The patient and family/caregiver are responsible for:

   A. Being fully informed by a physician of his or her medical condition, unless medically contraindicated and to be afforded the opportunity to participate in the planning of his or her medical treatment, including pain and symptom management and to refuse to participate in experimental research.

   B. Cooperating with the primary doctor, program staff and other caregivers.

   C. Advising the program of any problems or dissatisfaction with patient care.

   D. Notifying the program of address or telephone changes or when unable to keep appointments.

   E. Providing a safe environment in which care can be given. In the event that conduct occurs such that the patient's or staff's welfare or safety is threatened, service may be terminated.

   F. Obtaining medications, supplies and equipment ordered by the patient's physician if they cannot be obtained or supplied by the program.

   G. Reporting unexpected changes in the patient's condition.

   H. Understanding and accepting the consequences for outcomes if the care, services and/or treatment plan are not followed.

3. Upon admission, the admitting clinician will provide each patient or his/her representative with a written copy of the Bill of Rights.

4. The Bill of Rights statement will be explained and distributed to the patient prior to the initiation of organization services. This explanation will be in a language or communication method he/she can reasonably be expected to understand.

5. The patient will be requested to sign the Bill of Rights form. The original form will be kept in the patient's clinical record. A copy will be maintained by the patient. The patient's refusal to sign will be documented in the clinical record, including the reason for refusal.

6. The admitting clinician will document that the patient has received a copy of the Bill of Rights.

   A. If the patient is unable to understand his/her rights and responsibilities, documentation in the clinical note will be made.

   B. In the event a communication barrier exists, if possible, special devices or interpreters will be made available.

C. Written information will be provided to patients in English and predominant non-English languages of the population served.

7. When the patient's representative signs the Bill of Rights form, an explanation of that relationship must be documented and kept on file in the clinical record.

8. The family or guardian may exercise the patient's rights when a patient is incompetent or a minor.

9. All organization personnel, both clinical and non-clinical, will be oriented to the patient's rights and responsibilities prior to the end of their orientation program, as well as annually.

PURPOSE

To ensure that the hospice program maintains adequate human resources to meet caseload demands.

POLICY

The hospice program will recruit and retain qualified personnel. Documentation of these efforts will be evidenced in the hospice personnel and health record for each employee hired by the organization.

Personnel will be selected based on qualifications, education, experience, specialized training, communication and interpersonal skills, sensitivity to issues of loss and grief, and ability to deal effectively with the demand of the job, in accordance with job description requirements.

PROCEDURE

1. Professional personnel recruited and hired by Heart and Soul Hospice will be graduates of schools approved or accredited by their respective professional organizations.

2. Pre-employment interviews will be conducted and documented with a copy retained in the personnel file.

3. Prior to hire, the organization will secure multiple reference checks, health reports as required by the state or policy, criminal record checks when required by law, and proof of citizenship or documentation of resident status.

4. Professional personnel will submit copies of their diplomas or transcripts showing successful completion of the approved/accredited programs. The copies will be retained in the individual's personnel file.

5. Clinical personnel will maintain active licensure or certification. Verification of current licensure or certification will be filed in the personnel record.

6. All new employees will complete an orientation program including but not limited to:

    A. Review of policies and procedures

    B. Position responsibilities and limitations

    C. Ethics and confidentiality

    D. Communication techniques

    E. Supervision process

F. Safety

G. Infection control

H. Patient rights

I. Responding to emergencies

J. Documentation

K. Role of the health team
(See "Orientation" Policy No. 1-027.)

7. Completion of the orientation process will be documented in the personnel record.

8. Clinical competencies will be assessed during the orientation process and annually thereafter for professional and paraprofessional personnel providing direct patient care. Competency assessments will be documented and retained in the personnel file. (See "Competency Program" Policy No. 3-001 and the "Initial Competency Assessment Skills Checklists" in Section Three.)

9. All staff will receive an annual performance evaluation. (See "Performance Evaluations" Policy No. 1-026.)

DRAFT RECRUITMENT, RETENTION, DEVELOPMENT, AND CONTINUING EDUCATION

Policy No. 1-007

PURPOSE

To outline the guidelines for the planning of recruitment, retention, development, and continuing education of organization personnel.

POLICY

The Governing Body, through the Executive Director/Administrator and senior management, will provide for the needs of its patients by attracting and retaining the number and type of qualified, competent organization personnel needed to provide safe and effective care. The Governing Body accepts the responsibility and duty to provide good physical working conditions and to maintain and pursue personnel practices that are consistent with the welfare of personnel.

PROCEDURE

1. Senior management will address the issue of recruitment and retention, development and continuing education of organization personnel. Factors that will be considered include, but will not be limited to:

   A. The organization's mission

   B. The organization's strategic and business plan, including any new care/service programs and staffing needs

   C. The degree and complexity of care/service required by patients

   D. The technology used while providing care/service in the home

   E. The identified learning needs of the organization personnel, obtained through supervisory visits, questionnaires, performance improvement results, incident reports, etc.

   F. Mechanisms for recognizing the expertise and performance of organization personnel, including a formal recognition program, memos, and biannual organization-wide meetings

   G. Issues identified or stated by organization personnel that influence their decision to continue employment, through organization personnel opinion surveys, etc.

   H. The supervisor, in conjunction with the Executive Director/Administrator, will develop an overall plan for the recruitment, retention, development, and continuing education of organization personnel, both clinical and support services.

2. The plan will be evaluated as part of the organization's annual evaluation and may include such measures as:

   A. Turnover rate

   B. Percentage of unfilled positions

   C. Length of time for unfilled positions

   D. Percentage of exit interviews conducted

   E. Trends identified from exit interview information

   F. Review of compensation packages: The organization's compensation philosophy (lag, lead or match) in accordance with market data research in order to ensure fair and equitable pay practices that will assist in the attraction, retention and engagement of qualified employees.

Death with Dignity Policy - DRAFT

Heart'n Soul Hospice – Washington State Purpose

Heart'n Soul Hospice recognizes the legal right of eligible terminally ill patients in Washington State to voluntarily request and self-administer life-ending medication under the **Washington Death with Dignity Act (RCW 70.245)**. This policy provides guidance for our staff, patients, and their families regarding their rights, responsibilities, and our hospice's role in supporting patient autonomy while upholding ethical, compassionate care.

Scope

This policy applies to all Heart'n Soul Hospice employees, volunteers, and affiliated medical providers within the State of Washington.

Policy Statement

Heart"n Soul Hospice respects the rights of terminally ill patients who meet the legal criteria to request and self-administer life-ending medication under Washington's Death with Dignity Act. While our hospice participates in the prescription of life-ending medications, we cannot legally participate in the administration of life-ending medications. Heart n Soul Hospice is committed to:

• **Providing information** about the Death with Dignity Act upon patient or family request.

• **Supporting patient autonomy** by ensuring patients have access to unbiased education, resources, and referrals as needed.

• **Maintaining standard hospice care**, including pain management, symptom control, and emotional and spiritual support.

• **Respecting staff conscience rights** and ensuring that no staff member is required to participate in a Death with Dignity request if they object for personal, ethical, or religious reasons.

Definitions

1. **Death with Dignity Act** – Washington law allowing terminally ill patients with a prognosis of six months or less to voluntarily request and self-administer life-ending medication.

2. **Qualified Patient** – An adult who:

• Is a resident of Washington State.

• Has been diagnosed with a terminal illness with a prognosis of six months or less.

• Is capable of making and communicating healthcare decisions.

• Has voluntarily made the request in accordance with legal procedures.

3. **Attending Physician** – The physician primarily responsible for the care of the patient and authorized to prescribe life-ending medication under the law.

4. **Consulting Physician** – A second physician required to confirm the diagnosis, prognosis, and mental competence of the patient.

5. **Mental Health Evaluation** – An assessment that may be required if the attending or consulting physician suspects the patient has impaired judgment due to a mental health condition.

Procedures

1. Patient Inquiry & Education

• Upon request, hospice staff may provide general information about the Washington Death with Dignity Act, including state requirements and external resources.

• Staff may not provide legal advice or initiate a discussion about the option unless the patient inquires.

2. Hospice Role & Limitations

• **Heart n Soul Hospice does prescribe life-ending medication** and will ensure continuity of hospice care.

• Heart n Soul Hospice Medical Directors can be an attending or consulting physician for a hospice patient who meets the Washington State Death With Dignity Act criteria.

• Patients may choose to remain under hospice care while pursuing the Death with Dignity option through an external provider.

• If a patient self-administers life-ending medication, hospice will continue to support the family and handle post-death processes as with any natural death.

3. Staff Responsibilities & Conscience Rights

• No hospice employee is required to participate in any aspect of the Death with Dignity process if they object for personal, religious, or ethical reasons.

• Staff who object must refer patient requests to an appropriate administrator who can provide information and resources.

• Staff may not actively assist in the administration of life-ending medication.

4. Documentation & Compliance

• Any discussion regarding a patient's request for Death with Dignity must be documented in the patient's chart.

• Heart n Soul Hospice will comply with Washington State regulations, ensuring that patients seeking this option do so through legally authorized medical providers.

• If a patient self-administers medication, the hospice will document the death in accordance with standard reporting requirements.

Ethical Considerations & Bereavement Support

• Hospice will continue to offer emotional, spiritual, and bereavement support to patients and families, regardless of their decision.

• Social work, spiritual care services and counseling will be available for patients, families, and staff members who request support.

References

• Washington State Death with Dignity Act (RCW 70.245)
• Washington State Department of Health Guidance on Death with Dignity
• National Hospice and Palliative Care Organization (NHPCO) Best Practices

Exhibit 7 – Pro Forma

Pro Forma Revenue and Expense Projection
Heart'n Soul Hospice - Yakima County, Washington

	Half 2027	2028	2029	2030
	7/1/2027	1/1/2028	1/1/2029	1/1/2030
	12/31/2027	12/31/2028	12/31/2029	12/31/2030
Admissions	40	148	194	228
Patient Days	2,562	9,481	12,428	14,606
Average Daily Census	13.9	25.9	34.0	40.0
REVENUE				
Days of Care				
Routine Home Care - Tier 1	1,507	5,575	7,307	8,588
Routine Home Care - Tier 2	1,004	3,717	4,872	5,725
Total Routine Home Care	2,511	9,291	12,179	14,314
General Inpatient	18	66	87	102
Continuous Care	13	47	62	73
Inpatient Respite Care	20	76	99	117
Total	**2,562**	**9,481**	**12,428**	**14,606**
	-	-	-	-
Per Diem Rates				
Routine Home Care - Tier 1	$ 212.39	$ 212.39	$ 212.39	$ 212.39
Routine Home Care - Tier 2	$ 167.40	$ 167.40	$ 167.40	$ 167.40
General Inpatient	$ 1,103.99	$ 1,103.99	$ 1,103.99	$ 1,103.99
Continuous Care	$ 1,573.68	$ 1,573.68	$ 1,573.68	$ 1,573.68
Inpatient Respite Care	$ 489.93	$ 489.93	$ 489.93	$ 489.93
Gross Revenue by Level of Care				
Routine Home Care - Tier 1	$ 320,001	$ 1,184,004	$ 1,552,006	$ 1,824,007
Routine Home Care - Tier 2	$ 168,150	$ 622,154	$ 815,526	$ 958,454
Service Intensity Add-On	$ 73,438	$ 271,722	$ 356,176	$ 418,599
General Inpatient	$ 19,802	$ 73,268	$ 96,040	$ 112,872
Continuous Care	$ 20,162	$ 74,599	$ 97,786	$ 114,923
Inpatient Respite Care	$ 10,043	$ 37,160	$ 48,710	$ 57,247
Gross Revenue Total	**$ 611,597**	**$ 2,262,907**	**$ 2,966,244**	**$ 3,486,101**
Payer Mix				
Medicare and Medicare Managed Care	87.0%	87.0%	87.0%	87.0%
Medicaid and Medicaid Managed Care	8.0%	8.0%	8.0%	8.0%
Private Pay	1.5%	1.5%	1.5%	1.5%
Commercial	2.0%	2.0%	2.0%	2.0%
VA	1.5%	1.5%	1.5%	1.5%
Gross Revenue by Payer				
Medicare and Medicare Managed Care	$ 532,089	$ 1,968,730	$ 2,580,632	$ 3,032,908
Medicaid and Medicaid Managed Care	48,928	181,033	237,299	278,888
Private Pay	9,174	33,944	44,494	52,292
Commercial	12,232	45,258	59,325	69,722
VA	9,174	33,944	44,494	52,292
Gross Revenue	**$ 611,597**	**$ 2,262,907**	**$ 2,966,244**	**$ 3,486,101**
	-	-	-	-
Deductions from Revenue				
Bad Debt	3,058	11,315	14,831	17,431
Contractual Allowance	13,700	50,689	66,444	78,089
Charity Care	24,464	90,516	118,650	139,444
Total Net Revenue	570,375	2,110,388	2,766,319	3,251,138

	2027-Half	2028	2029	2030
PATIENT CARE COSTS				
Patient Care Salaries/Wages	390,569	960,624	1,158,166	1,281,618
Payroll Taxes & Benefits	70,302	172,912	208,470	230,691
Pharmacy	17,168	63,522	83,265	97,858
Durable Medical Equipment	22,805	84,380	110,606	129,991
Medical Supplies	11,582	42,854	56,173	66,018
Other Direct Costs (Med. Services, Lab fees, etc.)	3,075	11,377	14,913	17,527
Nurse Practitioner - Contracted	2,562	9,481	12,428	14,606
Dietitian	256	948	1,243	1,461
Physical Therapist	512	1,896	2,486	2,921
Speech Therapist	256	948	1,243	1,461
Occupational Therapist	512	1,896	2,486	2,921
Physician Face-to-Face Visit	641	2,370	3,107	3,651
General Inpatient Costs	16,832	62,278	81,634	95,941
Inpatient Respite Costs	5,524	20,438	26,790	31,486
5% Room & Board	5,894	21,806	28,584	33,593
Mileage Reimbursement	20,884	77,269	101,285	119,036
Total Patient Care Costs	569,375	1,534,999	1,892,878	2,130,779
Gross Margin	**1,000**	**575,389**	**873,441**	**1,120,358**
ADMINISTRATIVE & FACILITY COSTS				
Administrative Salaries/Wages	120,911	241,821	241,821	241,821
Payroll Taxes & Benefits	21,764	43,528	43,528	43,528
Medical Director	17,937	66,366	86,993	102,240
Advertising & Marketing	11,531	42,664	55,924	65,726
Cultural Competency Marketing	256	948	1,243	1,461
Rent	8,400	8,652	8,912	9,179
Utilities	10,800	21,600	21,600	21,600
Dues & Subscriptions	1,051	3,887	5,095	5,988
Office Expense	6,406	23,702	31,069	36,514
Property & Liability Insurance	6,406	23,702	31,069	36,514
License Fee	596	1,192	1,192	1,192
Legal & Professional	1,153	4,266	5,592	6,573
Banking Service Fees	179	664	870	1,022
Travel & Entertainment	384	1,422	1,864	2,191
Auto Expense	256	948	1,243	1,461
Miscellaneous Expense	512	1,896	2,486	2,921
Total Administrative Costs	208,542	487,259	540,501	579,930
Contribution Margin	**(207,543)**	**88,130**	**332,940**	**540,429**
Billing Fee	36,696	135,774	177,975	209,166
EBITDA	**(244,238)**	**(47,644)**	**154,966**	**331,262**
Depreciation	1,873	3,745	3,745	3,745
Washington State B&O Taxes	8,556	31,656	41,495	48,767
Net Income (Loss)	**(254,667)**	**(83,046)**	**109,725**	**278,750**

Balance Sheet Projection
Heart'n Soul Hospice - Yakima County, Washington

	Year Ended December 31				
	2027 - Pre-Opening	**2027**	**2028**	**2029**	**2030**
ASSETS					
Current Assets					
Cash & Cash Equivalents	$ 285,465	$ 23,258	$ (4,314)	$ 93,878	$ 363,985
Accounts Receivable, (net)		95,063	175,866	230,527	270,928
Prepaid Expenses	8,267	8,267	8,267	8,267	8,267
Total Current Assets	293,732	126,587	179,818	332,672	643,179
Property & Equipment					
Fixed Assets	-	22,770	22,770	22,770	22,770
Accumulated Depreciation	-	(1,873)	(5,618)	(9,364)	(13,109)
Total Property & Equipment	-	20,897	17,152	13,406	9,661
Other Assets					
Intangibles					
Loan Fees					
Accumulated Amortization					
Other Assets	-	-	-	-	-
TOTAL ASSETS	293,732	147,485	196,970	346,078	652,840
LIABILITIES					
Current Liabilities					
Accounts Payable & Accrued Expenses		58,124	100,562	130,232	152,175
Accrued Payroll & Related Payables		50,296	59,120	68,833	74,902
Notes Payable					
Current Portion LT Debt					
Total Current Liabilities	**-**	**108,419**	**159,682**	**199,065**	**227,077**
Long-Term Liabilities					
Long-Term Note Payable					
Less: Current Portion of LTD					
Total Long-Term Liabilities	**-**	**-**	**-**	**-**	**-**
TOTAL LIABILITIES	-	108,419	159,682	199,065	227,077
CAPITAL					
Capital	375,000	375,000	375,000	375,000	375,000
Retained Earnings (Losses)	(81,268)	(335,935)	(337,712)	(227,987)	50,763
Total Capital	**293,732**	**39,065**	**37,288**	**147,013**	**425,763**
TOTAL LIABILITIES & Capital	293,732	147,485	196,970	346,078	652,840

Heart'n Soul Hospice of Yakima County
Financial Assumptions

Revenue Rates:

Level of Care	2025 Medicare Rates	2025 Medicaid Rates
1-60 days	212.39	212.39
61+	167.40	167.40
Cont HC (per hour)	65.57	65.57
Respite	489.93	489.93
GIP	1,103.99	1,103.99
R&B Rate	374.40	
Service Intensity Add On	65.57	

*** R&B Rate is based on the Average Medicaid Rate for Yakima County SNF Rates**
*** SIA Amounts in Financials are assumed to be 28 hours per patient**
*** Forecast assumes 2026 rates will continue through the forecast period**

Level of Care

Level of Care	Percent of Total Days
General Inpatient Care	0.70%
Inpatient Respite Care	0.80%
Routine Home Care	98.00%
Continuous Home Care	0.50%

Please note that Medicare and other insurances pay per diem (daily rate or based on 1 calendar day) for the levels of care. Heart and Soul used Excel to round the level of care days to full days, which may vary slightly from the Total Days when calculated from Average Length of Stay times the Average Daily Census due to that full-day rounding.

Line Item	Assumption
Rounding	For reporting of numbers, Heart and Soul rounded to the nearest dollar using Excel. So when manually calculating, there may be immaterial differences in the results due to this rounding. The pro forma financials have been hand-keyed. As such, there are some rounding in both census (patient days and ADC) as well as the estimated dollars.
Contractual Adjustments	Approximately 0.5% of total gross revenue.
Charity Care	Approximately 4.0% of total gross revenue.
Bad Debt	Approximately 0.5% of total gross revenue.

Line Item	Assumption
Salaries and Benefits	Based on FTE and staffing, benefits are assumed to be 18% of salaries.
Contracted Services	For PT/ST/OT/RT/Nurse Practitioner/dietician/physician, assumed to be at the following rates per patient day (PPD): Nurse Practitioner - $1.00/PPD Dietitian/Nutritional Counseling - $0.10/PPD Physical Therapist (PT) - $0.20/PPD Speech/Respiratory Therapist (ST) - $0.10/PPD Occupational Therapist (OT) - $0.20/PPD Physician Face-to-Face Visit - $0.25/PPD
Pharmacy	Assumed to be $6.70/PPD
DME	Assumed to be $8.90/PPD
Medical Supplies	Assumed to be $4.52/PPD
Other Direct Expenses	Assumed to be $1.20/PPD
General Inpatient Costs	Assumed expense at 85% of the GIP revenue.
Inpatient Respite Costs	Assumed expense at 55% of the inpatient respite revenue.
5% room and board expense for Medicaid patients in nursing homes receiving routine care	Assumed to be $2.30/PPD
Mileage	Assumed to be $8.15/PPD

Line Item	Assumption
Administrative & Facility Costs	Non direct clinical staff including but not limited to Office Manager, Executive Director, Community Education Representative, PCS/Medical Records, and Intake Coordinator. Benefits are estimated at 18% of salary.
Contracted Medical Director	Assumed to be $7.00/PPD
Advertising and Marketing	Assumed to be $4.50/PPD
Cultural Competency Marketing	Assumed to be $0.10/PPD
Rent	Rent is based on the lease agreement in Exhibit 9.
Utilities	Assumed to be $1,800 per month – includes phone, internet, water, electricity, etc.
Dues/Subscriptions	Assumed to be $0.41/PPD
Office Expense	Assumed to be $2.50/PPD
Property and Liability Insurance	Assumed to be $2.50/PPD
License fee	Assumed to be $2,383.00 per month based on the size of the practice and the license fee renewal schedule
Legal and Professional Services	Assumed to be $0.45/PPD
Banking Service Fees	Assumed to be $0.07/PPD

Line Item	Assumption
Travel and Entertainment	Assumed to be $0.15/PPD
Auto Expense	Assumed to be $0.15/PPD
Miscellaneous Expense	Assumed to be $0.20/PPD
Billing Fee	Assumed to be 6 percent of gross patient revenue
Depreciation	Depreciation of capitalized furniture, office equipment, phones, computers, and printer over estimated useful lives ranging from five years to seven years.
Washington State B&O Taxes	Assumed to be 1.5 percent of net patient revenue.

Balance Sheet Projection
Heart and Soul Hospice - Kitsap and Yakima Counties, Washington

	Year Ended December 31				
	2027 - Pre-Opening	**2027**	**2028**	**2029**	**2030**
ASSETS					
Current Assets					
Cash & Cash Equivalents	$ 569,731	$ 184,841	$ 779,408	$ 1,770,957	$ 3,343,402
Accounts Receivable, (net)	-	222,288	419,995	537,217	649,665
Prepaid Expenses	16,533	16,533	16,533	16,533	16,533
Total Current Assets	586,264	423,663	1,215,936	2,324,707	4,009,601
Property & Equipment					
Fixed Assets	-	45,540	45,540	45,540	45,540
Accumulated Depreciation	-	(3,745)	(11,236)	(18,727)	(26,218)
Total Property & Equipment	-	41,795	34,304	26,813	19,322
Other Assets					
Intangibles	-	-	-	-	-
Loan Fees	-	-	-	-	-
Accumulated Amortization	-	-	-	-	-
Other Assets	-	-	-	-	-
TOTAL ASSETS	**586,264**	**465,457**	**1,250,240**	**2,351,520**	**4,028,923**
LIABILITIES					
Current Liabilities					
Accounts Payable & Accrued Expenses	-	119,113	210,045	266,361	319,087
Accrued Payroll & Related Payables	-	100,614	119,200	137,623	151,877
Notes Payable	-	-	-	-	-
Current Portion LT Debt	-	-	-	-	-
Total Current Liabilities	-	219,727	329,245	403,984	470,963
Long-Term Liabilities					
Long-Term Note Payable					
Less: Current Portion of LTD					
Total Long-Term Liabilities	**-**	**-**	**-**	**-**	**-**
TOTAL LIABILITIES	-	219,727	329,245	403,984	470,963
CAPITAL					
Capital	750,000	750,000	750,000	750,000	750,000
Retained Earnings (Losses)	(163,736)	(504,270)	170,994	1,197,535	2,807,959
Total Capital	**586,264**	**245,730**	**920,994**	**1,947,535**	**3,557,959**
TOTAL LIABILITIES & Capital	**586,264**	**465,457**	**1,250,240**	**2,351,520**	**4,028,923**

Exhibit 8 – Draft Medical Director Agreement

MEDICAL DIRECTOR SERVICES AGREEMENT

This Medical Director Services Agreement (the "Agreement") is made and entered into this _29th__ day of ____December_____, 2025 ("Effective Date"), by and between Heart'n Soul Hospice of Yakima, LLC ("Hospice"),and _____Josh Kretchmar DO_____, ("Physician"), an individual.

RECITALS

Hospice is licensed to operate a hospice program in the State of Washington and is certified to participate in the Medicare and Medicaid programs.

Physician is a duly licensed to practice medicine without restriction in the State of Washington.

Hospice wishes to engage Physician to assume responsibility for the medical services component of the Hospice care program in accordance with state and federal law.

Physician wishes to provide such services to Hospice in accordance with the terms and conditions of this Agreement.

NOW THEREFORE, in consideration of the mutual covenants and agreements of the parties contained herein, the parties agree as follows:

ARTICLE 1

SERVICES AND GENERAL DUTIES OF PHYSICIAN

1.1 Services- Generally. Medical Director agrees to provide Administrative Services and Clinical Services (together, the "Services"). Physician shall provide Services to Hospice and those Medicare-eligible and certified individuals that have elected to receive palliative care from Hospice ("Hospice Patients").

1.2 Services- Administrative. Physician shall be responsible for the overall supervision of all Hospice Patient care, including, but not limited to the following:

1.2.1 Eligibility and Certification of Terminal Illness. Physician shall assess patient eligibility for hospice care and assist in the development of admission criteria. Physician shall review, certify, and recertify Hospice Patient eligibility for Hospice services, in accordance with Hospice polices and any state of federal guidelines. Any certification or recertification of a Hospice Patient must be documented in writing within forty-eight (48) hours.

1.2.2 Interdisciplinary Team. Physician shall be a member of the inter-disciplinary team ("IDT") and will attend IDT meetings. As a member of the IDT, Physician shall participate in and direct decisions regarding the Hospice Patient's Plan of Care ("Plan of Care"), as established by the Hospice Patient's attending physician and the IDT.

1.2.3 Hospice Protocols and Policies. Physician shall participate in the development of medical protocols and review the medical policies of Hospice annually and provide any recommended changes to Hospice's governing body. Physician shall take appropriate steps to ensure care provided to Hospice Patients meets or exceeds the appropriate medical standard of care.

1.2.4 Performance Improvement Committee. Physician shall serve on the Hospice's Performance Improvement Committee and assist the Hospice Executive Director in implementing committee recommendations.

1.2.5 Education. Physician shall promote the education of other physicians in the community regarding Hospice services. Physician shall receive copies of, and be familiar with, Hospice policies and standards related to emergency procedures, communication with Hospice staff, grievance procedures, ethical concerns, confidentiality, patient/caregiver rights, and safety and infection control.

1.3 Services-Clinical

1.3.1 Hospice Patient Care. Physician shall provide medical care to Hospice Patients according to the Plan of Care and in the event the attending physician is unable to respond to the care needs of a Hospice Patient, including signing physician orders.

1.4 Coverage Requirements. Medical Director shall be available to provide Services twenty-four (24) hours a day in the case of an emergency, and otherwise as needed to provide advice, service, and consultation in person, by telephone, and through written reports and other documentation. During any absence, vacation, or other periods when Physician is unavailable, Physician shall arrange for a qualified physician, acceptable to Hospice, to perform the obligations of this Agreement.

1.5 Reports and Records.

1.5.1 Medical Records. Physician shall use Hospice medical records to promptly and accurately record all physician orders and Clinical Services. Physician shall provide a signed medical record entry at the time Clinical Services are provided to a Hospice Patient.

1.5.2 Documentation. Physician will prepare all documentation under this Agreement in accordance with sound medical practice, state and federal laws, rules, and regulations, Hospice policies, and other reasonable requirements of Hospice. All documentation shall be retained by the respective parties for a period of seven (7) years. All records remain the property of Hospice.

1.6 HIPAA and Confidentiality

1.6.1 HIPAA Compliance. Physician agrees to comply with the federal laws, rules, and regulations relating to the confidentiality of individually identifiable health information implementing the Health Insurance Portability and Accountability Act of 1996 (42 U.S.C. §1320(d)) ("HIPAA"), and shall protect from unlawful disclosure all medical records and

other protected health information ("PHI") disclosed to or created by Physician in the course of performing the Services.

1.6.2 Reporting Relationship. Physician will report directly to the Hospice Executive Director and will not have any authority to hire, fire, or discipline Hospice employees.

1.6.3 Confidential Hospice Information. Physician shall keep this Agreement and its contents confidential and not disclose to anyone other than legal and financial advisors, or otherwise as required by law, without the prior written consent of Hospice. Physician will have access to other information regarding Hospice operations and management, including, but not limited to, management reports, financial statements, internal memoranda, reports, confidential technology and other materials, records and/or information of a proprietary nature ("Confidential Information"). Physician agrees to not use Confidential Information except in connection with the performance of Services, or to divulge Confidential Information to any third party, unless Hospice consents in writing to such use, or if disclosure is required by law. If Physician receives a demand for disclosure of Confidential Information, Physician shall notify Hospice with two (2) days of receipt. Physician's duty to not disclose Confidential Information shall survive termination of this Agreement.

ARTICLE 2

COMPENSATION

2.1 Compensation. Hospice agrees to pay Physician as follows:

2.1.1 For Administrative Services. Physician shall be compensated $__7__ per patient day. Payment is conditioned on Physician compliance with this Agreement and Physician providing Hospice accurate and complete documentation of Administrative Services.

2.1.2 Clinical Services. When Physician provides direct personal care to a Hospice Patient, Physician shall submit accurate and complete bills, together with appropriate documentation, within fifteen (15) days after the end of the month in which Clinical Services were provided. Hospice shall compensate Physician for Clinical Services at 90% of the Medicare Allowable Rate, such amount shall be considered by Physician to be payment in full.

2.1.3 Mileage. Hospice shall pay Physician $ __.42__ per mile per visit to Hospice Patient.

ARTICLE 3
DUTIES AND OBLIGATIONS OF HOSPICE

3.1 Acceptance and Discharge of Clients. Hospice shall have authority to accept Hospice Patients for treatment and to assess their needs in compliance with Hospice admission policies. Hospice shall be in compliance with all state, federal and accrediting regulations. Hospice shall have authority to discharge Hospice Patients. Hospice shall also be responsible for identifying the need for a change in the level of a Client's care, for approving transfers based on established criteria, for identifying a need for additional services, and discharge as appropriate.

3.2 Plan of Care. A written Plan of Care shall be established and maintained for each Hospice Patient. The plan shall be established by the attending physician, the Physician (or designee), and the IDT prior to providing care. The plan shall be reviewed and updated, at intervals specified in the Plan of Care, by the attending physician, Physician (or physician designee), and IDT. These reviews shall be documented. The plan shall include an assessment of the Hospice Patient's needs and identification of services, including the management of discomfort and symptom relief. The plan shall state in detail the scope and frequency of services needed to meet the Hospice Patient's and caregiver's needs.

3.3 Coordination of Services. The IDT, Physician, and attending physician shall be responsible for developing, reviewing, revising, and evaluating each Plan of Care and assuring continuity between all involved agencies and disciplines. Hospice shall ensure that services are furnished in a safe and effective manner by qualified persons and in accordance with the Plan of Care.

3.4 Policies and Procedures. Hospice shall provide Physician with a copy of its applicable policies and procedures upon execution of this Agreement. Hospice reserves the right to alter or amend such policies and procedures from time to time, at the discretion of Hospice, and shall notify Physician of any such change(s). Hospice shall provide Physician with training and orientation to its standards of practice.

ARTICLE 4
TERM AND TERMINATION

4.1 Term. The term of this Agreement shall be for one (1) year, commencing on the Effective Date of this Agreement ("Initial Term"), unless earlier terminated as provided in this Agreement. Thereafter, this Agreement shall automatically renew for successive terms of one (1) year each (each a "Renewal Term") unless either party gives written notice of non-renewal to the other party at least sixty (60) days prior to the expiration of the then-current term. In the event this Agreement is terminated during the Initial Term, the parties shall not enter into another substantially similar agreement with one another until the one-year anniversary of the effective date of this Agreement.

4.2 Termination. This Agreement may be terminated prior to the end of a Term as follows

4.2.1 Without Cause. Either party may terminate this Agreement without cause by providing at least sixty (60) days prior written notice to the other party.

4.2.2 Breach. Either party shall have the right to terminate this Agreement in the event of the other party's material breach of this Agreement by providing at least thirty (30) days written notice to the other party. Any such notice shall specify the material cause of termination. The violating party shall have the thirty (30) day notice period in which to rectify the cause specified in the notice of termination, or, if such cause is not rectified to the satisfaction of the non-breaching party within such thirty (30) day period, the non-breaching party may immediately terminate the Agreement.

4.2.3 Material Change. To the extent that a material change in laws, rules, regulations, or the method or amount of reimbursement requires the restructuring of the relationship between the parties established by this Agreement, the parties shall negotiate in good faith to amend this Agreement and otherwise restructure their relationship in order to effectuate their mutually agreed upon purposes. If the parties are unable to resolve the matter within thirty (30) days, either party may, at its option, immediately terminate this Agreement.

4.2.4 Immediate Termination. Hospice may immediately terminate this Agreement upon the occurrence of any of the following events: (i) loss or suspension of any license of Physician required for the provision of Services pursuant to this Agreement or the imposition of any sanction against Physician under federal or state fraud and abuse laws and regulations or any other federal or state laws or regulations relating to Physician's participation in the Medicare or state Medicaid programs; (iii) violations of HIPAA Privacy or Security laws, rules, or regulations, or (iv) any jeopardy to the health or safety of Hospice patients.

4.2.5 Automatic Termination. This Agreement shall automatically terminate in the event either party is excluded from participation in any federally funded health care program, including Medicare or Medicaid, as of the effective date of such exclusion.

ARTICLE 5

REPRESENTATIONS AND WARRANTIES OF PHYSICIAN

5.1 Licenses and Certifications**.** Physician represents and warrants that during the term of this Agreement, the Physician is, and shall remain, duly licensed, registered and in good standing under the laws of the State of Washington to engage in the unrestricted practice of medicine and to administer and prescribe controlled substances, and is certified, or board-eligible for certification by the appropriate specialty board. Physician agrees to take all necessary action, include continuing medical education, to maintain such licensure and board-certification.

5.2 Credentials. Physician shall provide Hospice with credentialing information and other pertinent documentation as reasonably requested by Hospice, including, but not limited to, the following: background checks, copies of professional licenses, proof of competency, health statements (to demonstrate freedom from signs and symptoms of communicable disease), education and experience, and proof of professional liability insurance.

5.3 Disciplinary Actions. Physician further represents and warrants that he or she has not been subject to disciplinary or corrective action by any hospital or licensing agency or to exclusion or debarment from either the Medicare or Medicaid program. Physician shall immediately notify Hospice with respect to any formal allegation (including any complaint, indictment, or initiation of proceedings) made against him or her before any court of law, licensing or regulatory agency or authority, professional organization, or the medical staff or committee of any hospital.

ARTICLE 6

INSURANCE AND INDEMNIFICATION

6.1 Physician's Insurance. Physician shall, at Physician's sole cost and expense, procure, keep and maintain throughout the term of this Agreement, professional liability insurance coverage in the minimum amount of One Million Dollars ($1,000,000) per occurrence and Three Million Dollars ($3,000,000) annual aggregate, for professional liability, negligence, errors and omissions, and commercial general liability and applicable state statutory limits for workers compensation. If such professional liability insurance is issued on a claims-made basis and subsequently canceled or otherwise terminated, Physician shall obtain and maintain an extended reporting endorsement (also known as a "tail coverage") or obtain a policy with prior acts coverage covering all activities performed under this Agreement. The duty to maintain such coverage shall survive the expiration or termination of this Agreement

6.2 Hospice's Insurance. Hospice shall, at all times during the terms of this Agreement, maintain professional and general liability insurance in a minimum amount of One Million Dollars ($1,000,000) per occurrence and Three Million Dollars ($3,000,000) aggregate, and shall provide Medical Director with proof of such insurance upon execution of this Agreement, and periodically as requested. Hospice shall notify Physician of any change, modification, or termination of coverage thirty (30) days prior to the date of the change, modification of termination.

6.3 Indemnification. Physician and Hospice each agree to indemnify and hold harmless the other party for any and all losses, damages, costs and expenses arising from any omission, fault, negligence or other misconduct by it or its agents. These indemnification provisions shall survive termination or expiration of the Agreement.

ARTICLE 7

COMPLIANCE

7.1 No Obligation to Refer. The parties acknowledge that although Physician is obligated to provide certain Services (as specified in this Agreement), Physician is under no obligation to refer any patients to Hospice. Notwithstanding any unanticipated effect of any provisions of this Agreement, neither party shall conduct itself intentionally in a manner that would violate any federal or state law or any Medicare/Medicaid regulations relating to patient referrals in exchange for direct or indirect renumeration.

7.2 Fair Market Value. The parties acknowledge that compensation has been set in advance, is consistent with fair market value for Physician Services, and does not take into account the volume or value of any referrals.

7.3 Advance Directives. Hospice and Physician shall comply with the federal "Patient Self-Determination Act." Hospice and Physician shall inform each Hospice Patient about advance directives under applicable state law and ascertain the Hospice Patient's competency in accordance with applicable state and federal laws.

ARTICLE 8

MISCELLANEOUS

8.1 Independent Contractor. Nothing in this Agreement shall be construed to create an employee/employer, partnership, joint venture, association or other relationship between Hospice and Physician other than that of an independent contractor relationship.

8.2 Taxes. It is expressly agreed and understood that Hospice is not obligated to and shall not withhold any amount from Physician fees earned hereunder with respect to any state or federal employment compensation taxes. The payment of any such taxes is the sole obligation of Physician

8.3 Civil Rights. Hospice shall operate in accordance with the Civil Rights Act of 1964 and in accordance with other federal, state, and local rules and regulations as may be applicable in the delivery of Hospice services. No person shall be denied the services of Hospice based on race, creed, national origin, sex, age, color, religion, physical handicap, sexual preference or inability to pay.

8.4 Entire Agreement. This Agreement shall constitute the entire agreement between the parties and supersedes all prior oral or written agreements or understandings. This Agreement may be modified or amended only in a writing signed by the parties hereto.

8.5 Headings. Section headings contained in this Agreement shall be used for convenience of reference only and shall not be considered a part of the Agreement or used to define or affect the meaning, construction, or scope of any provision of the Agreement.

8.6 Severability. Each provision of this Agreement shall be deemed independent and terminable. The exclusion of any provision herein found to be invalid or illegal shall not affect enforcement of the remaining provisions of the Agreement.

8.7 Assignment. This Agreement shall not be assignable in whole or in part by either party without prior written consent by the other party.

8.8 Amendment. This Agreement may not be modified, amended or supplemented, nor may any term or condition be waived, except by an agreement in writing signed by both parties.

8.9 Arbitration**.** Any dispute or controversy arising under this Agreement or any amendment hereof or the breach hereof shall be settled by arbitration in the county in which the Hospice is located, in accordance with the rules of the American Health Lawyers Association Alternative Dispute Resolution Service and applying the laws Washington. Any award rendered by the arbitrator shall be final and binding upon each of the parties, and judgment thereof may be entered in any court having jurisdiction.

8.10 Attorney's Fees**.** If Physician or Hospice brings any action to interpret or enforce this Agreement, or for damages for any alleged breach hereof, whether by arbitration or otherwise, the prevailing party in any such action shall be entitled to reasonable attorneys' fees

and costs as awarded by the arbitrator in addition to all other recovery, damages and costs. This Section 8.10 shall survive the expiration or termination of this Agreement.

8.11 Governing Law. The rights and obligations of the parties under this Agreement, and any claims or disputes relating hereto, shall be governed by the laws of the State of Washington, without regard to any conflict of laws provisions, and all applicable federal laws, rules, and regulations.

8.12 Counterparts. This Agreement may be executed in any number of counterparts, each of which shall be deemed an original, but all of which shall constitute one and the same agreement.

8.13 Waiver. The waiver by a party of a breach or violation of any provision of this Agreement shall not be construed as a waiver of any subsequent breach or violation.

8.14 Notices. Any notices required under this Agreement shall be in writing and shall be deemed to have been given if: (a) sent by certified or registered mail, return receipt requested, with adequate postage; (b) sent by means of an express delivery service if it obtains a receipt to confirm delivery; or (c) by facsimile if such facsimile transmission permits confirmation to the addresses or facsimile numbers set forth below:

If to Physician: Dr. Josh Kretchmar

2222 E. Meridian Avenue East, Suite A
Edgewood, Washington. 98371
Ph: (206) 407-4171

If to Hospice: _____

_____

_____

Fax: _____

IN WITNESS WHEREOF, the parties or their duly authorized representatives have executed this Agreement on the date first set forth

Heart/n Soul Hospice of Yakima LLC	PHYSICIAN
_____	_____
Signature-Authorized Agent	Signature
_____	_____
Title	Print Namee.

Exhibit 9 – Draft Lease Agreement

Larson Building, LLC
901 Summitview Avenue #250, Yakima, WA 98902
ph: 509.453.8161 *christina@megalodonllc.net* *fx: 509.453.8189*

OFFICE LEASE

1.0 **LEASE SUMMARY:** The pages of this document herein constitute a **Lease** by and between **Larson Building, LLC,** hereinafter called "**Lessor**," and **Heart 'N Soul Hospice of Yakima, LLC,** hereinafter called "**Lessee**," made and entered into on **December 29th, 2025,** for the more or less **369** BOMA square foot premises ("Leased Premises") **Suite #706,** described as follows:

2.0 **LESSOR:** Larson Building, LLC
C/O Megalodon, LLC
901 Summitview Avenue, Suite #250
Yakima, WA 98902
(509) 453-8161

2.1 **LESSEE:** Heart 'N Soul Hospice of Yakima, LLC
6 South 2nd Street, Suite #706
Yakima, WA 98901
(206) 659-9998

3.0 **PREMISES:** For and in consideration of the covenants and agreements hereinafter set forth, **Lessor** does by this present Lease and let to **Lessee**, and **Lessee** does hereby Lease and rent from **Lessor** the following described property situated in Yakima, Yakima County, Washington:

> **Suite #706,** consisting of approximately **369** BOMA square feet ("Leased Premises") in the Larson Building, located at 6 South Second Street, Yakima, Washington, legally described as follows: The East 15 feet of Lot 10 and all of Lots 11 and 12, Block 31, TOWN O F NORTH YAKIMA, now the City of Yakima, Washington, according to the plat thereof as recorded in Volume "A" of Plats, page 10, and as recorded in Volume "E" of Plats, page 1, records of Yakima County, Washington.

4.0 **TERM:** This **Lease** creates a term of tenancy as follows:

4.1 **COMMENCEMENT DATE:** This Lease shall commence on **January 1st, 2026.**

4.2 **TERMINATION DATE:** This Lease shall terminate on **December 31st, 2030**, subject to Option to Renew as noted below.

5.0 **OPTION TO RENEW:** Provided Lessee is not in default, if Lessor or Lessee does not give written notice at least sixty (60) days prior to the termination of this Lease, described in

subsection 4.0.2, then this Lease shall automatically renew for an additional term of two (2) years, upon the same terms and conditions as contained herein. The cost of rent for a Lease renewal shall be figured as the dollar amount of one of the two following options, whichever is higher: (1) the average price per square foot being charged at that time for rent of office space in the Larson building; or (2) the rent amount given in Section 6.0 adjusted to reflect changes in the Consumer Price Index (CPI) between the commencement date and the termination date of this Lease with a minimum of 3% increase annually (January 1st). ***Under no circumstance shall the rent amount be less than Seven Hundred Dollars ($700.00) per month if renewed.** ****Note: BOMA standards upon renewal of Lease will be calculated and adjusted if necessary at that time.***

6.0 **RENT: Lessee** agrees to pay as rent the sum of **Seven Hundred Dollars ($700.00)** per month, in advance, on or before the first day of each calendar month beginning **January 1st, 2026. It shall be the responsibility of the Tenant to see that Owner receives the rent on the above agreed date. Lessor does not send out monthly statements. WE DO NOT HONOR POSTMARK DATES AS DATE RECEIVED.**

Yearly rent increases will be applied as follows:

Lease Term:

January 1st, 2026 through December 31st, 2026 $700.00 rent per month for a total of $8,400.00 for the year.

January 1st, 2027 through December 31st, 2027 $721.00 rent per month for a total of $8,652.00 for the year.

January 1st, 2028 through December 31st, 2028 $743.00 rent per month for a total of $8,916.00 for the year.

January 1st, 2029 through December 31st, 2029 $765.00 rent per month for a total of $9,180.00 for the year.

January 1st, 2030 through December 31st, 2030 $788.00 rent per month for a total of $9,456.00 for the year.

First Renewal Term:

January 1st, 2031 through December 31st, 2031 $812.00 rent per month for a total of $9,744.00 for the year.

January 1st, 2032 through December 31st, 2032 $836.00 rent per month for a total of $10,032.00 for the year.

6.1 **PRO-RATED RENT:** N/A.

6.2 **LATE CHARGE:** A late charge of **Ten Percent (10%)** of the monthly Lease amount will be charged if the rent is not received by **Lessor** by the Tenth **(10th)** day following the day the rent is due. Lessee and Lessor agree that said late charge is a reasonable estimation of Lessor's administrative expense in monitoring Lessee's non-payment. Payments shall be applied first to any late fees, then to any attorney fees or notice charges, then to any other charges authorized under the terms of this Lease, and then to the oldest month's rent owing.

6.3 **NSF CHECK FEE:** Lessee agrees to pay **Twenty-Five dollars ($25.00)** for each NSF check plus any expenses incurred by owner as a result thereof and acknowledges that late charges may be applied if beyond due date in relation to thereof.

6.4 **COST OF LIVING INCREASES:** The amount of rent provided for in paragraph 6, shall be adjusted in accordance with the following procedure: The Consumer Price Index for Urban Consumers (CPI-U) for western Type B/C cities Area Index 1996= 100 (hereinafter called the "Index") published by the Bureau of Labor Statistics of the United States, Department of Labor, shall be determined as of the commencement date of this Lease and as of each anniversary date of this Lease, including each anniversary date during any renewal term. If the Index is higher on any anniversary date than the previous Index as of the commencement date or last preceding anniversary date used to calculate the rent adjustment, then the monthly rent commencing for the period beginning with such anniversary date shall be increased by One Hundred Percent (100%) of such increase in the Index. If publication of the Index shall be discontinued, the parties shall there after accept comparable statistics on the cost of living for the City of Yakima or a city of comparable size and location as they shall be computed and published by an agency of the United States or a responsible financial periodical of recognized authority selected by **Lessor** with a **minimum of 3% increase annually (January 1st)**.

7.0 **SECURITY DEPOSIT: Lessee** shall pay **Lessor** a **Non-refundable Cleaning Fee of Two Hundred Dollars ($200.00)** and a Security Deposit of **Seven Hundred Dollars ($700.00)** on or before the commencement date of this Lease, or a total amount of **Nine Hundred Dollars ($900.00).**

8.0 **USE:** The premises shall be used by **Lessee** for conducting a **Home Healthcare and Hospice Services** business and activities reasonably related thereto which constitute a similar usage as to wear and tear and shall not be used for any other purpose without the express written consent and approval of **Lessor**. It is understood and agreed that there are no express or implied warranties as to fitness of said premises for said use. Leased property will not be used as a residence. No pets will be kept or kenneled in space.

9.0 **CONDITION OF PREMISES: Lessee** has inspected the premises, is fully familiar with and knows their condition, and accepts the same in their present condition **"AS/IS"** without any representation of **Lessor** regarding the condition thereof, the improvements thereon, or their tenant ability, *provided, however;* that **Lessor** shall be responsible for any modifications or alterations in the premises on **Lessor's** part to be performed to the extent expressly provided in "Exhibit C."

10.0 **SURRENDER OF PREMISES:** Upon termination of this Lease for any reason whatsoever, **Lessee** shall surrender the premises to **Lessor**, together with all alterations and replacements thereof, in good order, condition, and repair, except for reasonable wear and tear and damage by insured risk.

11.0 **IMPROVEMENTS:** All permanent improvements constructed on the demised premises shall be considered a part of the real estate and belong to the **Lessor** upon termination of this Lease.

12.0 **MAINTENANCE AND REPAIRS: Lessee** shall, at all times, preserve and maintain the leased premises

in good order, condition, and repair, except for reasonable wear and tear and damage by insured risk, **Lessee** shall permit no waste, injury, or damage to the premises. If **Lessee** fails or neglects to perform its obligation to preserve and maintain the premises in good condition and repair, then **Lessor** may, at its option, cause the same to be put into good condition and repair; and in such case, **Lessee** shall pay the cost thereof on demand.

12.1 **MAINTENANCE OF COMMON AREAS:** **Lessor** shall, as much as reasonably possible, maintain the condition and safety of the common areas, exterior building, including graffiti control and building signage. All repairs and maintenance of the premise's common areas as well as suite exterior windows and walls will be paid by **Lessor** which will be reimbursed by **Lessee** for damages caused by the acts of **Lessee**, its employees or clients.

13.0 **ALTERATIONS:** **Lessee** shall not make any additions or material alterations to or upon the premises without first obtaining the written consent of **Lessor**, and any such additions or alterations authorized by **Lessor** shall be at **Lessee's** sole expense. In making any alterations or additions, **Lessee** shall comply with all building code provisions, municipal ordinances and regulations, and state laws which may affect or govern such work. All such additions or alterations shall become and remains the property of **Lessor**, provided, however, that upon the termination of this Lease, **Lessee** shall, at its expense, promptly remove such additions or alterations if so requested by **Lessor**.

14.0 **UPKEEP AND REPAIRS:** **Lessee** agrees during the term of this Lease to make all reasonably necessary repairs and maintain at all times, the plate glass, painting, upkeep, and maintenance necessary to keep the leased property in as good a condition as at the commencement of the term hereof, reasonable and ordinary wear and tear only excepted. Should **Lessee** not perform any reasonably necessary repair and maintenance, **Lessor** may do so, and **Lessee** agrees to reimburse **Lessor** for the total of all such repairs and maintenance, within ten (10) days after written demand thereof. **Lessee** shall repair all damage caused by robberies, vandalism and/or break-ins. This includes doors, thresholds, glass, and walls. **Lessee** agrees to perform all of the same promptly as necessary and in a good and workmanlike manner. **Lessor** will maintain exterior of the building in a proper condition for occupancy and make all repairs relating to structural walls, roofs, foundations, electrical, plumbing, and HVAC systems as necessary for operation of the premises.

15.0 **LIENS:** **Lessee** shall not cause or permit any liens of any nature to be placed against the premises except liens placed thereon by **Lessor**, and **Lessee** shall save **Lessor** harmless from and on account of all liens and all expenses and indebtedness connected therewith, except those relating to liens placed thereon by **Lessor**. **Lessee** will not permit any mechanics' or other liens to be imposed or stand against the leased premises for work done or materials furnished.

16.0 **COMPLIANCE WITH LAWS:** **Lessee** covenants, promises, and agrees to comply with all charters, laws, ordinances, rules, and regulations, to obtain all necessary licenses and permits applicable to **Lessee's** activities and to the premises, and to pay all fees and charges in connection therewith or by reason of inspections thereof.

17.0 **TAXES AND ASSESSMENTS:** **Lessor** shall pay all real estate taxes and assessments levied against the Leased premises. **Lessee** shall pay all other taxes, assessments, inspection fees and charges hereafter levied against the Leased premises or the Lease during the term of this Lease, arising out of the occupancy or use thereof, or arising out of the conduct of **Lessee's** activities thereon.

18.0 **UTILITIES:** As long as **Lessee** is not in default in any of the provisions of this Lease, **Lessor** shall furnish a reasonable amount of electricity for normal lighting and office equipment customarily found in commercial offices, heat, water, elevator service, and normal office air conditioning but shall not be liable for, nor shall rental be abated for, interruption of such services caused by accident or necessary for repairs, for improvements, or for any other reason beyond **Lessor's** control. **Lessor** shall furnish common area janitorial service for the premises to **Lessor's** specifications. Janitorial service to include trash removal up to three times per week and vacuuming of the floors at least every other week to the interior of leased suites.

19.0 **INSURANCE:** **Lessee** shall maintain public/general liability insurance in the amount of no less than $1,000,000.00 at all times during the term of this Lease. **Lessor** and **Lessee** may carry, at their own expense, other such fire and extended coverage insurance as each of them deems necessary for their own protection and neither party shall have an interest in or claim to such insurance of the other party. **Lessee** shall not permit the premises to be used in such a manner as to create a fire hazard or to cause a change in the insurance rating applicable to the premises.

20.0 **DAMAGE WAIVER:** **Lessor** and **Lessee** do hereby release and discharge each other from and against all liability for loss or damage caused by any of the perils covered by insurance policies which are in force and effect at the time of any such loss or damage, even though such loss or damage may be due to the negligence, act, or neglect of **Lessor** or **Lessee**, or agents or employees of either party. It is expressly understood and agreed that it is the intention hereof to constitute a waiver and release of any and all subrogation rights which the insurance companies might have under such insurance policies.

21.0 **INDEMNITY:** **Lessee** covenants and agrees to defend, indemnify, and hold **Lessor** harmless from and against any and all claims or liability for injury, damage, or loss, including attorneys' fees and costs, which may arise or to which **Lessor** may be subjected during or as a result of **Lessee's** occupation or use of the premises or the conduct of any activities thereon by **Lessee** or with **Lessee's** permission or knowledge.

22.0 **RISK OF LOSS:** All property of any kind on the premises shall be at the risk of **Lessee**, and **Lessor** shall not be liable, and **Lessee** waives all claims for any loss, damage, or injury either to persons or property sustained by **Lessee** or any other person upon or about the premises, or due to the structures or any improvements upon the premises or the adjoining premises or any part thereof, becoming out of repair or arising from the overflow of water or the freezing, bursting, or leakage of water, gas, heating or steam pipes or due to any act, omission, or neglect of **Lessor** or **Lessee**, or any of their agents or employees, or any other person upon or about the premises, or any other cause of any nature whatsoever. No eviction from the premises shall be claimed by **Lessee** by reason of the happening of any or all of the foregoing. Without limiting the generality of the foregoing, **Lessee** shall be solely responsible for theft or other similar loss of fixtures, equipment, or other property Leased herein and any other property of **Lessee**.

23.0 **INGRESS AND EGRESS:** **Lessor** reserves the right of ingress and egress to and from the leased premises for the purpose of inspecting the same at all reasonable times, with 24 hour advance notice, except in the case of emergency or equipment failure, in which case as much notice as is practicable shall be given, and for making such repairs as **Lessor** is obligated to make under the terms of this Lease. No compensation shall be made to or claimed by **Lessee** from **Lessor** by reason of inconvenience, annoyance, or damage of any kind whatsoever arising from the making of repairs to or maintenance or alteration of the building or appurtenances to the leased premises. **Lessor** reserves the right to make repairs, alterations, connections, or extensions when and where the same may be deemed by **Lessor** to be necessary.

24.0 **ASSIGNMENT AND SUBLETTING:** **Lessee** shall not assign this Lease or sublet the premises herein, or any portion thereof, without first obtaining the written consent of **Lessor**; nor shall there be any transfer or

assignment of this Lease from **Lessee** by operation of law, either voluntarily or involuntarily or by dissolution, consolidation, or merger of **Lessee**. Consent to such assignment or subletting shall not operate to relieve **Lessee** of any of its covenants and obligations under this Lease or relieve **Lessee** or its successor in interest from the necessity of obtaining like consent for any subsequent assignment or subletting.

25.0 **DAMAGE OR DESTRUCTION:** If the premises are destroyed or damaged by fire or other casualty rendering them, in **Lessor's** judgment, un-leasable, **Lessor** may, at its option, cancel this Lease or may immediately proceed to rebuild and restore the same. Within ten (10) days after such destruction or damage, **Lessor** shall notify **Lessee** in writing whether **Lessor** elects to cancel this Lease or rebuild and restore the premises. In the event **Lessor** elects to cancel this **Lease**, the rent shall be paid to the date of destruction or damage, and all obligations of the parties hereto with respect to the unexpired portion of the term shall thereupon terminate. In the event **Lessor** elects to rebuild and restore the premises, such rebuilding or restoration shall be commenced as soon as practical, shall be completed with due and reasonable diligence, and shall replace the improvements as nearly as practicable to the condition existing immediately prior to such damage or destruction; *provided* that any delay occasioned by governmental regulations or any other cause beyond the direct control of **Lessor** shall be taken into consideration in determining the promptness with which **Lessor** commences and completes the restoration and rebuilding. Rent shall be abated during the rebuilding and restoration proportionately in the same ratio as the un-leasable portion of the premises bears to the whole premises. Notwithstanding **Lessor's** election, in the event the premises are destroyed or damaged by fire or other casualty rendering them, in **Lessee's** judgment, un-leasable, **Lessee** may, at its option, cancel this Lease.

26.0 **CONDEMNATION:** If the whole or any substantial part of the premises ("substantial" being defined as reasonably preventing or unreasonably interfering with the conduct of **Lessee's** activities) be taken or condemned by the competent authority, this Lease shall terminate upon the date when possession of the premises so taken shall be acquired by such authority, and the rent shall be prorated as of the date of such termination. If less than a substantial part of the premises be taken or condemned by any competent authority, the rent shall be abated, proportionately in the same ratio that the part of the premises taken or condemned bears to the whole premises, from the date when possession of that part of the premises so taken shall be acquired by such authority. **Lessor** shall be entitled to the full amount of any condemnation award for the leased premises, and **Lessee** hereby expressly waives any right or claim to any part thereof as damages or otherwise and any right or claim against **Lessor** as a result of such taking or condemnation. **Lessee** shall have the right to claim and recover from the condemning authority such compensation as may be separately awarded or recoverable by **Lessee** in **Lessee's** own right on account of any and all damage to **Lessee's** activities by reason of the condemnation and on account of any cost or loss which **Lessee** might sustain.

27.0 **DEFAULT:** Full and prompt performance by **Lessee** of all terms and conditions of this Lease is hereby made the essence of this Lease. If **Lessee** should be in default in any of the same and such default shall have continued for ten (10) days in the case of nonpayment of rent or other sums due from **Lessee** and for thirty (30) days in the case of any other default after written notice by **Lessor** to **Lessee** setting forth the particular default claimed, or if **Lessee's** leasehold estate shall be taken on execution, or if **Lessee** shall be declared bankrupt or insolvent according to law, or if **Lessee** shall make an assignment for the benefit of creditors, or if a receiver shall be appointed for **Lessee** and continue in office thirty (30) days without discharge, then in any such event this Lease shall, at the option of **Lessor**, be forfeited. In such event, **Lessor** shall provide **Lessee** ten (10) business days written notice to cure the defect or vacate the premises. If such ten (10) day period expires without the defect being cured, **Lessor** may lawfully enter into and upon said premises or any part thereof, repossess the same, and expel **Lessee** and those claiming under and through **Lessee** and remove their effects, forcibly if necessary, without being deemed guilty of any manner of trespass, but without prejudice to any remedies which might otherwise be used by **Lessor** for the collection of rent or for any

breach by **Lessee** of the covenants herein contained. It is further agreed that after service of notice as above set forth, an additional condition to avoid forfeiture shall be payment by **Lessee** of **Lessor's** costs and expenses, including reasonable attorneys' fees, for the preparation and service of such notice. Nothing contained herein shall release or diminish **Lessee's** obligation to pay rent for the full term of this **Lease**, except the net amount of rent **Lessor** receives from any subsequent **Lessee** during the term hereunder. **Lessor** shall take reasonable action to mitigate all damages resulting from **Lessee**'s default including attempting to rent the premises on reasonable terms and conditions determined by **Lessor** and apply the proceeds to reduce the rent due and owing from **Lessee** and **Lessee** shall be liable and will confess judgment for any deficiency. As an additional and not alternative remedy, optional with **Lessor**, if **Lessee** should be in default hereunder other than a default in the payment of rent, **Lessor** may cure or correct the same; and the cost of such action by **Lessor** shall immediately be due and payable from **Lessee**, plus interest on said sum at the rate of twelve percent (12%) per annum until paid; and nonpayment of said sum by **Lessee** shall be adequate grounds for **Lessor** to invoke the other provisions of this paragraph.

28.0 **NOTICE:** Any notices served upon **Lessee** for failure to comply with the terms of the Lease agreement, rules or regulations, or nonpayment of rent, shall incur charges payable to **Lessor** of **One Hundred Fifty Dollars ($150.00)** per notice served. All notices required under this Lease by either party shall be effective; (1) If mailed by certified mail, postage prepaid, and properly addressed to the parties at the address shown below their signature lines. Such notices shall be effective as of the date of mailing, as evidenced by the official United States Post Office date and time stamp on the certified mail receipt or; (2) All notices personally served by either party shall be effective as of the date of personal service thereof or the date of distribution, and properly addressed to the other party at the following address or as the same may be changed from time to time by written notice to the other party:

Lessor: Larson Building, LLC
C/O Megalodon, LLC
901 Summitview Avenue, Suite #250
Yakima, WA 98902

Lessee: Heart 'N Soul Hospice of Yakima, LLC
15 S Grady Way #522
Renton, WA 98057-3217

29.0 **NOTICE TO VACATE:** The **Lessee** understands that to terminate tenancy, Sixty (60) days written notice must be given to Owner or Agent prior to the end of any monthly term, after said Lease has expired during any holdover period. Lessor also reserves the right to issue notice to Lessee on same Sixty (60) day term. According to the terms of the agreement, rent has been prorated to the first day of each calendar month.

30.0 **ATTORNEYS' FEES AND COSTS: Lessee** shall pay **Lessor** all costs and expenses, including reasonable attorneys' fees, incurred by **Lessor** in the preparation and service of any notice which **Lessor** gives to **Lessee** under the terms of this Lease. In any court suit or action brought by either party concerning this Lease, the court shall award to the prevailing party reasonable attorneys' fees in addition to the costs allowed by law, and the other party agrees to pay the same.

31.0 **GOVERNING LAW AND VENUE:** This Lease shall be construed in accordance to the laws of the State of Washington. Venue for any action arising out of this Lease shall be in the County of Yakima.

32.0 **NONWAIVER:** The consent of **Lessor** to any variation of the terms of this Lease, or the receipt of rent with knowledge of any breach, shall not be deemed to be a waiver of any breach or covenant of this Lease, and no waiver shall be claimed by **Lessee** unless it be a written waiver signed by **Lessor**. Any waiver, express or implied, by **Lessor** of any breach by **Lessee** of any of the covenants of this Lease shall not be construed to be a waiver of any subsequent breach of the same or any other covenant in this Lease, or affect or prejudice any of **Lessor's** rights or remedies hereunder. After service of notice of default or commencement of suit, **Lessor** may receive and collect rent due; and the same shall not affect such notice or suit or any judgment, nor shall it be deemed a waiver in any sense.

33.0 **DELAYED POSSESSION:** In the event of the inability of **Lessor** to deliver possession of the premises at the time of the commencement of the term of this Lease, neither **Lessor** nor its agents shall be liable for any damage caused thereby; nor shall this Lease thereby become void or voidable; nor shall the term herein specified be in any way extended; but in such event, **Lessee** shall not be liable for any rent until such time as **Lessor** can deliver possession.

34.0 **SUBORDINATION:** This Lease is subject to and is hereby subordinated to all present and future mortgages, deeds of trust, and other encumbrances granted by **Lessor** and affecting the demised premises or the property of which said premises are a part. **Lessee** agrees to execute, at no expense to **Lessor**, any instrument which may be deemed necessary or desirable by **Lessor** to further effect the subordination of this **Lease** to any mortgage, deed of trust, or encumbrance. **Lessee** hereby irrevocably appoints and constitutes **Lessor** as the true and lawful attorney of **Lessee** at any time for **Lessee**, and in **Lessee's** name, place, and stead, to execute proper subordination agreements to this effect, provided that **Lessor** shall notify **Lessee** of any such actions as **Lessee's** attorney in fact.

35.0 **HOLDING OVER:** If **Lessee,** with the consent, expressed or implied, of **Lessor**, shall hold over after the expiration of the term of this Lease, **Lessee** shall remain bound by all the terms, covenants, and agreements hereof, except that the tenancy shall be one from month to month. During any holdover period rent shall automatically increase by a minimum of 3% on each yearly anniversary date (January 1st) of said original lease.

36.0 **SUCCESSORS:** Subject to the foregoing provisions relating to assignment and subletting, this Lease shall inure to the benefit of and shall be binding upon the successors and assigns of the respective parties hereto.

37.0 **RULES AND REGULATIONS – FAILURE TO COMPLY CONSTITUTES GROUNDS FOR EVICTION OR NON-RENEWAL OF LEASE:** **Lessee** agrees to abide by all reasonable and lawful rules and regulations of the building which may be made by **Lessor** from time to time and shall use all reasonable methods to require employees, customers, clients, and all persons invited by **Lessee** into the building to observe the same. **Lessee** hereby acknowledges receipt of a copy of the current Larson Building Rules, attached hereto as Exhibit 'B' and incorporated herein by reference. ***Note: If Lessee does not abide by the agreed upon Rules set forth in this agreement the Lessor has the right to terminate this Lease at any time or may choose not to renew this Lease at the end of this term.***

38.0 **CUMULATION OF REMEDIES – NO WAIVER:** No right to remedy herein expressly conferred upon or reserved to **Lessor** is intended to be exclusive of any other right or remedy, and each and every right and remedy shall be cumulative and in addition to any other right or remedy given hereunder or now or hereafter existing at law or in equity or by statute. The failure

of **Lessor** to insist in any one or more instances on strict performance by **Lessee** of the covenants of this Lease or to exercise in any option herein contained shall not be construed as a waiver or a relinquishment for the future of any such covenant or option. The receipt by **Lessor** of rent with the knowledge of the breach of any covenant or agreement hereof shall not be deemed a waiver of such breach, and no waiver by **Lessor** of any provision of this Lease shall be deemed to have been made unless expressed in writing and signed by **Lessor** or its duly authorized agent.

39.0 **ENTIRETY OF AGREEMENT:** No prior stipulation, agreement, or understanding, verbal or otherwise, of the parties or their agents shall be valid or enforceable unless embodied in the provisions of this Lease. The terms and conditions of this Lease shall not be amended or modified, except in writing signed by both **Lessor** and **Lessee**. Headings used herein are for convenience only, are not a part of this Lease, and are not to be used in construing it. Each party agrees to execute, upon request of the other, a short form of this Lease for purposes of recordation, and to re-execute this Lease at any time upon the request of the other.

40.0 **SPECIAL PROVISIONS:** The provisions checked below are attached hereto as exhibits and are incorporated herein by reference:

- Addendum to Office Lease (If applicable) - Exhibit A
- Building Rules – Exhibit B
- Lessee/Lessor Improvements (If applicable) – Exhibit C

ACKNOWLEDGEMENT AND SIGNATURE PAGE TO FOLLOW

Larson Building, LLC
901 Summitview Avenue #250, Yakima, WA 98902
ph: 509.453.8161 *christina@megalodonllc.net* *fx: 509.453.8189*

ACKNOWLEDGEMENT AND SIGNATURE PAGE

LESSOR: Larson Building, LLC

By: _____ Date _____
Managing Member,
Larson Building, LLC

By: Mary Schwarz Date 12/30/2025
Commercial Manager

LESSEE: Heart 'N Soul Hospice of Yakima, LLC

By: _____ Date _____
Nathan Yemane, Managing Member
Heart 'N Soul Hospice of Yakima, LLC

By: _____ Date _____
Personal Guarantee

For Formal Notices

ADDRESS NOTICES:

Larson Building, LLC
C/O Megalodon, LLC
901 Summitview Ave., Suite #250
Yakima, WA 98902

Telephone: (509) 453-8161
Facsimile: (509) 453-8189

ADDRESS NOTICES:

Heart 'N Soul Hospice of Yakima, LLC

15 S Grady Way # 522
Renton, WA 98057-3217

Telephone: (206) 659-9998
Email: nyemane@heartnsoulhospice.com

For maintenance emergencies please call 509-930-3007 and leave a message.

Please make all checks and money orders payable to: *Larson Building, LLC*.

NOTARY PAGE TO FOLLOW

By: _____
Mary Schwarz, Commercial Manager **Date**
Larson Building, LLC

STATE OF WASHINGTON)
: ss.
County of Yakima)

ON THIS DAY Personally appeared before me **Mary Schwarz, Commercial Manager of Larson Building, LLC,** to me known to be or proven to be the individual described in and who executed the within and foregoing instrument, and who acknowledged to me that he/she signed the same as his/her free and voluntary act and deed, for the uses and purposes therein mentioned.

GIVEN Under my hand and official seal this _____ day of _____, 2025.

_____
NOTARY PUBLIC In and for the State of Washington;
Residing in Yakima County.

Name as Commissioned: _____

My Commission Expires: _____

Notary Signature: _____

By: _____
Nathan Yemane, Managing Member **Date**
Heart 'N Soul Hospice of Yakima, LLC

STATE OF Washington)
: ss.
County of Yakima)

ON THIS DAY personally appeared before me _____to me known to be or proven to be the individual described in and who executed the within and foregoing instrument, and who acknowledged to me that he/she signed the same as his/her free and voluntary act and deed, for the uses and purposes therein mentioned.

GIVEN Under my hand and official seal this _____ day of _____, 2025.

_____
NOTARY PUBLIC In and for the State of Washington;
Residing in Yakima County.

Name as Commissioned: _____

My Commission Expires: _____

Notary Signature: _____

Larson Building, LLC
901 Summitview Avenue #250, Yakima, WA 98902
ph: 509.453.8161 *christina@megalodonllc.net* *fx: 509.453.8189*

EXHIBIT B

LARSON BUILDING RULES

1. No advertisement, sign, lettering, notice or device shall be placed in or upon the premises, including windows, walls, and exterior doors, except with the prior written approval of **Lessor**.

2. The initial cost for lettering upon the building directory board and the doors shall be paid by **Lessor** and made by the sign company designated by **Lessor**. In the event a **Lessee** moves within the building or desires any changes in the lettering upon the building directory board and doors, the cost for such changes shall be paid by **Lessee** and made by the sign company designated by **Lessor**. Such costs shall be competitive and reasonably comparable to those of other sign companies located in Yakima, Washington. The directories of the building will be provided exclusively for the display of the name and location of **Lessees** and their managers or representatives only, and **Lessor** reserves the right to exclude any other names.

3. No additional locks shall be placed upon any doors of the premises, and **Lessee** agrees not to have any duplicate keys made without the prior written consent of **Lessor**. If more than two keys for any door lock are desired, such additional keys shall be paid for by **Lessee**. Upon termination of this Lease, **Lessee** shall surrender all keys.

4. No furniture, freight, or objects of any kind shall be brought into the building in which the premises are located without **Lessor's** consent, and all moving of articles into, out of, or within the building shall be done at the time and in the manner that **Lessor** designates. **Lessor** shall have the right to prescribe the weight, size, and position of all safes and other heavy objects brought into the building and the time and manner of moving them into, out of, or within the building. If it is considered necessary by **Lessor**, safes or other heavy objects shall stand on structural elements to properly distribute the weight. In the event that **Lessor**, at its option, elects to paint or replace any floor covering within the premises occupied by **Lessee**, any moving of objects of furniture or other objects of **Lessee** within the premises for such purposes shall be at **Lessee's** sole expense. All damage done to the building by moving, locating, or maintaining any property moved into, out of, or within

the building by or for **Lessee** shall be repaired at **Lessee's** cost. Any modifications necessary to support the weight of heavy objects shall be at **Lessee's** sole cost and expense.

5. The entrances, corridors, stairways, and elevators shall not be obstructed by **Lessee** or used for any purpose other than ingress and egress to and from the leased premises.

6. The entrances, corridors, stairways, and elevators shall not be used as a waiting area for guests or clients. If **Lessee** conducts its business practices in a manner that results in guests or clients to enter the Larson Building and not immediately receive services from the **Lessee** (i.e. guests or clients are often required to wait before being served by the Lessee), the Lessee must provide a suitable "waiting area" as part of the leased premises described in Section 3.0 of the Lease.

7. **Lessee** shall not disturb other occupants of the building by making any undue or unseemly noise or otherwise. **Lessee** shall not, without **Lessor's** written consent, install or operate in or upon the premises any machine or machinery causing noise or vibration perceptible outside of premises, electric heater, or stove or carry on any mechanical business thereon or keep or use thereon oils, burning fluids, camphene, kerosene, naphtha, gasoline, or other burning or combustible materials. No explosives shall be brought into the building.

8. The interior walls, other than stone or paneling, are made of plaster. **Lessee** shall not mark, drive nails, or screw or drill into woodwork or plaster, paint, or in any way deface the building or premises or any part thereof or fixtures therein. The expense of remedying any breakage, damage, or stoppage resulting from a violation of this rule shall be borne by **Lessee**. Before attaching anything to the interior walls, **Lessee** must first contact the maintenance supervisor or building management.

9. Canvassing, soliciting, and peddling in the building are prohibited, and **Lessee** shall cooperate to prevent such activity.

10. Requests by **Lessee** will be attended to only upon application at the building office. Building employees shall not perform any work or do anything outside of their regular duties, except on issuance of special instructions from the office of the building. If building employees are made available for the assistance of **Lessee**, **Lessor** shall be paid for their services by **Lessee** at reasonable hourly rates. No building employee is required to admit any person (**Lessee** or otherwise) to any office without specific instructions from the office of the building.

11. **Lessor** reserves the right to close and keep locked all entrance and exit doors of the building on Saturdays, Sundays, legal holidays, and between the hours of 5:30 p.m. of any day and 6:30 a.m. of the following day, and during such hours as **Lessor** may deem advisable for the adequate protection of the building and the property of the **Lessees**. **Lessee** and each of its employees designated by **Lessee** shall be issued a code for the purpose of access to the building during days and hours that the building is closed. **Lessee** and each of its designated employees shall maintain the confidentiality of such codes and shall not disclose or allow the use of such codes by persons other than **Lessee** or its

designated employees.

12. Subject to Section 10 of the Rules and Regulations, written above, **Lessor** may take steps to control access to the building when it is locked. **Lessor** assumes no responsibility for and shall not be liable for any damage resulting from any error in regard to identification of **Lessee** or its employees or resulting from admission to or exclusion from the building or leased premises.

13. **Lessee** shall exercise care and caution to ensure that all water faucets or water apparatus, electricity, and gas are carefully and entirely shut off and that all windows are closed before **Lessee** or its employees leave the building, so as to prevent waste or damage. **Lessee** shall be responsible for any damage to the leased premises or the building and for all damage or injuries sustained by other Tenants/**Lessees** or occupants of the building arising from **Lessee's** failure to observe this provision.

14. **Lessee** is responsible for its guests, invitees, clients, and customers, and shall be held liable for the damages that such individuals may cause upon the property of the **Lessor** or of other building **Lessees** and their guests and invitees.

15. **Lessor** reserves the right to exclude or expel from the building any person who, in the judgment of **Lessor**, is disruptive, causes a nuisance, damages or steals property, is under the influence of liquor or drugs, or who shall, in any manner, do any act in violation of any of the rules and regulations of the building.

16. **Lessor** reserves the right to make such other reasonable regulations as, in its judgment may, from time to time be needed or desirable for the safety, care, and cleanliness of the premises or building and the preservation of good order therein.

17. Without prior written consent of **Lessor**, **Lessee** shall permit no auction sales, fire sales, bankruptcy sales, or "going out of business" sales from, on, or around the premises.

_____

Lessee Signature Date

Exhibit 10 – Availability of Funds Letter

December 30, 2025

Eric Hernandez, Program Manager
Certificate of Need Program
Department of Health
111 Israel Road Southeast
Tumwater, WA 98501

Dear Mr. Hernandez:

As one of the governing members of Heart'n Soul Hospice of Yakima County, I am writing this letter to confirm Heart'n Soul's financial commitment to provide the funding needed for the capital expenditure, initial operating losses, and any start-up costs.

In addition, also included with the application, is a letter from Regions Bank documenting that the funds for this commitment are available.

Please do not hesitate to contact me if you have any questions or require any additional information.

Sincerely,

Nathan Yemane
Governing Member

Heart'N Soul Hospice
15 S. Grady Way #101 Renton, WA 98057
Ph: 206-659-9998
Fax: 425-572-6639

REGIONS

December 22, 2025

Heart N Soul Hospice LLC
7240 Chase Oaks Blvd.
Plano, TX 75025

To Whom It May Concern:

Please accept this letter as confirmation that Heart N Soul Hospice of Yakima has adequate funds available to meet the required $500,000 capital expenditure and start- up costs related to the establishment of a Hospice agency in Yakima County.

Please contact me if you have any questions or need additional information at 469-608-2693.

Kindest Regards,
Kalela Woolen

Kalela Woolen
Commercial Relationship Specialist II
1717 McKinney Avenue, Suite 1100 | Dallas, TX 75202
Ph: (469) 608-2693 | Email: kalela.woolen@regions.com

Confidential